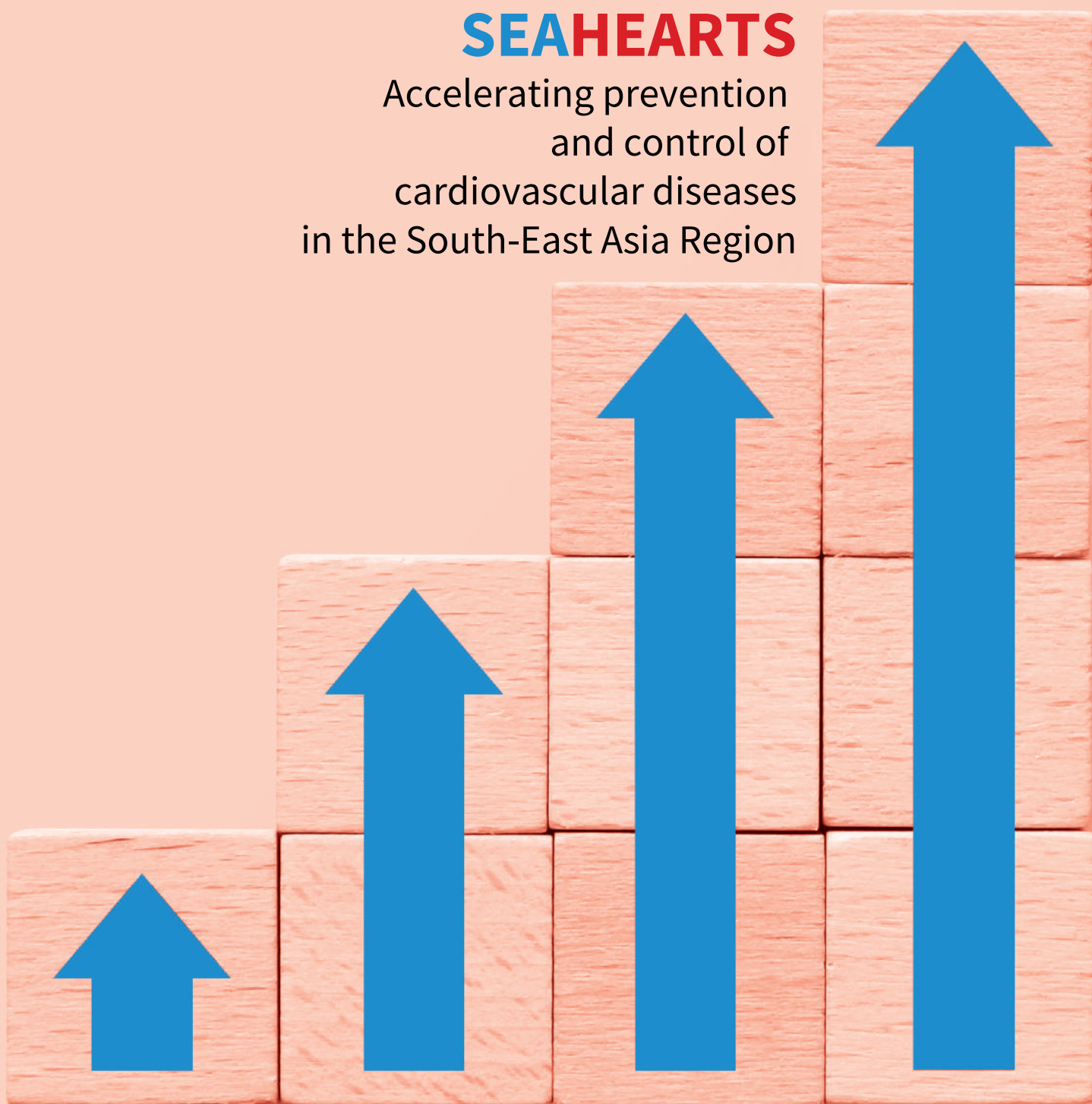


Progress report

SEAHEARTS

Accelerating prevention
and control of
cardiovascular diseases
in the South-East Asia Region



Progress report

SEAHEARTS: Accelerating prevention and control of cardiovascular diseases in the South-East Asia Region



Progress report – SEAHEARTS: Accelerating prevention and control of cardiovascular diseases in the South-East Asia Region

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Facts and data pertaining to the Republic of Indonesia are included in this report as the Member State was affiliated to the World Health Organization's South-East Asia Region at the time the report was commissioned and prepared. It is to be noted here that Indonesia was reassigned to the WHO Western Pacific Region with effect from June 2025 following a resolution to that effect adopted by the Seventy-eighth World Health Assembly in Geneva in May 2025.

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Contents

List of figures and tables.....	iv
Abbreviations and acronyms.....	v
Acknowledgments.....	vi
Foreword.....	vii
1. Introduction	1
2. Data collection and validation.....	3
3. Progress of the SEAHEARTS milestones	4
– Progress on the milestone on hypertension and diabetes treatment coverage	5
– Progress on the milestone for tobacco control.....	6
– Progress on the milestone for reducing salt intake.....	7
– Progress on the milestone for trans-fatty acid elimination policies and measures	8
– Progress on the SEAHEARTS resolution mandate to WHO	9
4. Planning for SEAHEARTS milestones 2030.....	17
5. References.....	18
6. Country status reports on the SEAHEARTS initiative implementation	21
7. Annexure 1. Checklist to guide the national implementation of SEAHEARTS milestones	89

List of figures and tables

Figures

- Fig. 1. Status of the achievement of the SEAHEARTS milestones, December 2024
- Fig. 2. Country commitments to improve hypertension and diabetes treatment coverage by 2025
- Fig. 3. Status of hypertension and diabetes treatment coverage in the Region, December 2024
- Fig. 4. Status of reported numbers of individuals with controlled hypertension and diabetes among those on treatment in the Region, December 2024
- Fig. 5. Status of implementing individual WHO MPOWER measures at the highest level of achievement in the WHO SE Asia Region, December 2024
- Fig. 6. Status of implementing WHO SHAKE policies and interventions in the WHO SE Asia Region, December 2024
- Fig. 7. Status of implementing TFA elimination policies and measures in the WHO SE Asia Region, December 2024
- Fig. 8. Roadmap for scaling up hypertension and diabetes coverage in India
- Fig. 9. Subnational (municipality-wise) targets for hypertension and diabetes treatment coverage to reach national target, Timor-Leste

Tables

- Table 1. Status of hypertension and diabetes treatment coverage (as of December 2024)
- Table 2. Status of WHO MPOWER measure implementation (as of December 2024)
- Table 3. Status of WHO SHAKE measure implementation (as of December 2024)
- Table 4. Status of WHO REPLACE measure implementation (as of December 2024)

Abbreviations and acronyms

ANM	auxiliary nurse midwife
CVD	cardiovascular diseases
DHIS	District Health Information Software
FoPL	front-of-pack labelling
GTCR	Global Tobacco Control Report
HA	health assistant
HbA1c	glycated haemoglobin
HDL	high-density lipoprotein
HMIS	Health Management Information System
LDL	low-density lipoprotein
LMIS	Logistics Management Information System
MSAP	Multisectoral Action Plan
NCD	noncommunicable disease
NCD CCS	Noncommunicable Disease Country Capacity Survey
NEML	National Essential Medicines List
PEN	package of essential noncommunicable disease interventions
PHC	primary health care
SE	South-East
TFA	trans-fatty acid
WHO	World Health Organization
WHO-SEARO	WHO Regional Office for South-East Asia



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Foreword



Cardiovascular diseases (CVDs) remain the leading cause of death in the WHO South-East Asia Region, responsible for nearly one in four lives lost. The majority of these deaths are preventable. Recognizing the need for action, Member States, at the Seventy-sixth session of the WHO Regional Committee for South-East Asia in 2023, adopted the resolution 'SEAHEARTS: Accelerating Prevention and Control of Cardiovascular Diseases in the South-East Asia Region'.

SEAHEARTS outlines four ambitious milestones to be achieved by 2025: reducing tobacco use, reducing dietary salt intake, protecting people from the harmful effects of trans-fatty acids, and improving the treatment coverage of hypertension and diabetes. I am pleased to note that one year into implementation, we have made significant progress: Over 77 million people have been placed on protocol-based management for hypertension and diabetes; more than 2 billion people are covered by measures to reduce salt intake and protect against trans-fatty acids; and significant advances have been made in tobacco control. These achievements are evidence of the region's determination to address the burden of CVDs through collective and coordinated actions.

This first progress report documents the actions taken, the lessons learnt, and the challenges that remain. It also highlights the opportunities to build on momentum for regional collaboration and shared learning, and to accelerate results.

I commend Member States, national partners, and the health workforce in driving the SEAHEARTS agenda. Building on the progress achieved, we must maintain momentum, scale up efforts, and ensure that gains are sustained to meet the 2025 targets and beyond.

This report is both a recognition of progress made, as well as a call to action—to transform policy into impact for a stronger and healthier South-East Asia.

Dr Catharina Boehme
Officer-in-Charge
WHO South-East Asia

Introduction

Cardiovascular diseases (CVD) are a leading cause of mortality and morbidity in the WHO South-East (SE) Asia Region. Global Health Estimates of 2021 indicated that these contributed to 4.3 million deaths, which constitute one third (32%) of all deaths in the Region. Nearly half (44.7%) of CVD deaths are premature in the age group of 30–70 years (1). Main risk factors for CVD include modifiable lifestyle factors – tobacco use, unhealthy diets including high dietary salt intake, trans-fatty acids, lack of physical activity and alcohol consumption. Raised blood pressure and blood glucose levels are key metabolic risk factors contributing to mortality due to CVD in the Region.

In 2023, the Seventy-sixth Session of the WHO Regional Committee for South-East Asia adopted the resolution on “SEAHEARTS: Accelerating prevention and control of cardiovascular diseases in the South-East Asia Region (SEA/RC76/R5)” (2). The resolution aims to accelerate the national responses with time-bound commitments to reducing CVD risk factors, and strengthening and reorienting health systems to meet the 2030 targets of the Sustainable Developmental Goals (SDGs).

The commitment on SEAHEARTS brought together measures to reduce risk factors (tobacco control, salt reduction and trans-fatty acids), along with steps to scale up hypertension and diabetes coverage and control in primary health care (PHC). The four milestones agreed upon by the Member States to be achieved by 2025 are:

- 100 million people with hypertension and/or diabetes are placed on protocol-based management.
- One billion people are covered by at least three WHO MPOWER measures for tobacco control.
- One billion people are covered with at least one of the WHO SHAKE package measures for reducing salt intake.
- Two billion people are protected from the harmful effects of trans-fatty acids through best practices or complementary policy measures of WHO REPLACE.

The resolution indicated prioritized catalytic actions for Member States as the path to achieve the four milestones. It mandated WHO to:

- support Member States to develop and prioritize country-specific roadmaps with baselines and targets to accelerate the implementation of SEAHEARTS;
- support Member States to leverage legislative, regulatory and fiscal policies and other measures to reduce risk factors for CVD;
- provide technical support in monitoring and evaluation, and documenting good practices and lessons learnt in implementing SEAHEARTS; and
- set up a platform for Member States to regularly share and exchange best practices.

This report is developed in response to the mandate to WHO and presents the progress on milestones in its first year of implementation till December 2024. Efforts undertaken at country and

regional levels to implement the catalytic actions identified in the SEAHEARTS resolution, along with WHO's support to Member States in fulfilling its mandate, are also presented. The report also serves as an opportunity to advocate for renewed and scaled-up regional SEAHEARTS milestones for 2030 by leveraging the momentum gained for further reduction in incidence and outcomes of CVD in the SE Asia Region.



Data collection and validation

The data on the progress of the SEAHEARTS milestones at the country level are based on the agreed SEAHEARTS Monitoring Framework (3). The Framework comprises operational definitions and metadata for indicators for each of the SEAHEARTS milestones, and enables countries to report on them.

The Framework was developed through two consultations with nominated government officials of the Member States: (1) “Regional workshop on strengthening routine facility data analysis and use”, held in April 2024 in Paro, Bhutan; (2) and “Regional consultation on SEAHEARTS Monitoring Framework and measuring progress on milestones”, held in July 2024 (virtual) (4, 5).

In the present progress report up to December 2024, the SEAHEARTS milestone on the number of people placed on protocol-based management for hypertension and/or diabetes accounted for persons based on unique identifiers, ensuring that individuals are not counted multiple times. The three SEAHEARTS milestones, related to tobacco control, reducing salt intake and protecting people from harmful effects of trans-fatty acids, were attributed to the total population of the country, if the country met the criteria of the operational definition of the indicators based on the SEAHEARTS Monitoring Framework. Country population figures for the year 2023 were used and sourced from the *2024 Revision of the World Population Prospects*, the official United Nations (UN) estimates and projections (6).

A standardized reporting template (Annexure 1) was designed and used to collect data required for the indicators. The reporting template was shared with the nominated officials of the ministries of health on two occasions, with corresponding timelines for reporting set for June 2024 and December 2024.

Where available, data submitted through the template were validated against key global benchmarks, including the WHO Global Tobacco Epidemic Report, WHO Noncommunicable Diseases (NCDs) Country Capacity Survey, Sodium Score Card and Trans-Fatty Acid Score Card of the WHO Global Database on the Implementation of Food and Nutrition Action (7, 8, 9). Follow-up meetings were held with country focal points to clarify any inconsistencies in the data. The verified data for July 2024 and December 2024 have been published on the SEAHEARTS Monitoring Platform (10).

NCD and other relevant focal points at the ministries of health of the countries were consulted to gather inputs on the progress and achievements related to the elements of the WHO technical packages, namely HEARTS (CVD management in primary health care), MPOWER (tobacco control), SHAKE (salt reduction) and REPLACE (elimination of industrial trans-fats) (11). The information gathered are presented as country status reports, each with a feature story showcasing the good practices implemented at the country level to promote cross-learning.

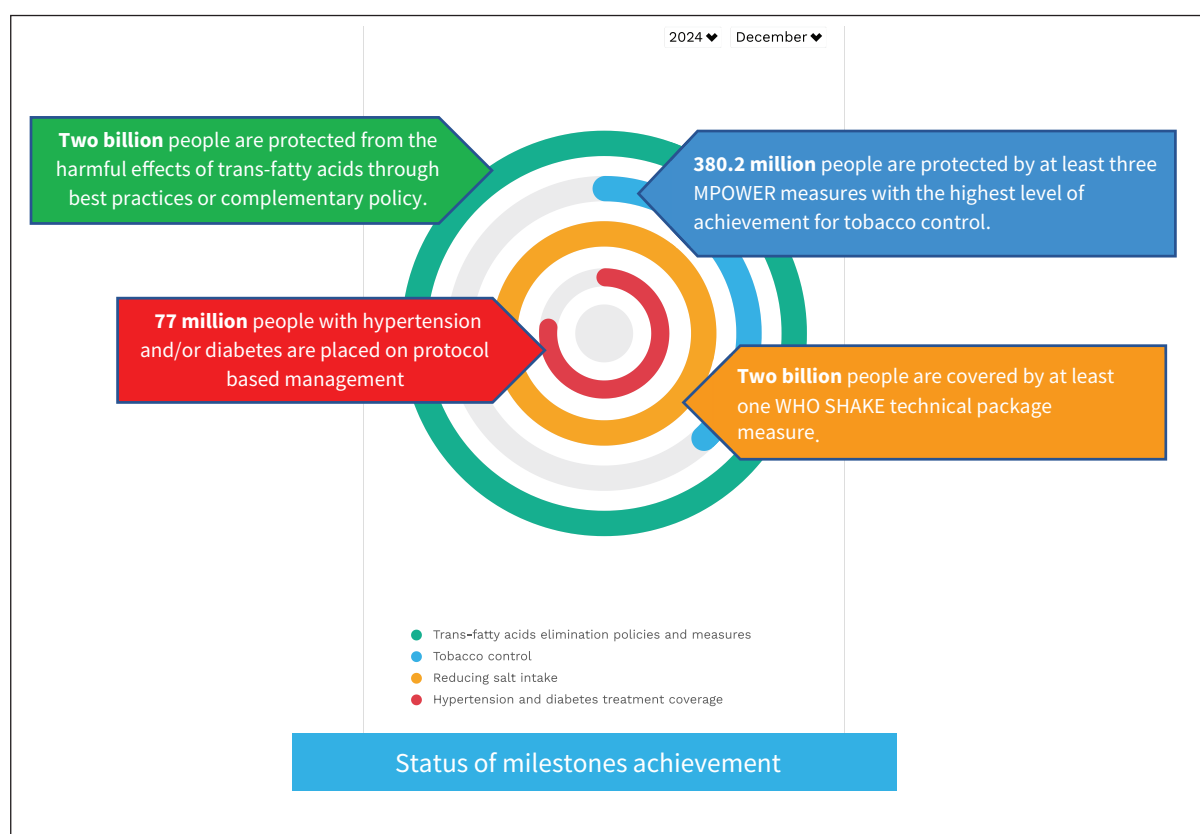
Progress of the SEAHEARTS milestones

The SEAHEARTS resolution prompted countries to initiate several prioritized policy and clinical intervention measures. These include accelerated implementation of WHO technical packages, namely HEARTS (technical package for CVD management in primary health care), MPOWER [measures to reduce tobacco demand under the WHO Framework Convention on Tobacco Control (FCTC)], SHAKE (technical package for salt reduction) and REPLACE (technical package for eliminating industrially produced trans-fatty acids).

The efforts undertaken by the countries helped the SE Asia Region achieve the following for each of the SEAHEARTS milestones by December 2024, within the time frame of 14 months (Fig. 1):

- **77.5 million** people with hypertension and/or diabetes have been enrolled in protocol-based management, against a target of **100 million by 2025**.
- **2.08 billion** people are covered by at least one WHO SHAKE technical package measure, surpassing the 2025 target of **1 billion by 2025**.
- **380.2 million** people are protected by at least three MPOWER measures with the highest level of achievement for tobacco control, against a target of **1 billion by 2025**.
- **2.03 billion** people are protected from the harmful effects of trans-fats through best-practice or complementary policy measures, exceeding the target of **2 billion by 2025**.

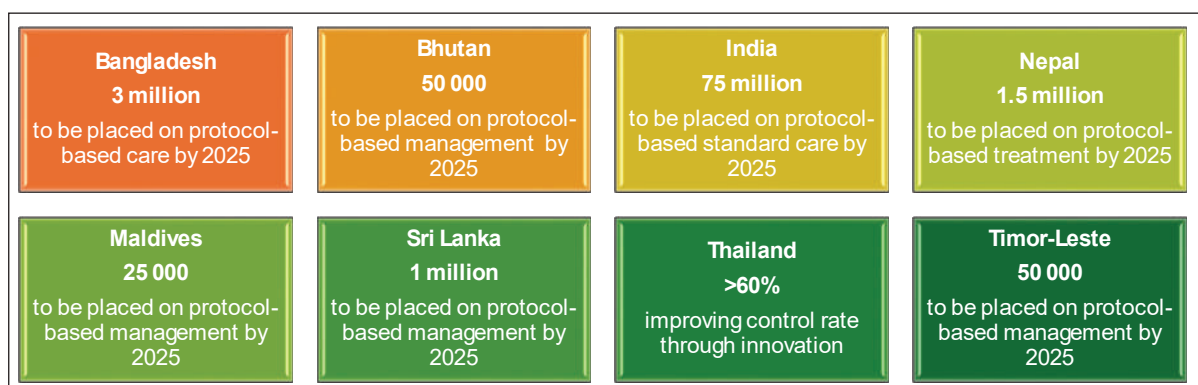
Fig. 1. Status of the achievement of the SEAHEARTS milestones, December 2024



Progress on the milestone on hypertension and diabetes treatment coverage

With the support from WHO, eight Member States have developed the country-level commitments towards the collective target of placing 100 million people with hypertension and/or diabetes on protocol-based management (Fig. 2).

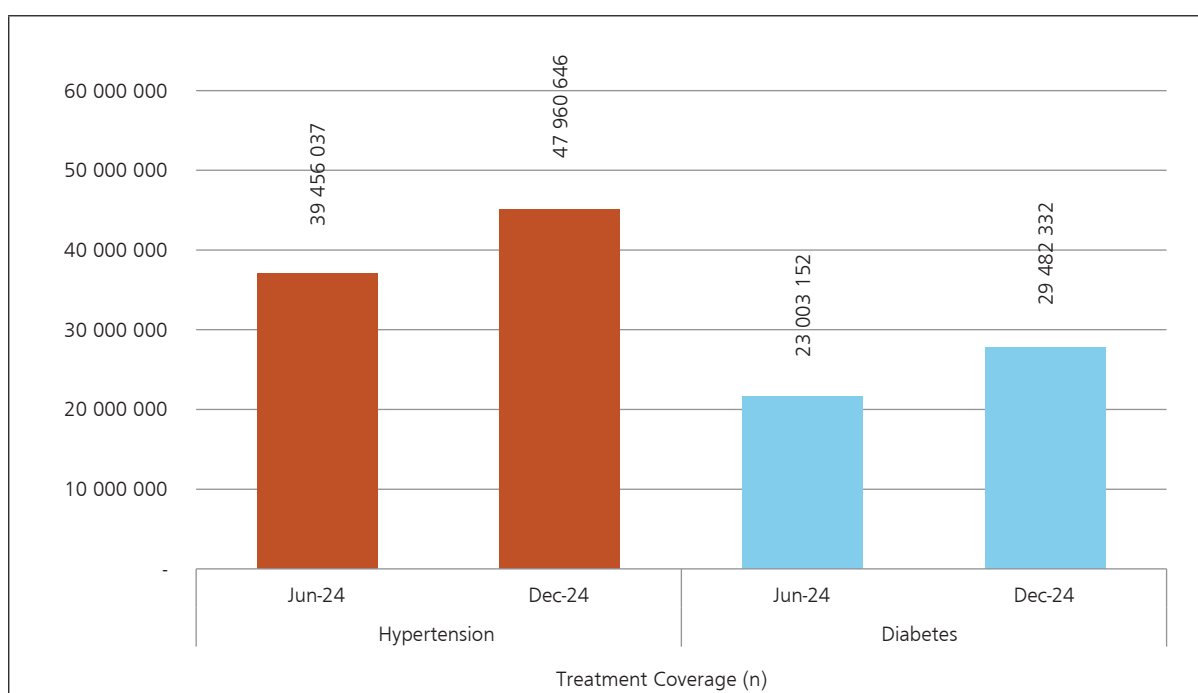
Fig. 2. Country commitments to improve hypertension and diabetes treatment coverage by 2025



The assessment of progress in June 2024 revealed that countries had collectively placed 62.4 million people with hypertension and/or diabetes on protocol-based management, which increased to 77.5 million by December 2024, progressing towards the milestone of 100 million people by December 2025. The relative increase over the period of six months (June 2024–December 2024) exceeds 25% (Fig. 3).

Disaggregating the numbers by the diseases condition, for hypertension, the number of people enrolled in protocol-based management rose from 39.4 million in June 2024 to 48 million by December 2024. For diabetes, the number increased from 23 million to 29.4 million over the same period.

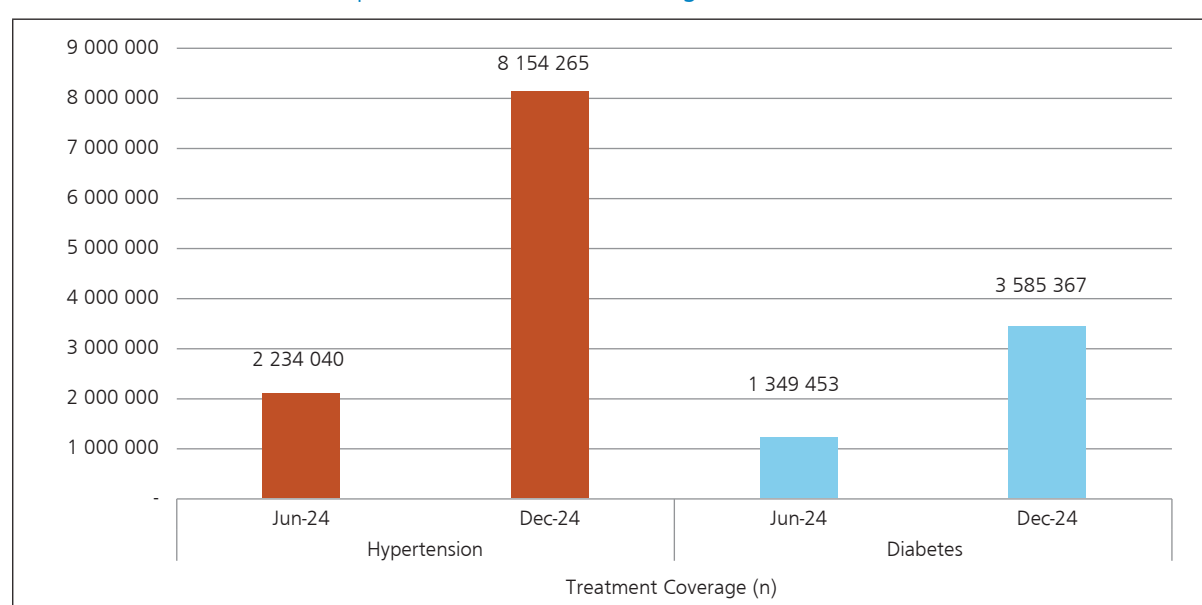
Fig. 3. Status of hypertension and diabetes treatment coverage in the Region, December 2024



The SEAHEARTS Monitoring Framework also includes an additional indicator under the milestone for hypertension and diabetes treatment coverage. This indicator applies to countries that routinely record the control status of persons receiving treatment, enabling them to report on hypertension and diabetes control outcomes. In June 2024, only three countries reported data on controlled status; by December 2024, this number had increased to eight. This growth reflects an improvement in health information systems across the Region.

For hypertension, as of June 2024, 2.23 million people were reported to have achieved blood pressure control. This increased to 8.1 million by December 2024. Similarly, the reported number of people with glycaemic control increased from 1.3 million in June 2024 to 3.6 million by December 2024 (Fig. 4).

Fig. 4. Status of reported numbers of individuals with controlled hypertension and diabetes among those put on treatment in the Region, December 2024



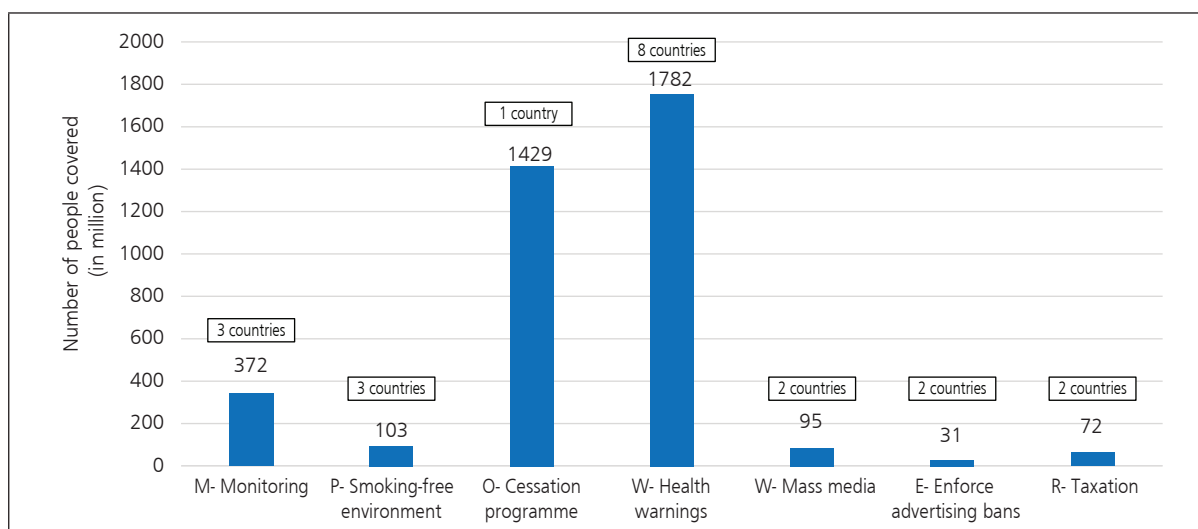
By December 2024, more than 175 000 public primary health care facilities in the Region reported implementing protocol-based management and tracking treatment coverage for hypertension and diabetes. Table 1 provides the details of the implementation of interventions to improve hypertension and diabetes coverage. Detailed progress on the implementation of the WHO HEARTS technical package by each country is documented in the respective country status report.

Progress on the milestone for tobacco control

As of December 2024, 380.2 million people in the SE Asia Region were protected by three MPOWER measures implemented at the highest level of achievement.

The number of people covered by the highest level of implementation of individual MPOWER measures, as of December 2024, is shown in Fig. 5. Health warnings on tobacco product packaging represent the MPOWER measure that has reached the highest level of achievement in the greatest number of countries across the Region (eight countries), collectively covering more than 1.78 billion people.

Fig. 5. Status of implementing individual WHO MPOWER measures at the highest level of achievement in the WHO SE Asia Region, December 2024



The country-wise comparison of implementing each MPOWER measure at the highest level of achievement, as of December 2024, is provided in Table 2.

Progress on the milestone for reducing salt intake

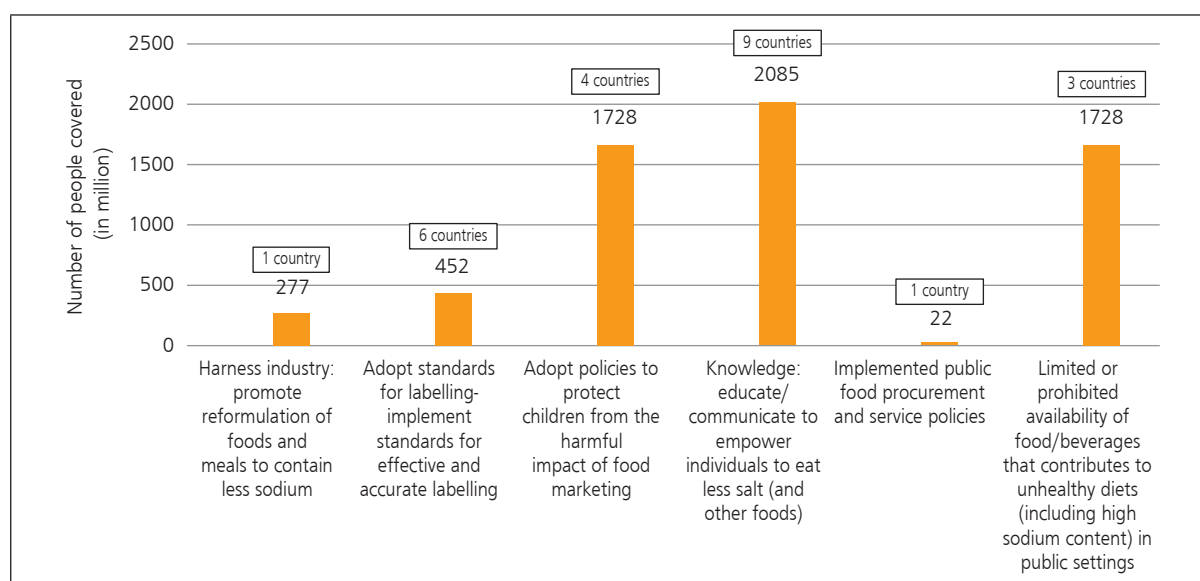
By December 2024, the Region had surpassed the milestone of covering one billion people with at least one of the WHO SHAKE package measures for reducing salt and covered 2.08 billion people with at least one SHAKE measure.

The SEAHEARTS Monitoring Framework also adopted an additional indicator to measure the total population covered by at least one SHAKE policy measure implemented after the adoption of the SEAHEARTS resolution. An analysis of the SHAKE implementation by Member States (Table 3), as of December 2024, shows that five countries (Bangladesh, Democratic People's Republic of Korea, Indonesia,¹ Maldives and Nepal) introduced at least one SHAKE measure after the adoption of the resolution. As a result, post resolution, an additional 503 million people were covered with one or more SHAKE measures through the introduction and implementation of new policies or measures at the country level.

In terms of progress in implementing individual SHAKE measures, as of December 2024 (Fig. 6), eight countries launched behavioural change communication (BCC) or mass media campaigns aimed at reducing sodium intake and empowering individuals to consume less salt, reaching a combined population of more than two billion people. Additionally, four countries have implemented measures to restrict the marketing of foods and non-alcoholic beverages that are high in sodium to children, covering more than 1.7 billion people (Table 3).

¹ Data from Indonesia are included as it was part of the South-East Asia Region at the time this report was commissioned and prepared. Indonesia was reassigned from the WHO SE Asia Region to the Western Pacific Region with effect from June 2025 vide a resolution to that effect promulgated by the Seventy-eighth World Health Assembly.

Fig. 6. Status of implementing WHO SHAKE policies and interventions in the WHO SE Asia Region, December 2024

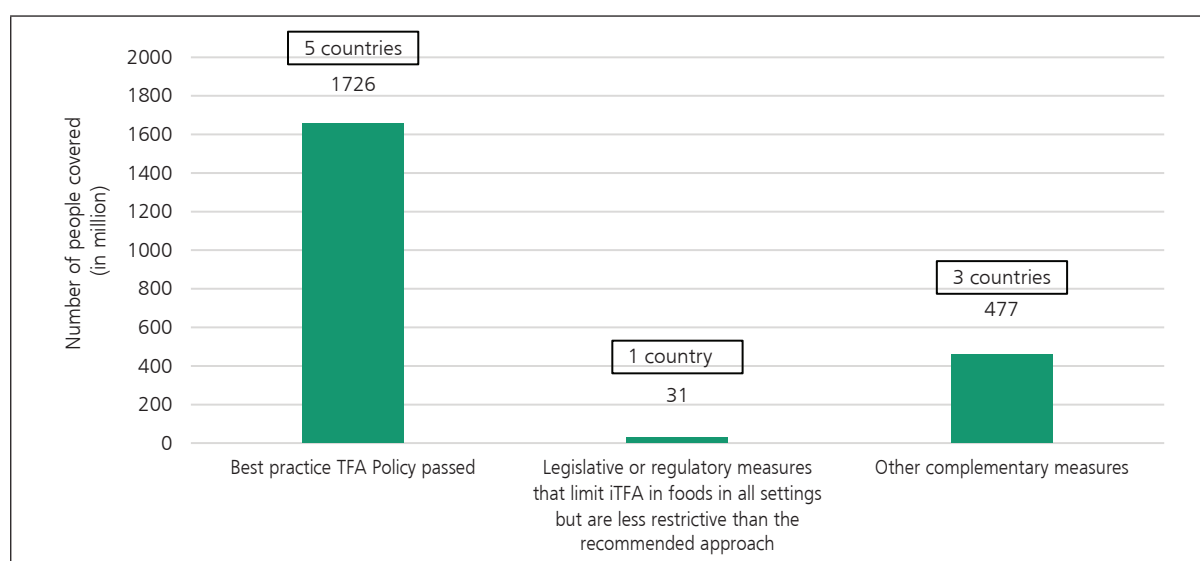


Progress on the milestone for trans-fatty acid elimination policies and measures

By December 2024, the Region had achieved the milestone of protecting more than 2.03 billion people from the harmful effects of trans-fatty acids (TFA) through the implementation of best practices or complementary policy measures.

In terms of progress in implementing best-practice measures of WHO REPLACE, five countries in the Region have enacted legislative or regulatory measures to limit industrially produced TFA in all food settings (Table 4). These best-practice TFA policies are aligned with WHO recommendations and currently protect over 1.72 billion people in the SE Asia Region (Fig. 7).

Fig. 7. Status of implementing TFA elimination policies and measures in the WHO SE Asia Region, December 2024



Post resolution, Nepal and Indonesia introduced measures as best practices or complementary policy measures to protect people from the harmful effects of TFA. In terms of trans-fatty acid elimination, Thailand stands out as the only country to have received the WHO Validation Certificate, recognizing its continued efforts in monitoring and enforcement.

Progress on the SEAHEARTS resolution mandate to WHO

(A) Support Member States to develop and prioritize country-specific roadmaps with baselines and targets to accelerate the implementation of SEAHEARTS

SEAHEARTS roadmaps and country-specific technical briefs to set up national targets:

Country-specific roadmaps with baselines and targets to accelerate the implementation of SEAHEARTS were prepared using the most recent data available on the hypertension and diabetes care cascade. Projected treatment coverage under various scenarios within each country context and the potential impact on achieving SEAHEARTS milestones, modelled based on the South-East Asia Region NCD impact simulation tool, were presented (12). These technical briefs supported countries in setting national targets for protocol-based management of hypertension and/or diabetes for 2025, as shown in Fig. 2.

In-country technical support missions were utilized as opportunities to support national technical teams to develop subnational and facility-level targets and plan the subnational roadmaps to reach national targets. A SEAHEARTS checklist was prepared as a tool to facilitate analysis of the key health systems elements at the primary health care level to implement the WHO HEARTS technical package; it served as the basis for countries to identify the gaps that need to be addressed (5). The sample roadmap of India and subnational targets of Timor-Leste are shown in Figs. 8 and 9.

Fig.8. Roadmap for scaling up hypertension and diabetes coverage in India

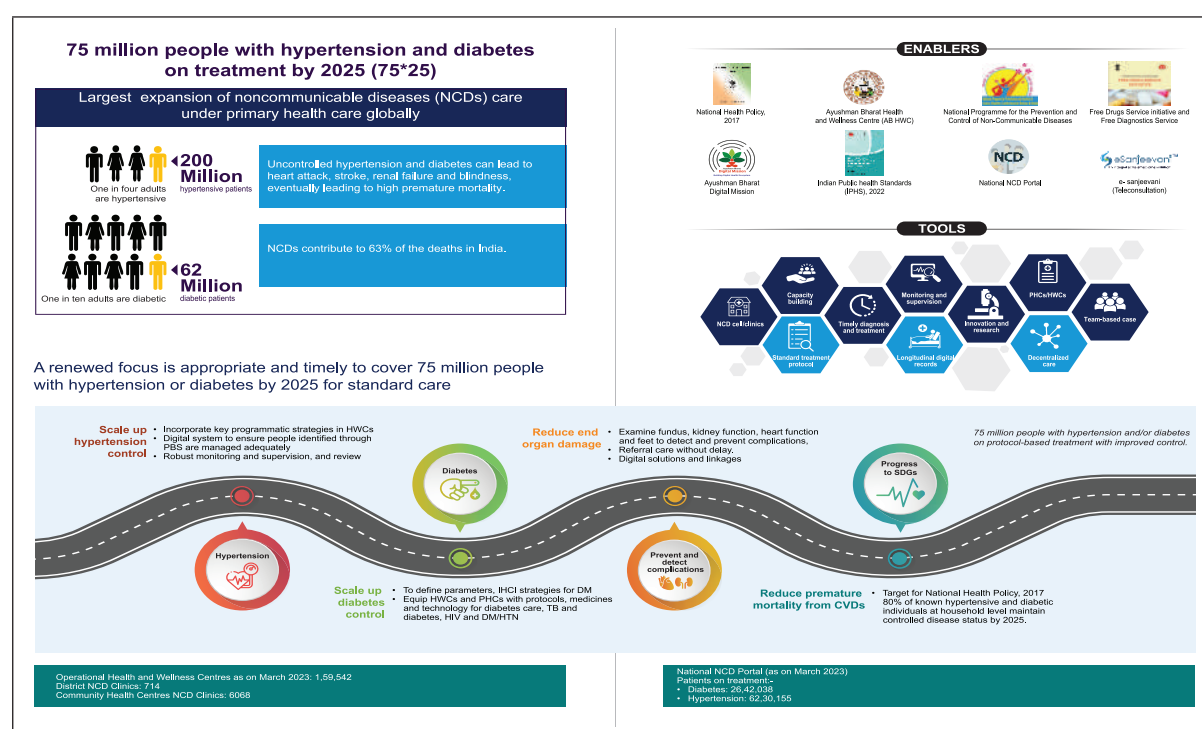


Fig. 9. Subnational (municipality-wise) targets for hypertension and diabetes treatment coverage to reach national target, Timor-Leste

Municipality wise targets										
Municipality	Total population	Population above 30 years (40% of total population)	Likely number of HTN in each municipality (average prevalence 26%)	At Least identify 50% with HTN	Treat at least 75% of the identified HTN patients in each Municipality	Likely number of DM in each municipality (average prevalence 6%)	At Least identify 50% with DM	At least treat 75% of the identified DM patients in each Municipality	Total HTN and DM to be put on treatment	Rounded off target for each Municipality
Aileu	54 631	21 852	5 682	2 841	2 131	1 311	656	492	2 622	2 700
Ainaro	72 989	29 196	7 591	3 795	2 847	1 752	876	657	3 503	3 600
Atauro	10 302	4 121	1 071	536	402	247	124	93	494	500
Baucau	133 881	53 552	13 924	6 962	5 221	3 213	1 607	1 205	6 426	6 500
Bobonaro	106 543	42 617	11 080	5 540	4 155	2 557	1 279	959	5 114	5 200
Covalima	73 909	29 564	7 687	3 843	2 882	1 774	887	665	3 548	3 600
Dili	324 269	129 708	33 724	16 862	12 646	7 782	3 891	2 918	15 565	15 600
Ermera	138 080	55 232	14 360	7 180	5 385	3 314	1 657	1 243	6 628	6 700
Lautem	69 836	27 934	7 263	3 631	2 724	1 676	838	629	3 352	3 400
Liquica	83 689	33 476	8 704	4 352	3 264	2 009	1 004	753	4 017	4 100
Manatuto	50 989	20 396	5 303	2 651	1 989	1 224	612	459	2 447	2 500
Manufahi	60 536	24 214	6 296	3 148	2 361	1 453	726	545	2 906	3 000
Viqueque	80 054	32 022	8 326	4 163	3 122	1 921	961	720	3 843	3 900
ZEEMS (Oecusse)	80 726	32 290	8 396	4 198	3 148	1 937	969	727	3 875	3 900
-	1 340 434	536 174	139 405	69 703	52 277	32 170	16 085	12 064	64 341	65 200

“Colombo Call to Action for strengthening prevention and control of diabetes in the South-East Asia Region” was framed during the event marking the regional commemoration of World Diabetes Day 2024 (13, 14). The Call to Action outlines catalytic actions taken by countries reiterating the commitments of SEAHEARTS milestones to improving diabetes treatment and control. The event also promoted cross-regional learning and emphasized the need for advancing the implementation of the WHO Global Diabetes Compact in the SE Asia Region.

SEAHEARTS session on international forum – “Taking the pressure off two billion people”

Experiences of the SEAHEARTS interventions and good practices across countries in the WHO SE Asia Region were shared at the sixth World Hypertension Congress during a session titled “Taking the pressure off two billion people”, organized by the WHO Regional Office for South-East Asia (WHO-SEARO) and the World Hypertension League (15).

Webinars on SEAHEARTS

A series of webinars was organized to support Member States to accelerate the implementation of SEAHEARTS by exchanging experiences of challenges and solutions related to the prevention and management of CVD in the context of SEAHEARTS. The webinars brought together representatives from ministries of health, academic institutions, civil society organizations and development partners.

These included:

- SEAHEARTS: World’s largest expansion of hypertension coverage through PHC 2024 (May 2024) (16);
- Scaling up capacity for management of acute coronary syndrome and stroke (October 2023) (17); and
- Achieving population-level hypertension control in the WHO South-East Asia Region (May 2025) (18).

(B) Support Member States to leverage legislative, regulatory and fiscal policies, and other measures to reduce CVD risk factors

Together with WHO headquarters (WHO HQ) and country offices, WHO-SEARO has coordinated assistance for legislative and regulatory actions to support healthy diet policies.

On population-level sodium reduction, advocacy and technical support has been provided to five countries through the Global Regulatory Capacity-Building Programme. Regulatory capacity to implement interventions aimed at reducing population-level salt intake has been strengthened among multisectoral government staff and relevant stakeholders in many Member States, leading to the planning or implementation of a range of regulatory policies. In 2023, the South-East Asia regional sodium benchmarks were developed. Currently, two countries are adapting them for food reformulation. WHO has conducted advocacy events and technical workshops on development of legislations and surveillance to eliminate trans-fats from national food supplies. A part of this work has been supported by Resolve to Save Lives.

Countries received support to adopt the “WHO Quit Tobacco” campaign for tobacco cessation across the Region (19). Countries initiating the next rounds of the Global Adult Tobacco Survey (India) and the Global Youth Tobacco Survey (India, Nepal) were supported as well. Bangladesh, India and Indonesia were supported for tobacco tax work to progress towards the highest level of achievement. The passage of the newly adopted Omnibus Health Law in Indonesia in 2024 led to progress in implementing smoke-free policies and an increase in the size of pictorial health warnings. The regulation also mandates improvements in other tobacco control measures, including a ban on TAPS on social media; a prohibition on the sale of single-stick cigarettes and to individuals under 21 years of age; and a ban on cigarette and nicotine product sales near schools and children’s play areas.. The Regional Office published the document, “Two decades of WHO FCTC implementation in SE Asia Region” to record the progress made to implement the treaty.

In 2024, Indonesia enacted the Omnibus Health Law, which introduced and enabled the implementation of key policy measures on food reformulation, front-of-pack labelling (FoPL) and restrictions on food marketing to children. WHO supported the finalization of the food marketing regulation in Thailand, aimed at protecting children from exposure to harmful food marketing. In addition, Nepal received support to enact legislation aimed at eliminating trans-fatty acids from the national food supply.

(C) Provide technical support to the monitoring and evaluation (M&E) and documentation of good practices and lessons learnt while implementing SEAHEARTS.

The SEAHEARTS Monitoring Framework was developed through a series of regional consultations with Member States. The Framework comprises operational definitions and metadata for indicators for each of the SEAHEARTS milestones (3). The hypertension and diabetes indicators are aligned with the WHO NCD Facility-Based Monitoring Framework and key global benchmarks, including the Global Tobacco Epidemic Report, the Sodium Score Card and the Trans Fatty Acid Score Card of the WHO Global Database on the Implementation of Food and Nutrition Action.

Regional consultations on SEAHEARTS milestone progress were organized in July 2024 and in July 2025, with the participation of government-nominated programme managers, responsible for prevention and management of NCDs, as well as focal points for health information management systems to technically support the monitoring and evaluation and documentation of good practices and lessons learnt while implementing SEAHEARTS (5). The objectives of the consultations were twofold: for WHO to update the officials on technical guidance and for the government officials from the Member States to share progress, good practices and challenges to implementing SEAHEARTS interventions.

(D) Set up a platform for Member States to regularly share and exchange best practices on the implementation of SEAHEARTS.

The SEAHEARTS Monitoring Platform was developed by WHO as a digital dashboard designed to track progress on four key regional milestones using indicators outlined in the SEAHEARTS Monitoring Framework (10). It provides biannual progress updates based on the country confirmed data. At present, it includes the progress of June 2024 and December 2024. In addition to progress tracking, the Platform highlights the national targets countries have committed to, case studies, best practices, key interventions and learning resources to support CVD and diabetes prevention and control. It serves as a hub for knowledge-sharing, and documentation of learning and regional collaboration.

Table 1. Status of hypertension and diabetes treatment coverage (as of December 2024)

Country	Number of people placed on protocol-based management		Number of people or percentage of people with controlled status		Number of health facilities or percentage of health facilities providing protocol-based management	Inclusion of protocol medicines in the National Essential Medicines List (NEML)	Provision for medication refill for the duration of 30 days or more	Mechanism to report stockout of NCD medicines in primary health care	Provision to provide statin to people having a 10-cardiovascular risk ≥20 in primary health care	Type of system used to record and track hypertension and diabetes indicators in primary health care facilities	Provision to visualize performance and trends of key hypertension and diabetes indicators in primary health care
	Hypertension	Diabetes	Hypertension	Diabetes							
Bangladesh	447 093	324 183	186 340	56 214	414	Yes	Yes	Yes	Yes	Mixed	Yes
Bhutan	37 489	7723	Not specified	Not specified	187	Yes	Yes	Yes	Yes	Mixed	Yes
DPR Korea	1 422 711	691 350	Not specified	Not specified	Not specified	Not specified	Yes	Yes	Yes	Electronic	Not mentioned
India	37 034 645	22 523 053	3 389 563	2 091 215	165 879	Yes	Yes	Yes	Not specified	Electronic	Yes
Indonesia	1 593 548	2 280 164	Not specified	Not specified	10 416	Yes	Yes	Not specified	Yes	Mixed	Yes
Maldives	2113	1139	1124	278	>75%	Yes	Yes	Yes	Yes	Mixed	Yes
Myanmar	498 677	302 111	Not specified	Not specified	219	Yes	Yes	Yes	Yes	Mixed	Yes
Nepal	29 434	5000	Not specified	Not specified	505	Yes	Yes	Yes	Not specified	Mixed	Yes
Sri Lanka	99 952	56 196	57%	28%	200	Yes	Yes	Yes	Yes	Mixed	Yes
Thailand	7 347 670	3 640 410	4 232 959	1 291 682	100%	Yes	Yes	Yes	Yes	Electronic	Yes
Timor-Leste	31 892	2941	Not specified	Not specified	67	Yes	Yes	Yes	Yes	Paper	Yes

Table 2. Status of WHO MPOWER measure implementation (as of December 2024)

Country	Total population (2023)	Monitor tobacco use and prevention policies.	Protect people from tobacco smoke.	Offer help to quit tobacco use.	Warn about the dangers of tobacco (health warning).	Warn about the dangers of tobacco (mass media).	Enforce bans on tobacco advertising, promotion and sponsorship.	Raise taxes on tobacco.
Bangladesh	172 954 000	No	No	No	Yes	No	No	No
Bhutan	787 000	Yes	No	No	No	No	No	No
DPR Korea	26 161 000	No	No	No	No	Yes	No	No
India	1 428 628 000	No	No	Yes	Yes	No	No	No
Indonesia	277 534 000	Yes	Yes	No	Yes	No	No	Yes
Maldives	521 000	No	No	No	No	No	Yes	No
Myanmar	54 578 000	No	No	No	Yes	No	No	No
Nepal	30 897 000	No	Yes	No	Yes	No	Yes	No
Sri Lanka	21 894 000	No	No	No	Yes	No	No	No
Thailand	71 801 000	Yes	Yes	No	Yes	Yes	No	Yes
Timor-Leste	1 361 000	No	No	No	Yes	No	No	No

Table 3: Status of WHO SHAKE measure implementation (as of December 2024)

Country	Total population (2023)	Harness industry: promote reformulation of foods and meals to contain less sodium.		Adopt standards for labelling – implement standards for effective and accurate labelling.		Adopt policies to protect children from the harmful impact of food marketing.		Knowledge: educate/communicate to empower individuals to eat less salt (and other foods).		Environment: support settings to promote healthy eating.			
		Adopted and set national/subnational targets for maximum sodium content levels in one or more selected food categories, with timelines for meeting the targets (voluntary or legislative approach)	Policy/ intervention introduced after October 2023	Implemented voluntary or mandatory front-of-pack labelling on pre-packaged commercially manufactured foods	Policy/ intervention introduced after October 2023	Implemented policies to protect children from the harmful impact of food marketing on diet	Policy/ intervention introduced after October 2023	Implemented a BCC/mass media campaign for healthy diets at national or subnational level that includes salt reduction within past three years	Policy/ intervention introduced after October 2023	Implemented public food procurement and service policies; these could include any of the following: restricted foods contributing to unhealthy diets, including high salt, sugar or fat through cafeteria policies (schools, government institutions)	Policy/ intervention introduced after October 2023	Limited or prohibited availability of food/beverages that contributes to unhealthy diets (including high sodium content) in public settings	Policy/ intervention introduced after October 2023
Bangladesh	172 954 000	No	No	No	No	No	No	Yes	Yes	No	No	No	No
Bhutan	787 000	No	No	No	No	No	No	No	No	No	No	No	No
DPR Korea	26 161 000	No	Yes	Yes	Yes	No	No	Yes	Yes	No	No	No	No
India	1 428 628 000	No	No	No	No	Yes	No	Yes	No	No	No	Yes	No
Indonesia	277 534 000	Yes	Yes	No	No	Yes	Yes	Yes	No	No	No	Yes	Yes
Maldives	521 000	No	Yes	No	No	Yes	Yes	Yes	Yes	No	No	No	No
Myanmar	54 578 000	No	Yes	No	No	No	No	Yes	Yes	No	No	No	No
Nepal	30 897 000	No	No	Yes	Yes	No	No	Yes	No	No	No	No	No
Sri Lanka	21 894 000	No	Yes	No	No	Yes	No	Yes	No	Yes	No	Yes	No
Thailand	71 801 000	No	Yes	No	No	No	No	Yes	No	No	No	No	No
Timor-Leste	1 361 000	No	No	No	No	No	No	No	No	No	No	No	No

Table 4: Status of WHO REPLACE measure implementation (as of December 2024)

Country	Total population (2023)	Best-practice trans-fatty acid (TFA) policy passed			Other complementary measures effected legislative or other measures that encourage consumers to make healthier choices about industrially produced TFA		
		Mandatory national limit of 2 g of industrially produced TFA per 100 g of total fat in all foods OR	Mandatory national ban on the production or use of PHO as an ingredient in all foods OR	Less restrictive measures: legislative or regulatory measures that limit iTFA in foods in all settings, but are less restrictive than the recommended approach (e.g. 2% limit for iTFA in oils and fats only)	Mandatory declaration of TFA on nutrition labels OR	Front-of-pack labelling system that includes TFA OR	Reformulation or mandatory limits on industrially produced TFA in foods in specific settings (e.g. public institutions) or specific products (e.g. complementary foods).
Bangladesh	172 954 000	Yes			Yes		
Bhutan	787 000						
DPR Korea	26 161 000					Yes	
India	1 428 628 000	Yes					
Indonesia	277 534 000						Yes
Maldives	521 000						
Myanmar	54 578 000						
Nepal	30 897 000	Yes		Yes			
Sri Lanka	21 894 000	Yes					
Thailand	71 801 000		Yes				
Timor-Leste	1 361 000						

Planning for SEAHEARTS milestones 2030

Given the significant progress on three of the four milestones and the continued implementation of the SEAHEARTS roadmaps at subnational and national levels, the Region is likely to remain on track to reach the SEAHEARTS milestones 2025. This progress report will serve as an impetus for Member States to refine their plans and navigate the challenges, aligning with the best practices across the countries to further accelerate the responses for collective success.

The SEAHEARTS initiative was conceptualized in 2023 by WHO in response to the high burden of CVD as the leading cause of mortality and morbidity in the WHO SE Asia Region. The Global Health Estimates and their projections indicate that the CVD burden is likely to continue rising globally and in the SE Asia Region, driven by population growth and population ageing. This underscores the need for urgent, concerted and sustained action to change the trajectory. Against this background, and building on the momentum and anticipated success of collective efforts to accelerate CVD prevention and control, it is prudent for the Region to plan to extend and expand the SEAHEARTS milestones to be achieved by 2030.

Discussions with nominated government officials from Member States during the regional consultation held in July 2025 led to a consensus on a set of extended and expanded SEAHEARTS 2030 milestones to accelerate the prevention and control of CVD in the SE Asia Region.

WHO will continue to provide technical guidance and support for the implementation of WHO technical packages to reach the SEAHEARTS 2030 milestones, and to monitor and report the progress.

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Country status reports on the SEAHEARTS initiative implementation



BANGLADESH

NCD prevention and management



Bangladesh has made significant strides in strengthening noncommunicable disease (NCD) management within its primary health care system. The 2018 Multisectoral Action Plan for prevention and control of noncommunicable diseases (MSAPNCD 2018) emphasized a comprehensive, multisectoral approach involving various ministries, departments and private sector stakeholders to improve NCD prevention and control.¹ A key focus of the Plan was on restructuring primary health care facilities to integrate NCD services, ensuring early screening, timely diagnosis and an efficient referral system to higher-level health-care facilities.

The government has actively promoted public awareness of NCD prevention through national campaigns and street advertisements to spread key health messages, particularly on healthy diets and lifestyle modifications. Bangladesh's health-care system operates through a tiered structure, with primary health care services provided via an extensive network of community clinics (CCs), union health and family welfare centres (UHFwCs), and *upazila* health complexes (UHCs). The country has established 14 336 community clinics, 3161 UHFwCs and 492 UHCs, ensuring widespread access to essential health-care services. A significant innovation in NCD management is the introduction of "NCD corners" in UHCs. These dedicated units provide person-centred care for hypertension, diabetes, cardiovascular diseases and other NCDs.²

The integration of NCD services is supported by clinical guidelines emphasizing both pharmacological and non-pharmacological interventions. The WHO package of essential noncommunicable (PEN) disease interventions for primary health care protocol is implemented in all "NCD corners" to standardize care. Digitalization of NCD services is also underway, with 230 of the 492 UHCs now equipped with digital systems for improved patient management, data tracking, and follow-up.

National commitment: The Ministry of Health and Family Welfare of Bangladesh announced in June 2023 that the country was committed to making all efforts to place three million people with hypertension and/or diabetes on protocol-based care by 2025.³



Screening for early detection of hypertension

Healthy-lifestyle counselling



- National management protocols for hypertension and diabetes include counselling patients on tobacco use, diet and physical activity, using the 5As and 5Rs brief intervention approach.

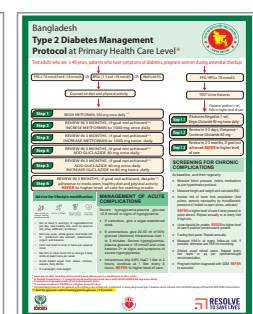
Evidence-based treatment protocols



- Evidence-based drug- and dose-specific national protocols for management of hypertension and diabetes in primary health care, aligned with WHO HEARTS and WHO PEN technical packages, are being implemented since 2018.⁴
- More than 75% of the primary health care facilities (UHCs) in the public sector are estimated to be implementing protocol-based treatment for hypertension and diabetes.⁵
- According to the protocol, there is a provision for medication refill for 30 days for anti-hypertensives and 30 days for anti-diabetes medications.



Hypertension management protocol



Diabetes management protocol

Access to medicines and technology



Medicines[#]

- The Bangladesh National Formulary 2019 includes medicines (amlodipine, losartan, hydrochlorothiazide, metformin and gliclazide) that are part of the hypertension and diabetes management protocol used at the primary health care level.⁶
- Medicines for management of hypertension and diabetes are estimated to be “generally available” (available at 50% or more pharmacies) at the primary care facilities of the public health sector.⁵
- An electronic Logistics Management Information System (LMIS) is in place to support procurement, distribution and monitoring of medicines across health facilities.
- Medicines are procured annually or biannually through a mix of centralized and decentralized mechanisms by Essential Drugs Company Ltd., the Central Medical Stores Depot and the Directorate General of Health Services.⁷
- According to demand, over the past one year, more than 75% of primary health care facilities in the public sector reported stockout of hypertension and diabetes medicines that are listed in the national management protocol.

Technology^{##}

- Equipment for measuring blood glucose and blood pressure, and urine strips for ketone and glucose are estimated to be “generally available” at primary health care facilities of the public and private health sectors.⁵
- Testing for HbA1c and urine strips for albumin assay are reported to be “generally not available” at the primary health care facilities of the public and private health sectors.

Risk-based CVD management



- Cardiovascular risk stratification is included in the nationally approved protocol for hypertension and diabetes management.
- Less than 25% of primary care (“NCD corners” in UHCs) facilities provide non-laboratory-based risk stratification.⁵
- There is a national protocol on “Management of diabetes and hypertension in primary health care settings” to provide statin to people having a 10-cardiovascular risk ≥ 20 .

[#] “Generally available” is defined as “at 50% or more pharmacies”.

^{##} “Generally available” is defined as “at 50% or more health-care facilities”.

Team-based care



- ▶ Screening for hypertension and diabetes is conducted by community health workers at the community clinics (CCs) only. CCs have no doctors. Community health care providers (CHCPs) generally preserve paper-based information and provide basic health services. Digital technology was recently launched at some CCs. CCs are implementing monthly refill schemes for NCD medications to improve access to medication and ensure continuity of treatment, especially in rural areas where access to health care is often limited.
- ▶ Primary health care centres (PHCCs) or *upazila* health complexes provide one-stop services for NCDs, including services, diagnosis, treatment and monitoring of patients and appropriate referrals. One medical officer, one nurse and one sub-assistant community medical officer (SACMO) usually provide services from “NCD corners”. Patients receive drugs from the “NCD corners” of the UHCs.
- ▶ Health personnel in PHCCs/UHCs and community health workers function as a team to conduct screening, detection and management of hypertension and diabetes.
- ▶ Treatment initiation is carried out by a medical doctor while a nurse and pharmacist facilitate drug refills.
- ▶ 75% of health-care providers were trained in protocol-based hypertension and diabetes care in 2024.
- ▶ Provision is usually not available for health-care providers at primary health facilities to consult specialists on specific or complicated cases of hypertension and diabetes virtually. Routine teleconsultation services between community clinics and higher-level facilities are usually not available; however, few facilities have these provisions, but there is no structured mechanism.

Systems for monitoring



- ▶ Currently, a mixed system of paper-based and electronic options is utilized to collect and report NCD programme indicators at primary health care facilities.⁵
- ▶ 15 districts (out of a total of 28 districts) and 182 subdistricts (out of a total of 495 subdistricts) have digital data recording systems to identify/track hypertension and diabetes patients using unique identifiers at public health facilities.⁸
- ▶ Simple App is currently being used to monitor the situation.⁹
- ▶ More than 75% of primary health care facilities use dashboards to visualize performance and trends of key hypertension and diabetes indicators (DGHS data).
- ▶ Country has District Health Information System 2 (DHIS2) to facilitate reporting of aggregated patient data on NCDs from primary health facilities to district/subnational and national levels. The digital MIS for NCD collects data from “NCD corners” and report to DHIS.
- ▶ There is a mechanism/provision for indicators of hypertension and diabetes treatment coverage data made available on a regular basis in the public domain.
- ▶ The majority of the “NCD corners” use the Simple App for the digital surveillance system, which is completely integrated into DHIS2.

Progress of hypertension and diabetes coverage and control

The country has progressed in scaling up services for coverage and control of hypertension and diabetes management

414 health facilities providing protocol-based management for hypertension and diabetes

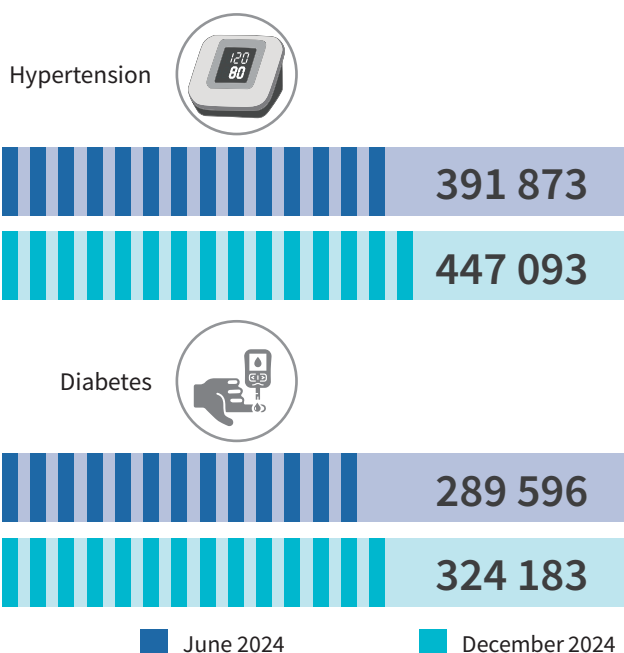


Figure 1: Number of persons with hypertension and/or diabetes placed on protocol-based management in health care facilities/clinics (2024)

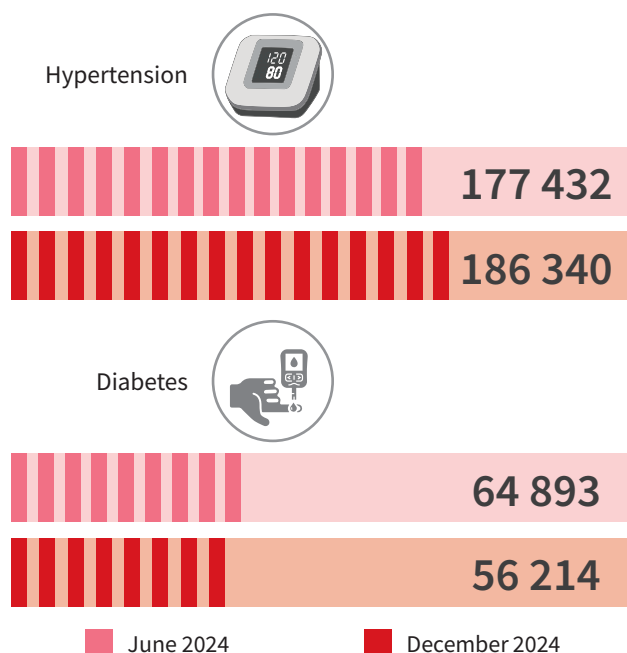


Figure 2: Hypertension and diabetes control status among people on treatment in health care facilities/clinics (2024)

MPOWER		Coverage	Achievements
M onitor tobacco use and prevention policies		Minimal measure achieved	- Regularly monitored the use of tobacco products among adults. - Achieved the highest level of implementation in M in GTCR 2019 and GTCR 2021. - Tobacco Questions for Surveys (TQS) are integrated in national health surveys or multiple risk factor surveys.
P rotect people from tobacco smoke		Minimal measure achieved	- Three to five public places completely smoke-free. - WHO Bangladesh has collaborated closely with the Bangladesh Medical Students' Society (BMSS) and the National Tobacco Control Cell (NTCC) of the Ministry of Health and Family Welfare (MoHFW) on tobacco-free medical and dental colleges through promoting the enforcement of tobacco-free policies within the campus.
O ffer help to quit tobacco use		Minimal measure achieved	WHO Bangladesh has collaborated closely with the NTCC on offering tobacco cessation services through supporting initiatives aimed at assisting tobacco users in quitting through cessation programmes and accessible support services.
W arning about dangers of tobacco	Health warnings on Cigarette Packages	Complete measure achieved	Pictorial health warnings covering 50% of the package area are mandated by law
	Anti-tobacco Mass Media Campaigns	Moderate measure achieved	Achieved best-practice in mass media campaigns in GTCR 2013, 2015 and 2017 by conducting regular national-level anti-tobacco campaigns with at least 7 appropriate characteristics, including airing on television and/or radio.
E nforce bans on tobacco advertising, promotion and sponsorship		Moderate measure achieved	There is a comprehensive ban on several forms of TAPS.
R aise taxes on tobacco		Moderate measure achieved	The share of total taxes in the retail price of the most widely sold brand of cigarettes in the country is 73.1%.





REPLACE
TRANS FAT

IMPLEMENTATION OF WHO REPLACE TECHNICAL PACKAGE¹⁰



Best-practice TFA policy:
Legislative or regulatory measures that limit industrially produced TFA in foods in all settings and are in line with the recommended approach

Achieved
(mandatory national limit of 2 g of industrially produced TFA per 100 g of total fat in all foods)



Monitoring mechanism for mandatory TFA limits

Not achieved



Other complementary measures:
Legislative or other measures that encourage consumers to make healthier choices about industrially produced TFA in foods in specific settings

In progress



IMPLEMENTATION OF WHO SHAKE TECHNICAL PACKAGE FOR REDUCING SALT INTAKE

Technical package element	Implementation Status		
Food reformulation policies	Policies that set scope and criteria for maximum limits or targets for the sodium content in prepackaged food		
Food and nutrition labelling policies	Policies that set scope and criteria to:	List and declare sodium on packaged food	In progress
		Regulate the use of sodium-related claims on packaged food	In progress
		Use front-of-pack labelling (FOPL) to provide interpretive information about sodium content	In progress
Food marketing restriction policies	Policies that set scope and criteria for restricting food marketing to protect children from its harmful impact on diet, including consumption of high-sodium food		
Behavior change communication and mass media campaigns for healthy diets	A set of communication activities designed to change behaviors to reduce consumption of salt and high sodium food		
Food procurement and service policies	Policies that set scope and criteria for sodium content in food served or sold in public settings and/or procured by government for food programmes		
Taxation on unhealthy food	Policies that set scope and criteria for taxes on unhealthy food, including high-sodium food		
			In place (SSB taxation, taxation on sugary foods)

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Prevention of diabetes and related cardiometabolic parameters through faith-based interventions¹⁻⁵

In Bangladesh, type 2 diabetes mellitus (T2DM) has emerged as a major public health challenge, driven by sedentary lifestyles, unhealthy diets and limited physical activity. With nearly 89% of the population identifying themselves as Muslim, religious leaders hold significant influence within communities, often serving as trusted voices on both spiritual and social matters. Recognizing this unique societal structure, the Diabetic Association of Bangladesh (BADAS) initiated an innovative programme titled “Diabetes Prevention through Religious Leaders”, a faith-based intervention designed to prevent diabetes and reduce related cardiometabolic risks through the power of community and spiritual engagement.

This initiative effectively harnessed the influence of religious leaders to spread awareness and encourage healthier lifestyles. Religious leaders, trained to deliver health messages during prayers, religious gatherings and counselling sessions, educate the community on diabetes prevention, healthy eating, exercise and stress management, and advocate, promote and encourage a healthy lifestyle through religious sermons as well as running “diabetes corners”. In fact, the establishment of the “diabetes corners”, which provided screening services for diabetes, hypertension and nutritional status, was a cornerstone of the intervention. These centres utilized digital tools, including a mobile app, to collect sociodemographic, clinical, biochemical and financial data, which were integrated with the Nationwide Diabetes Registry of BADAS. The “corners” were also linked to government-run upazila “NCD corners” and BADAS hospitals for referrals and follow-up care.



Faith-based health session: Community members engage in a discussion on preventing diabetes and improving cardiometabolic health

To expand outreach, BADAS partnered with local mosques to organize workshops and health camps in both rural villages and urban slums, ensuring accessibility for underserved populations. Educational materials were distributed widely and religious leaders worked hand in hand with health-care professionals to develop culturally sensitive health promotion messages tailored to local beliefs and customs.

Over 100 religious leaders were trained in diabetes and cardiometabolic health promotion. More than 50 000 individuals received screening and educational services through “diabetes corners”, while around five million people received information on healthy lifestyle. Moreover, 300 000 mosques became active platforms for disseminating prevention messages, creating a network that connected with 20 million people across the country.

Key outcomes included a 25% reduction in new cases of diabetes and hypertension in targeted areas. Faith leaders emerged as powerful advocates for health, enhancing trust in medical advice and promoting long-term behavioural change. The intervention proved sustainable – many communities continued promoting health messages even after the formal programme had ended.

This initiative was made possible through strong collaboration between the Bangladesh Ministry of Health and Family Welfare, the Ministry of Religious Affairs, local religious institutions and NGOs.

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BHUTAN

NCD prevention and management



Bhutan was the first country in the WHO SE Asia Region to adopt the WHO PEN package for primary health care in low-resource settings in 2009. The country scaled up the implementation nationwide in 2013–2014. Bhutan adopted the people-centred approach, titled “Service with Care and Compassion Initiative (SCCI)”, to further address gaps and improve NCD services. The approaches employed by this model are known as “7Rs” and “3Cs”. “7Rs” stand for: robust team building, reaching out to homebound services, refill of medicines, recall and reminders, responsive referrals, reliable and people-centred laboratory diagnosis, and real-time monitoring and supportive supervision. “3Cs” stand for: comprehensive, collaborative and continuum of care.¹

In May 2023, Bhutan embraced the SEAHEARTS initiative by leveraging achievements and learnings of the SCCI, and scaled up hypertension and diabetes services according to WHO HEARTS technical packages. The country conducted a nationwide screening of NCDs (14 November–7 December 2024), coinciding with World Diabetes Day during which people were informed about the preventive measures for NCDs and their risk factors.²

National commitment: The Ministry of Health, Royal Government of Bhutan, announced its commitment in May 2023 to place at least 50 000 Bhutanese on hypertension and diabetes protocol-based management by 2025.³



© LSRDP, Ministry of Health, Bhutan

An elderly screened for blood pressure during NCD outreach screening program

Healthy-lifestyle counselling

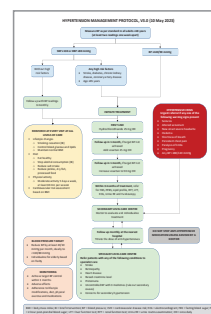


- ▶ National management protocols for hypertension and diabetes include counselling patients on tobacco use, diet, alcohol consumption and physical activity, using the 5As brief intervention approach.

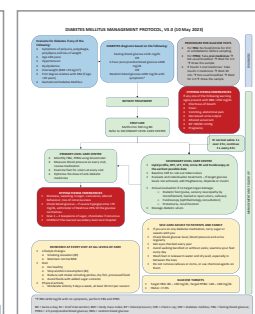
Evidence-based treatment protocols



- ▶ Evidence-based drug- and dose-specific national protocols for management of hypertension and diabetes in primary health care, aligned with WHO HEARTS and PEN technical packages, are being implemented since 2023.^{4,5}
- ▶ All primary health care facilities (187) in the country are implementing protocol-based treatment for hypertension and diabetes.⁶
- ▶ The protocol allows 1–2-month medication refill depending on the disease control status.
- ▶ Private sector contribution to control and management of NCDs is minimal in the country.



Hypertension management protocol



Diabetes management protocol

Access to medicines and technology



Medicines[#]

- ▶ As part of the SCCI, all persons (aged 40 years and above) visiting health facilities are screened for common NCDs. Home care is provided to home-bound patients. All health facilities are responsible for medicine refills, treatment and medication reminders, follow-ups, referrals, real-time monitoring and outreach to underserved populations. The National Essential Medicines List 2023 includes medicines (hydrochlorothiazide, losartan, metformin and insulin) that are part of the hypertension and diabetes management protocol used at the primary health care level.⁷
- ▶ According to WHO NCD CCS 2023, these medicines are estimated to be “generally available” (available at 50% or more pharmacies) at the primary care facilities of the public health sector.⁸
- ▶ The electronic-Bhutan Medical Supplies Inventory System (e-BMSIS) is used for forecasting, procurement, distribution and monitoring of medicines at the health facilities.⁹
- ▶ Medicines are centrally procured annually by the Department of Medical Product.
- ▶ Provision is in place to request (indent) medicines at the PHC level for specific individuals, if prescribed by a specialist from a higher-level facility, using Form III.
- ▶ No public primary health facilities reported stockout of hypertension and diabetes medicines from the national protocol in 2024.

Technology^{##}

- ▶ Equipment for measuring blood glucose and blood pressure are estimated to be “generally available” at primary care facilities of both public and private health sectors, whereas urine strips for glucose and ketone are “generally available” at the primary care facilities of the public health sector only.⁸
- ▶ Equipment for dilated fundus examination, ECG and foot vibration perception by tuning fork are reported to be “generally not available” across public and private primary health care facilities.
- ▶ Testing for HbA1c, total cholesterol, HDL, LDL, serum creatinine and blood urea and urine albumin assay is reported to be “generally not available” across public and private primary health care facilities.

[#] “Generally available” is defined as “at 50% or more pharmacies”.

^{##} “Generally available” is defined as “at 50% or more health-care facilities”.

Risk-based CVD management



- ▶ Cardiovascular risk stratification is included in the nationally approved protocol for hypertension and diabetes management.
- ▶ Over 50% of primary care facilities provide non-laboratory-based risk stratification.
- ▶ Statins are recommended per national protocol to individuals with a 10-year cardiovascular risk of $\geq 20\%$.

Team-based care



- ▶ At primary health care facilities, health assistants and NCD focal persons work as a team for screening and management of hypertension and diabetes. Doctors initiate treatment while health assistants handle drug refills and follow-ups, including referrals.
- ▶ Since 2020, all primary health care providers have been trained in protocol-based hypertension and diabetes care, with most staff members, including pharmacists, laboratory personnel and nurses, trained in the SCCI model as well.¹⁰
- ▶ Primary health providers can consult specialists on individual cases, but routine teleconsultation services are unavailable. Consultations with higher facilities occur, as needed, via text messaging platforms, such as WhatsApp and Telegram.

Systems for monitoring



- ▶ The national system (NCD dashboard) is being designed and rolled out in a phased manner, and a mixed system is being used in the interim period.
- ▶ A standardized system for recording patient-level data that include NCD status and risk factors is estimated to be existing at more than 75% of primary health care facilities.¹¹ This will be further standardized with integration of the monitoring into the Electronic Patient Information System (ePIS).
- ▶ The patient information system at primary health care facilities is capable of identifying and tracking hypertension and diabetes patients, using a unique identifier during each visit.
- ▶ All 187 primary health care facilities use dashboards to visualize performance and trends of key hypertension and diabetes indicators.
- ▶ Currently, the NCD dashboard is being used to record the NCD data across health facilities. The dashboard will be integrated into the ePIS. It will help in providing aggregated and real-time disaggregated data.

Progress of hypertension and diabetes coverage and control

The country has progressed in scaling up services for coverage and control of hypertension and diabetes management.

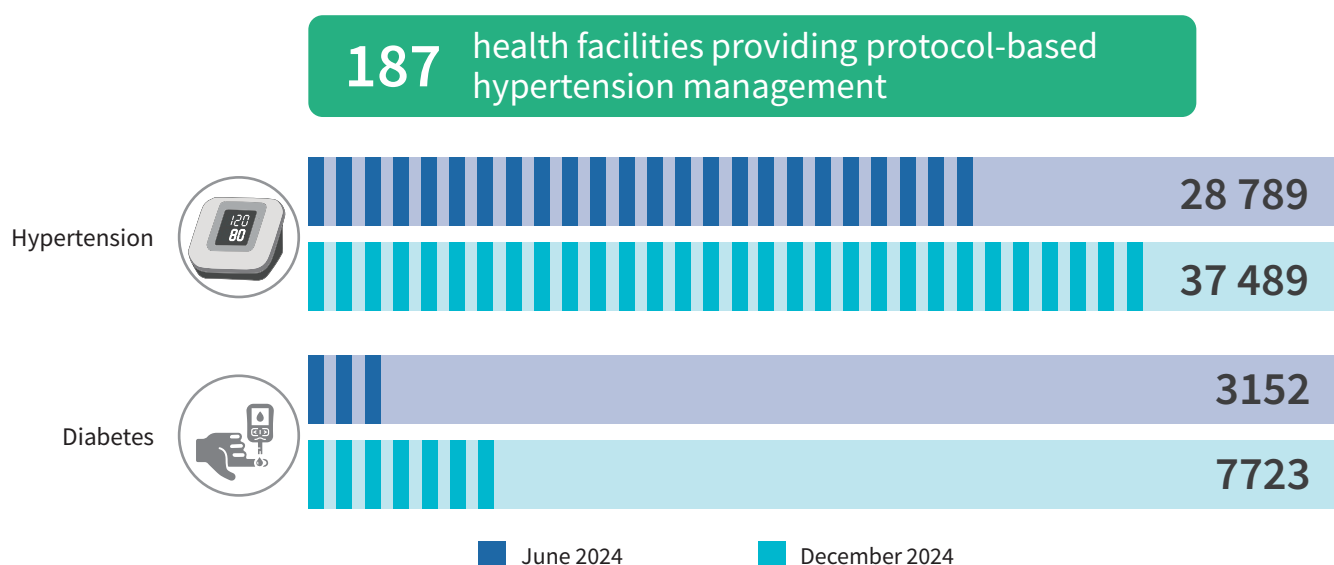


Figure 1: Number of persons with hypertension and/or diabetes placed on protocol-based management in health care facilities/clinics (2024)

MPOWER	Coverage	Achievements
M onitor tobacco use and prevention policies	 Complete measure achieved	- Achieved the highest level of implementation in M in GTCR 2023 and 2025. - The Tobacco Questions for Surveys (TQS) are integrated in national health surveys.
P rotect people from tobacco smoke	 No measure or weak measure achieved	Achieved the highest level of implementation in P in GTCR 2015.
O ffer help to quit tobacco use	 Moderate measure achieved	Smoking cessation support available in most health clinics across the country.
W arning about dangers of tobacco	Health warnings on Cigarette Packages  No measure or weak measure achieved	Bhutan has recently shown commitment to formulate and enforce necessary legislation to implement large and effective graphic health warnings on tobacco products.
	Anti-tobacco Mass Media Campaigns  Moderate measure achieved	Achieved best-practice in mass media campaigns in GTCR 2015.
E nforce bans on tobacco advertising, promotion and sponsorship	 Moderate measure achieved	Comprehensive ban on several forms of Tobacco advertising, promotion and sponsorship (TAPS).
R aise taxes on tobacco	 Weak measure achieved	The share of total taxes in the retail price of the most widely sold brand of cigarettes in the country is 23.6%.



REPLACE TRANS FAT IMPLEMENTATION OF WHO REPLACE TECHNICAL PACKAGE¹²

	A national policy commitment to reduce industrially produced TFA in the food supply	Achieved		Best-practice TFA policy: Legislative or regulatory measures that limit industrially produced TFA in foods in all settings, and are in line with the recommended approach	Not Achieved
	Less restrictive TFA limits: Legislative or regulatory measures that limit industrially produced TFA in foods in all settings, but are less restrictive than the recommended approach	Not Achieved		Other complementary measures: Legislative or other measures that encourage consumers to make healthier choices about industrially produced TFA or mandatory limits on industrially produced TFA in foods in specific settings	Achieved

IMPLEMENTATION OF WHO SHAKE TECHNICAL PACKAGE FOR REDUCING SALT INTAKE

Technical package element		Implementation Status
Food reformulation policies	Policies that set scope and criteria for maximum limits or targets for the sodium content in prepackaged food	Not achieved
Food and nutrition labelling policies	Policies that set scope and criteria to:	Not achieved
	List and declare sodium on packaged food	Not achieved
	Regulate the use of sodium-related claims on packaged food	Not achieved
	Use front-of-pack labelling (FOPL) to provide interpretive information about sodium content	Not achieved
Food marketing restriction policies	Policies that set scope and criteria for restricting food marketing to protect children from its harmful impact on diet, including consumption of high-sodium food	Not achieved
Behavior change communication and mass media campaigns for healthy diets	A set of communication activities designed to change behaviors to reduce consumption of salt and high sodium food. ¹³	Achieved
Food procurement and service policies	Policies that set scope and criteria for sodium content in food served or sold in public settings and/or procured by government for food programmes	Not achieved
Taxation on unhealthy food	Policies that set scope and criteria for taxes on unhealthy food, including high-sodium food	Not achieved

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Fighting NCDs with HEARTS in Bhutan: A service with care and compassion¹⁻³

NCDs pose a major public health challenge in Bhutan, accounting for over 70% of total deaths in the country. The National Health Survey 2023 revealed alarming trends in key NCD risk factors, such as tobacco use, harmful alcohol consumption and physical inactivity. In response to this rising burden, Bhutan's 13th Five-Year Health Sector Plan (2024–2029) has placed a strong emphasis on the prevention and control of NCDs.

To address gaps in NCD service delivery, Bhutan launched the Service with Care and Compassion Initiative (SCCI). This flagship initiative aims to ensure comprehensive NCD screening and management for all individuals accessing health-care facilities.

The core components of the SCCI include home-based care, medication refills, follow-up visits, treatment reminders, referrals and real-time monitoring of care delivery. One of the key features of the SCCI is its integration of screening into routine service delivery – every patient visiting a health facility is screened for major NCDs. This proactive approach is supported by periodic community-based screening campaigns to enhance early detection. The dual approach of facility-based and community-based screening ensures that individuals at risk are identified early and brought into the continuum of care. The success of the initiative led to its rapid scale-up to all 20 districts of Bhutan.

The country has also been a pioneer in the WHO SE Asia Region, becoming the first nation to adopt the WHO PEN interventions and later supplementing these with the WHO HEARTS technical package for cardiovascular disease management. With these frameworks in place, Bhutan has strengthened NCD services across more than 50 hospitals and 187 primary health centres nationwide. Bhutan has also made a bold regional commitment under the SEAHEARTS initiative: to place 50 000 individuals with hypertension and diabetes on protocol-based management. This commitment was strategically broken down to the district level and further distributed to each primary health care unit, ensuring accountability and clear targets at every level of the health system.

To support the effective delivery of NCD services, several systemic improvements were made. This included strengthening the availability of essential medicines and validated blood pressure measuring devices, building the capacity of health-care providers and upgrading patient information systems. All primary health centres now have at least one trained health worker capable of delivering protocol-based care. Furthermore, the information system allows for patient tracking through unique identifiers and supports real-time dashboards to monitor progress against the SEAHEARTS targets.

The SCCI has proven to be more than just a programme – it is a reflection of Bhutan's commitment to compassionate, patient-centred care and a testament to the power of integrated health system responses in addressing the NCD epidemic. The collaborative efforts of the Ministry of Health (MoH), Khesar Gyalpo University of Medical Sciences, local governments and the World Health Organization have been central to this success. Additionally, WHO Bhutan has supported the MoH and Dratshang to develop the Health Coordinator's Guide and has trained religious health coordinators to raise awareness of prevention of NCDs in their institutions and during religious gatherings in the community.



Launch of Dratshang Health Coordinators' Guide and screening of monks in the Tashichodzong on World Diabetes Day 2024

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DEMOCRATIC PEOPLE'S REPUBLIC OF KOREA

NCD prevention and management



The National Strategic Plan for the prevention and control of NCDs 2014–2020 is committed to strengthening and improving preventive health care, facilitating regular screening, early detection, registry and management of chronic diseases, including cardiovascular diseases, hypertension and diabetes.¹ The National Strategic Plan also focuses on strengthening multisectoral response, prevention, treatment and management of NCDs, expansion of population-based screening as well as health promotion and communication on risk factors.

In the past, information, education and communication activities have been actively conducted, focusing on healthy diet and physical activity across the country. The health agenda including healthy lifestyle has been disseminated almost every day through mass media campaigns involving television, audio, newspaper and national websites. This has largely contributed to raising awareness of NCDs and their prevention and management.

Orientation and application of WHO PEN and HEARTS to local primary settings have been initiated as well.

In 2018, “National guideline for the prevention and management of cardiovascular diseases and diabetes in clinics and Ri-hospitals” was developed in alignment with WHO PEN. It has prioritised strengthening the functions and roles of the “household (HH) doctor system”, a population-based health-care system established at the primary health care level.

This HH doctor system plays an important role in the prevention and control of NCDs in PHC settings. In this system, a household doctor takes charge of 130 households on average in the country, thereby playing a key role in raising awareness of healthy lifestyles.

National commitment: Health policy prioritizes increasing life expectancy by preventing and controlling diseases such as cardiovascular conditions and cancer, which contribute significantly to morbidity, mortality and disability. The government has placed specific emphasis on improving access to health services by strengthening the household doctor system, enhancing the quality of primary health care and promoting health through information, education and communication (IEC) activities.



Household (HH) Doctor System – a population-based healthcare approach at the primary care level

Healthy-lifestyle counselling



- ▶ National guideline for hypertension and diabetes includes health promotion and counselling on healthy lifestyles, such as quitting smoking, reducing harmful use of alcohol, and promoting healthy diet and physical activity using the 5As brief intervention approach.
- ▶ At all health-care facilities, staff are mandated to provide lifestyle counselling before delivering medical services. Household doctors conduct regular visits for screening, counselling and providing IEC on healthy lifestyles.
- ▶ Each primary care facility has a dedicated health-care provider, known as household doctor, responsible for regular screening and counselling during home visits and for visitors at the facility.

Evidence-based treatment protocols



- ▶ “National guideline for management of cardiovascular diseases and diabetes in clinics and Ri-hospitals” was developed in line with WHO PEN and disseminated in 2018. It is a protocol-based guidelines including medicine- and dose-specific guidance for management of hypertension and diabetes.
- ▶ Primary health care facilities in some pilot areas introduced protocol-based treatment and management for hypertension and diabetes in alignment with WHO PEN.²
- ▶ According to the protocol, there is a provision for medication to be refilled for a duration of 30 days.

Guideline for integrated management of cardiovascular diseases and diabetes in clinics and Ri hospitals

2018

Hypertension and diabetes management protocol

Access to medicines and technology



Medicines[#]

- ▶ The National List of Essential Medicines includes medicines [thiazide diuretics, calcium channel blocker, beta-blocker (atenolol), angiotensin-converting enzyme inhibitor, statin, metformin, glibenclamide and aspirin] that are part of the hypertension and diabetes management protocol used at the primary health care level.²
- ▶ Protocol medicines to manage hypertension and diabetes are estimated to be available at more than 50% of pharmacies in the PHC facilities.
- ▶ The medicine management system is being used for distribution and monitoring of medicines at PHC facilities.³
- ▶ Medicines are supplied regularly through the centralized national medicine supply system.

Technology^{##}

- ▶ Stethoscope, sphygmomanometer, measurement tape and weighing machines, and blood glucose measurement are “generally available” at the primary care facilities of the public health sector only.
- ▶ Urine strips for glucose and ketones, total cholesterol measurement and HbA1c are not available.

[#] “Generally available” is defined as “at 50% or more pharmacies”.

^{##} “Generally available” is defined as “at 50% or more health-care facilities”.

Risk-based CVD management



- ▶ Cardiovascular risk stratification is included in the “National guideline for management of hypertension and diabetes”.
- ▶ Some primary health care facilities provide cardiovascular risk stratification to manage patients at high risk of heart attacks and strokes.²
- ▶ The “National guideline for management of hypertension and diabetes” stipulates that statin be provided to people having a 10-cardiovascular risk $\geq 20\%$.

Team-based care



- ▶ Household (HH) doctors collaborate with community health workers in PHC settings to conduct regular screenings, facilitate early detection and diagnosis, and ensure timely treatment and ongoing management of chronic conditions such as cardiovascular diseases, cancer and diabetes.
- ▶ HH doctors routinely consult within clinics for diagnosis and treatment. Medicines are managed by pharmacists led by the clinic head, in cooperation with household doctors. Further consultation with experts at referral hospitals is ensured, as needed.
- ▶ Patients who need further diagnosis and treatment consult the Chief of the Household Doctors Department and the head of clinics, and are referred to district/county hospitals.

Systems for monitoring



- ▶ An electronic-based reporting system is utilized to collect and report NCD programme indicators at PHC facilities.
- ▶ The patient information system at primary health care facilities is capable of identifying and tracking hypertension patients using their names, age and address during each visit.
- ▶ Country has an electronic system to facilitate reporting of aggregated patient data on NCDs from primary health facilities to district/subnational and national levels.²

Progress of hypertension and diabetes coverage and control

The country has progressed in scaling up services for coverage and control of hypertension and diabetes management.

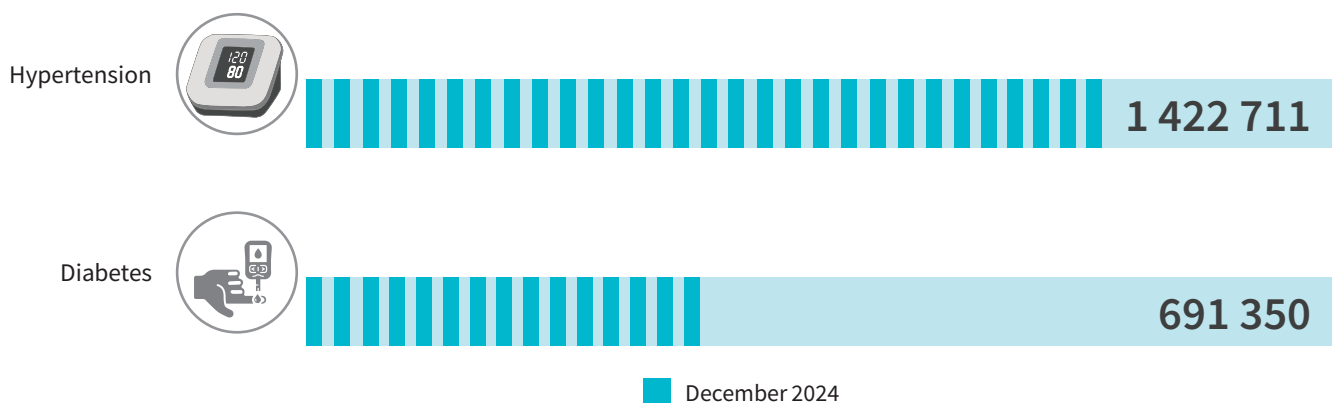


Figure 1: Number of persons with hypertension and/or diabetes placed on protocol-based management in health care facilities/clinics (2024)

MPOWER		Coverage	Achievements
M onitor tobacco use and prevention policies		Minimal measure achieved	- Conducted GATS in 2013 and 2017. - KAP survey on cessation of smoking was conducted in 2016-17.
P rotect people from tobacco smoke		Moderate measure achieved	- Incremental progress in implementation of smokefree laws. - Currently 6-7 public places completely smokefree.
O ffer help to quit tobacco use		Moderate measure achieved	- NRT is legally available in the country and can be purchased in pharmacies with a prescription. - Tobacco cessation services are available in most health facilities.
W arning about dangers of tobacco	Health warnings on Cigarette Packages	No measure or weak measure achieved	Textual health warnings on 30% of the front pack of cigarettes need to be implemented in the country.
	Anti-tobacco Mass Media Campaigns	Complete measure achieved	Achieved best-practice in mass media campaigns in GTCR 2017 and GTCR 2025 by conducting regular national-level anti-tobacco campaigns including airing on television and radio.
E nforce bans on tobacco advertising, promotion and sponsorship		No measure or weak measure achieved	Achieved the highest level of achievement (ban on all forms of direct and indirect advertising) in bans on TAPS in GTCR 2009.
R aise taxes on tobacco		No measure or weak measure achieved	Data not available.



IMPLEMENTATION OF WHO REPLACE TECHNICAL PACKAGE⁴

 National policy commitment to eliminate TFA: National policies, strategies or action plans that express a commitment to reduce industrially produced TFA in the food supply	Not Achieved	 Best-practice TFA policy: Legislative or regulatory measures that limit industrially produced TFA in foods in all settings and are in line with the recommended approach	Not Achieved
 Less restrictive TFA limits: Legislative or regulatory measures that limit industrially produced TFA in foods in all settings, but are less restrictive than the recommended approach	Not Achieved	 Other complementary measures: Legislative or other measures that encourage consumers to make healthier choices about industrially produced TFA in foods in specific settings	Not Achieved

IMPLEMENTATION OF WHO SHAKE TECHNICAL PACKAGE FOR REDUCING SALT INTAKE

Technical package element		Implementation Status
Food reformulation policies	Policies that set scope and criteria for maximum limits or targets for the sodium content in prepackaged food	Not achieved
Food and nutrition labelling policies	Policies that set scope and criteria to:	Achieved
	List and declare sodium on packaged food	Not achieved
	Regulate the use of sodium-related claims on packaged food	Not achieved
Food marketing restriction policies	Use front-of-pack labelling (FOPL) to provide interpretive information about sodium content	Not achieved
	Policies that set scope and criteria for restricting food marketing to protect children from its harmful impact on diet, including consumption of high-sodium food	Not achieved
	Behavior change communication and mass media campaigns for healthy diets	Achieved
Food procurement and service policies	A set of communication activities designed to change behaviors to reduce consumption of salt and high sodium food	Not achieved
Taxation on unhealthy food	Policies that set scope and criteria for sodium content in food served or sold in public settings and/or procured by government for food programmes	Not achieved
	Policies that set scope and criteria for taxes on unhealthy food, including high-sodium food	Not achieved

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The most effective primary health care services delivered by household doctors in DPR Korea¹

Across the globe, more than 80% of people in over 170 of WHO's 194 Member States rely on some form of traditional medicine – be it herbal remedies, yoga, Ayurveda, acupuncture, acupressure or indigenous therapies. For many, traditional medicine is not just complementary, it is the first and often the most trusted line of care, especially for managing chronic conditions. As the world increasingly embraces traditional medicine within the frameworks of PHC and universal health coverage (UHC), DPR Korea stands out for its deeply integrated and structured approach.

In DPR Korea, nearly 70% of the population uses Koryo traditional medicine at the PHC level, while 30%–40% use it at the central level and 40%–60% resort to it at the county level. What makes the DPR Korea model unique is its balance – traditional medicine is not used in isolation but is integrated with allopathic (Western) treatments to enhance care delivery and improve population health outcomes. At the heart of this health system

is the household doctor – or HH doctor – who serves as the backbone of care. All household doctors are qualified medical graduates from accredited institutions, including University of Medicine. On average, one HH doctor is responsible for the health of approximately 130–135 households. Their role extends far beyond treating illnesses; they lead health education campaigns, conduct screenings, ensure early diagnoses and provide timely treatment.

Chronic diseases, including hypertension and diabetes, are managed proactively. Patients are registered and followed up on regularly, with Koryo traditional medicine playing a key role in managing these conditions. In DPR Korea, PHC facilities, including clinics and Ri-hospitals, are distributed proportionately, according to administrative divisions, population density and regional requirements, playing a vital role in delivering health services to the populations under their care. Group discussions among HH doctors and consultations with experts from referral hospitals help ensure accurate diagnoses and effective treatment plans.

Data including the number of patients and controlled patients are electronically collected, collated and reported to higher-level health facilities, finally reaching the Ministry of Public Health. The outcome of this integrated model is Koryo traditional medicine getting successfully embedded within the broader health system, playing a vital role in chronic disease management across all care levels. Household doctors ensure personalized, continuous care for every family, delivering a powerful mix of prevention, education and treatment. The registry and management of chronic diseases are systematized, enabling consistent follow-up and better health outcomes.

Key collaborators driving this model include the Ministry of Public Health, HH doctors and Koryo traditional medicine practitioners, who are working together to ensure that health care truly reaches every household.



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Primary health care services delivered by household doctors

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INDIA

NCD prevention and management



To address noncommunicable diseases (NCDs), the Government of India launched the national programme in 2010, aiming to cover the entire population. In 2023, updated operational guidelines for 2023–2030 were introduced under the National Programme for Prevention and Control of NCDs (NP-NCD).¹ These guidelines seek to enhance the prevention and management of NCDs at all levels of health care.

Additionally, preventive and management measures for NCDs have been strengthened via the Ayushman Bharat Health and Wellness Centres, also known as Ayushman Arogya Mandirs. These initiatives include community-level risk assessments using the Community-Based Assessment Checklist (CBAC), facility-level screening for conditions such as hypertension and diabetes, and follow-up activities adopting a continuum-of-care approach. These services are now essential at subhealth centres (SHC-HWC) and primary health centres (PHC-HWC), alongside enhanced health promotion, management and referral services. Currently, over 177 142 functional health and wellness centres (Ayushman Arogya Mandirs) operate across the country.²

Since 2016, the National Multisectoral Action Plan (NMAP) has been implemented to provide a roadmap and policy options, guiding collaborative efforts across various ministries and departments. The Food Safety and Standards Authority of India (FSSAI) has launched a national campaign called the “Eat Right India” movement to improve public health and combat lifestyle diseases.³ The FSSAI has also launched the “Eat Right Toolkit” and the “Safe and Nutritious Food at Workplace” campaign. The “Fit India Movement” was initiated and awareness raised through social media, print and electronic media, organization of various events and collaboration with various ministries to promote physical activities.⁴

National commitment: In May 2023, the Ministry of Health and Family Welfare announced its commitment to screen and enroll 75 million individuals with hypertension or diabetes in protocol-based care by 2025.⁵



Hypertension screening at a primary health centre in India

Healthy-lifestyle counselling



- ▶ National management protocols for hypertension and diabetes include counselling patients on tobacco use, diet, alcohol consumption and physical activity, using the 5As brief intervention approach; warning signs of cancer, air pollution, obesity, importance of treatment adherence, etc.
- ▶ Country has dedicated personnel/health-care providers, such as counsellors and community health officers, at primary care facilities to provide counselling.

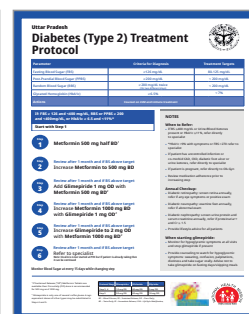
Evidence-based treatment protocols



- ▶ Evidence-based drug- and dose-specific national protocols for management of hypertension are being implemented in 33 States and Union Territories, while diabetes protocol is being implemented in 25 States and Union Territories.⁶
- ▶ At least 50% of the primary health care facilities in the country are estimated to be implementing protocol-based treatment for hypertension and diabetes.⁷
- ▶ According to the protocol implemented in the States and Union Territories, there is a provision for medication refill for a duration of minimum 30 days (monthly), ranging from 30 to 90 days.



Hypertension management protocol



Diabetes management protocol

Access to medicines and technology



Medicines[#]

- ▶ The National List of Essential Medicines 2022 includes medicines (metformin, insulin, sulphonyl urea, thiazide, ACE inhibitors, angiotensin II receptor blockers, calcium channel blockers, beta blockers, statins and aspirin, methyl dopa) that are part of the hypertension and diabetes management protocol used at the primary health care level.⁸
- ▶ These medicines are estimated to be “generally available” (available at 50% or more pharmacies) at the primary care facilities of the public health sector.⁷
- ▶ The Drug and Vaccine Distribution Management System (DVDMS) is an electronic logistics management information system for forecasting, procurement, distribution and monitoring of medicines at health facilities across States and Union Territories.⁹

Technology^{##}

- ▶ Equipment for measuring blood glucose and blood pressure, urine strips for ketone, glucose, and albumin assays, tuning forks for assessing foot vibration perception, torches and tongue depressors for oral visual examination (OVE), pulse oximeters and visual inspection with acetic acid (VIA) kits are “generally available” at primary health care facilities across both public and private sectors.⁷

Risk-based CVD management



- ▶ Cardiovascular risk stratification is included in the nationally approved protocol for hypertension and diabetes management.
- ▶ 25%–50% of primary care facilities provide primarily non-laboratory-based risk stratification.

[#] “Generally available” is defined as “at 50% or more pharmacies”.

^{##} “Generally available” is defined as “at 50% or more health-care facilities”.

Team-based care



- ▶ Screening for hypertension and diabetes is conducted by Accredited Social Health Activists (ASHA), using CBAC during their home visits, and by auxiliary nurse midwives (ANMs) at subcentres/health and wellness centres.
- ▶ Diagnosis, treatment initiation and monitoring are carried out at primary health centres. Opportunistic screening for NCDs is conducted for patients attending public health facilities.
- ▶ Community health workers and health professionals function as a team to conduct screening, detection and management of hypertension and diabetes in the community and at health facilities. Treatment initiation is ensured by doctors/community health officers (CHOs), but drug refills can be done by ANMs at subcentres.
- ▶ 218 SPOs/POs/DPMs, 12 203 medical officers, 9203 nurses, 21 482 CHOs, 6305 pharmacists and 20 404 ANMs had been trained under the 75 by 25 initiative in protocol-based hypertension and diabetes care till March 2025.
- ▶ Primary health care providers have access to specialist consultation services for case-specific clinical issues. Routine teleconsultation service between community clinics and higher facilities is available through the national telemedicine service, i.e. e-Sanjeevani.¹⁰

Systems for monitoring



- ▶ The country has a digital data recording system, known as the National NCD Portal, to identify/track hypertension and diabetes patients using unique identifiers (Ayushman Bharat Health Account) at public health facilities.¹¹
- ▶ More than 75% of primary health care facilities use dashboards to visualize performance and trends of key hypertension and diabetes indicators.⁷
- ▶ There is a provision for primary health facilities to generate information on the number of new patients, proportion of registered patients on treatment and proportions with control for hypertension and diabetes at public health facilities.
- ▶ There is a provision at national and subnational levels to publish the hypertension and diabetes treatment coverage data on a regular basis in the public domain.¹²
- ▶ There is an electronic system to facilitate reporting of aggregated patient data on NCDs from primary health facilities to district/subnational and national levels.¹¹
- ▶ The National NCD Portal helps health-care providers in delivering and monitoring population-based screening for NCDs. It standardizes electronic health records, enables unique health IDs, tracks referrals, ensures follow-ups and digitizes records, streamlining reporting, referrals and health personnel planning at all levels of the health-care system.

Progress of hypertension and diabetes coverage and control

The country has progressed in scaling up services for coverage and control of hypertension and diabetes management

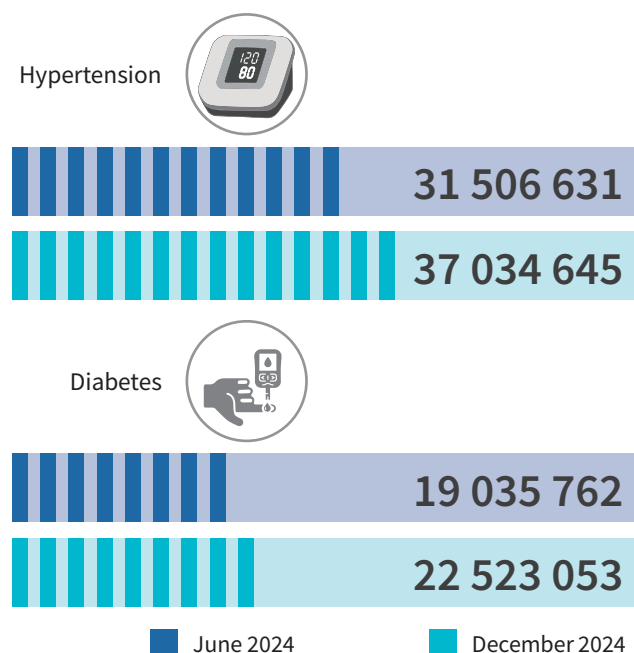


Figure 1: Number of persons with hypertension and/or diabetes placed on protocol-based management in health care facilities/ clinics (2024)

165 879 health facilities providing protocol-based hypertension management

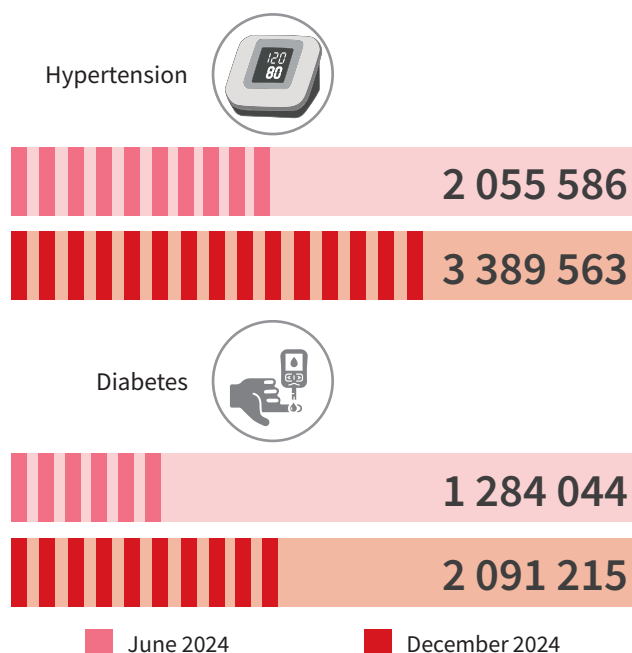


Figure 2: Hypertension and diabetes control status among people on treatment in health care facilities/ clinics (2024)

MPOWER		Coverage	Achievements
M onitor tobacco use and prevention policies		Moderate measure achieved	- Regularly monitored the use of tobacco products among adults as well as youth. - The country achieved the highest level of implementation in M in GTCR 2009, GTCR 2011, and GTCR 2013. - Tobacco Questions for Surveys (TQS) are being increasingly integrated in national health surveys.
P rotect people from tobacco smoke		Moderate measure achieved	- Six to seven public places completely smoke-free. - To address the growing tobacco use among children and adolescents, the Government of India had released the “Guidelines for Tobacco Free Educational Institutions” in 2008. These guidelines were updated in 2019.
O ffer help to quit tobacco use		Complete measure achieved	- India has strategically invested in scaling up tobacco cessation through mCessation services and the toll-free National Tobacco Quitline, which has four hubs servicing different regions of the country. - The Quitline is staffed by 100 trained counsellors providing services in over 15 languages to address the needs of tobacco users. - In GTCR 2017, GTCR 2019, GTCR 2021, GTCR 2023 and GTCR 2025, India has been placed in the best-practice group.
W arning about dangers of tobacco	Health warnings on Cigarette Packages	Complete measure achieved	Pictorial health warnings covering 85% of the principal display areas are mandated by law.
	Anti-tobacco Mass Media Campaigns	Moderate measure achieved	Achieved the best-practice level in mass media campaigns in GTCR 2011, GTCR 2013, and GTCR 2015 by conducting regular national-level anti-tobacco campaigns with at least seven appropriate characteristics, including airing on television and/or radio.
E nforce bans on tobacco advertising, promotion and sponsorship		Moderate measure achieved	The Government of India’s “film rules” offer a global first in countering TAPS and helping to denormalize tobacco use through the use of strong anti-tobacco messaging integrated into entertainment content. Additionally, India’s 2023 OTT tobacco control rules require all online content to display anti-tobacco warnings and disclaimers.
R aise taxes on tobacco		Moderate measure achieved	The share of total taxes in the retail price of the most widely sold brand of cigarettes in the country is 58.2%.





REPLACE
TRANS FAT

IMPLEMENTATION OF WHO REPLACE TECHNICAL PACKAGE¹³



Best-practice TFA policy: Legislative or regulatory measures that limit industrially produced TFA in foods in all settings and are in line with the recommended approach

Achieved



Monitoring mechanism for mandatory TFA limits

Achieved



Other complementary measures: Legislative or other measures that encourage consumers to make healthier choices about industrially produced TFA in foods in specific settings

Achieved



IMPLEMENTATION OF WHO SHAKE TECHNICAL PACKAGE FOR REDUCING SALT INTAKE

Technical package element		Implementation Status
Food reformulation policies	Policies that set scope and criteria for maximum limits or targets for the sodium content in prepackaged food	Not achieved
Food and nutrition labelling policies	List and declare sodium on packaged food	Achieved
	Regulate the use of sodium-related claims on packaged food	Achieved
	Use front-of-pack labelling (FOPL) to provide interpretive information about sodium content	In process
Food marketing restriction policies	Policies that set scope and criteria for restricting food marketing to protect children from its harmful impact on diet, including consumption of high-sodium food	Achieved
Behavior change communication and mass media campaigns for healthy diets	A set of communication activities designed to change behaviors to reduce consumption of salt and high sodium food	Achieved
Food procurement and service policies	Policies that set scope and criteria for sodium content in food served or sold in public settings and/or procured by government for food programmes	Not achieved
Taxation on unhealthy food	Policies that set scope and criteria for taxes on unhealthy food, including high-sodium food	Not achieved

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Pinch less salt, a pound more health: India's salt reduction revolution

Similar to other countries of the Region, India faces the challenge of excessive salt intake, estimated at nearly 10 g per person per day. This significantly increases the risk of hypertension and cardiovascular diseases.¹ Recognizing the need to limit salt intake to less than 5 g daily, as per WHO recommendation, the Government of India has implemented several interventions to reduce dietary salt consumption at the population level through a coordinated, multisectoral approach involving policy reform, updated dietary guidelines, food regulation and public education.

One of the key outcomes achieved includes the launch of the revised Dietary Guidelines for Indians (DGI) in 2024 by the National Institute of Nutrition (NIN),² under the Indian Council of Medical Research (ICMR). For the first time, these national dietary guidelines have emphasized the importance of minimizing added salt in diets across all population groups, especially among infants and young children, acknowledging the rising burden of NCDs and related premature mortality. The guidelines specifically recommend:

- limiting salt intake to less than 5 g per day for all age groups;
- limiting the use of salt in complementary food preparations; and
- restricting consumption of foods that are high in fat, salt and sugar, and ultra-processed foods.

Since several other countries in the South-East Asia Region refer to the Indian Dietary Guidelines, these guidelines will benefit not just the population in India but will have a regional impact as well.

These guidelines are being operationalized through the Ministry of Women and Child Development's (MoWCD) flagship programme, POSHAN 2.0/Integrated Child Development Services (ICDS), one of the largest food safety net programmes globally, to maximize outreach among women and children. In April 2025, the MoWCD issued an advisory to all States and Union Territories, mandating that the supplementary food, including the dry take-home rations (THR) and hot cooked meals (HCM), provided under the ICDS, should either contain no added salt or restrict it to a minimal level. The advisory also emphasized the elimination of refined sugar and foods high in fat, salt, sugar, etc. from these meals.

Given that most salt in India is consumed as table salt, another important initiative of the Government of India is the "Eat Right India" movement spearheaded by the Food Safety and Standards Authority of India (FSSAI) under the Ministry of Health and Family Welfare. This movement seeks to drive behavioural change among consumers to promote healthier dietary choices. It runs a variety of programmes to promote healthy eating, including reducing dietary salt consumption in various settings that encompass schools, colleges, workplaces, homes, markets, transport hubs and places of worship.³ The movement also promotes better understanding of nutrition labels. Campaigns such as "Aaj Se Thoda Kam" (less salt from today) encourage gradual reduction of salt, sugar and fat in diets, while the National Low Salt Cooking Challenge engages and sensitizes citizens to reducing the amount of salt in home-cooked foods.⁴

The "Eat Right Toolkit" has been developed to communicate key messages on food safety and nutrition to the population at large, with a focus on preventive health care.⁵ The toolkit is built on two broad components – "Eat Healthy" and "Eat Safe". It attempts to deliver clear and simple messages on foods to eat (balanced diet, fortified foods, nutrition during the first 1000 days) and foods to avoid (high fat, sugar and salt foods), and elimination of trans-fats from our diets.

Complementary regulatory measures further support these public health goals. The Food Safety and Standards (Advertising and Claims) Regulations, 2018, regulate the marketing of foods high in salt, sugar and fats, and prohibit advertising targeted at children in any form. The Food Safety and Standards (Safe Food and Balanced Diets for Children in School) Regulations, 2020, of the FSSAI prohibit marketing and sale of high fat, sugar and salt (HFSS) foods within a 50-metre radius of school premises.⁶ Schools are encouraged to adopt a comprehensive programme for promoting safe food and healthy diets among schoolchildren, and to convert school campuses into "Eat Right Schools" focusing on safe and healthy food, local and seasonal food, and no food waste, as per the specified benchmarks. School authorities are also guided on promoting the consumption of safe and balanced diet, in line with NIN guidelines.

In a recent development, the FSSAI has also notified the draft Food Safety and Standards (Labelling and Display) Amendment Regulations, 2022, targeting processed and packaged foods.⁷ These draft regulations require front-of-pack nutrition labels to give a star rating to foods based on the nutritional content, with foods higher in salt, sugar, fat and energy getting a lower score and hence, a lower star rating, along with declaration of the percentage of the recommended dietary allowance (RDA) for these nutrients being met per serving of the food.

India has taken bold and integrated steps to promote healthy diets, particularly by reducing dietary salt intake. According to the WHO Sodium Score Card, a two-point improvement from the 2019 baseline status, on the part of countries of the WHO SE Asia Region, could reduce the mean population sodium intake by 841 mg/day, equivalent to a reduction of 2.1 g of salt per day.⁸ Courtesy of the implementation of the latest dietary guidelines and the advisory on reducing salt in government nutrition programmes, the ongoing finalization of FoPL regulations and enhanced public awareness of the dietary risks associated with salt, India is progressing in the right direction and has the potential to achieve its commitment of a 30% reduction in sodium intake by 2030.

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INDONESIA

NCD prevention and management



Indonesia has implemented comprehensive strategies for the prevention and management of NCDs within its PHC system. Recognizing the significant burden posed by NCDs, such as hypertension, diabetes and cardiovascular diseases, the country has adopted the WHO PEN framework under the national name, *PANDU PTM (Pelayanan Terpadu Penyakit Tidak Menular)*, an integrated health service for NCDs.¹ Moreover, the government has included NCDs as a criterion for accreditation for primary health care services.

As part of the Health Transformation initiative, implemented as an integrated effort to prevent NCDs through the Free Health Check Programme, health checks are designed to address the full life-cycle of individuals within the community. These programmes focus on screening risk factors and early detection of obesity, hypertension, diabetes, hypercholesterolaemia, cancer, smoking, oral health, and eye and ear disorders.² These initiatives have been scaled up nationally since 2010, providing NCD screening services and health promotion that can be accessed at the grassroots level. The Ministry of Health has also launched the Healthy Indonesia Programme, which further emphasizes the importance of early detection in reducing the burden of NCDs.

Additionally, the *Posyandu Prima Program*, a national-scale initiative operating as a satellite primary health care facility in villages, plays a dual role in addressing stunting and supporting NCD screening and disease control programs. Although its primary objective is to reduce malnutrition and stunting, the program's infrastructure and resources are effectively leveraged to integrate NCD prevention services, expanding their reach to underserved populations. Community health workers, known locally as *kaders*, play a vital yet often invisible role in these efforts. They facilitate community-level screening and provide essential support for the management of NCDs, ensuring that health services are both accessible and sustainable.³

National commitment: NCD Directorate Report 2023 stated the targets of:

- 1) 70% of the population in the target age range;
- 2) 63% of hypertension controlled in primary care;
- 3) 58% of controlled glycaemic level; and
- 4) 474 cities/districts with no-smoke areas.



Community member undergoing blood pressure screening as part of efforts for early detection and management of hypertension

Healthy-lifestyle counselling



- ▶ National guidelines for hypertension include counselling patients on tobacco use, dietary approaches to stop hypertension (*DASH*), alcohol consumption, physical activity and stress management using the 5As intervention approach.
- ▶ Country has dedicated personnel/scope of work to provide counselling not only for diabetes and hypertension, but also for HIV, psychiatric conditions, issues faced by the elderly, neonatal conditions, etc.

Evidence-based treatment protocols



- ▶ Evidence-based drug- and dose-specific national protocols for management of hypertension in primary health care, aligned with WHO HEARTS and PEN technical packages as well as the “WHO Guideline for the pharmacological treatment of hypertension in adults”, are being implemented since 2024.
- ▶ At least 50% of the primary health care facilities in the country are estimated to be implementing protocol-based treatment for hypertension and diabetes.⁴
- ▶ According to the protocol, there is a provision for medication refill for a duration of 30 days.



Hypertension management protocol



Diabetes management protocol

Access to medicines and technology



Medicines[#]

- ▶ *Daftar Obat Esensial Nasional* includes medicines (amlodipine, lisinopril, ACE inhibitors, thiazides, metformin, sulphonyl ureas) that are part of the hypertension and diabetes management protocol used at the primary health care level.⁵
- ▶ These medicines are estimated to be “generally available” (available at 50% or more pharmacies) at the primary care facilities of the public health sector.⁴
- ▶ Current procurement and logistics management are through e-catalogue procurement and local government. The electronic logistics management information system for drug distribution and monitoring through *SATUSEHAT* application is under process.
- ▶ Medicines are procured on an ad hoc basis through a decentralized mechanism by facilities and provincial government, regulated by the Ministry of Health.⁶

Technology^{##}

- ▶ Equipment for measuring blood glucose and blood pressure and foot vibration perception by tuning fork are estimated to be “generally available” at primary health care facilities of the public and private health sectors.⁴
- ▶ Total cholesterol is “generally available” at the primary care facilities of both public and private health sectors, whereas HbA1c, urine strips for ketone and glucose testing are “generally not available”.

Risk-based CVD management



- ▶ Cardiovascular risk stratification is included in the nationally approved protocol for hypertension management.
- ▶ More than 50% of primary care facilities provide non-laboratory-based cardiovascular risk stratification.⁴
- ▶ There is a guideline to provide statin to people having a 10-cardiovascular risk ≥ 20 .

[#] “Generally available” is defined as “at 50% or more pharmacies”.

^{##} “Generally available” is defined as “at 50% or more health-care facilities”.

Team-based care



- ▶ The programme, Free Health Screening, targets individuals aged 15 years and above, offering five key services: 1) enrolment; 2) assessment of NCD risk factors, behaviour-related health; 3) anthropometric measurement; 4) basic medical check-up (glucose level, blood pressure, cholesterol level); and 5) counselling, reporting and referrals to higher health-care facilities. These services are provided by the health-care team.
- ▶ Treatment initiation is carried out by medical doctors, while drug refills require patients to be re-examined; these are then facilitated by frontline workers/pharmacists.
- ▶ There is currently no provision for health-care providers at primary health facilities to consult specialists regarding cases of hypertension and diabetes. Routine teleconsultation services between community clinics and higher-level facilities are not available.

Systems for monitoring



- ▶ About 95% of primary health care facilities use *Aplikasi Sehat IndonesiaKu (ASIK)* for electronic NCD reporting, with only remote areas still relying on paper-based records.⁴
- ▶ The patient information system at primary health care facilities is capable of identifying and tracking hypertension and diabetes patients using a unique identifier during each visit.
- ▶ >75% of primary health care facilities use dashboards to visualize performance and trends of key hypertension and diabetes indicators.
- ▶ Country has an electronic system to facilitate reporting of aggregated patient data on NCDs from primary health facilities to district/subnational and national levels.
- ▶ There is a mechanism/provision at the national level to publish the hypertension and diabetes treatment coverage data on a regular basis in the public domain.
- ▶ Indonesia has IT-enabled initiatives (*ASIK*, *SATUSEHAT*) for patient records and hospital data. A web-based NCD system with unique IDs links public primary care facilities (*Puskesmas*) and community activities to the health ministry's data centre, with plans to integrate with National Insurance and Civil Registration.
- ▶ Indonesia's NCD health data are monitored via *Sistem Informasi Penyakit Tidar Menular (SI-PTM)* under MoH supervision. Additional platforms under *Jaminan Kesehatan Nasional (JKN)*, including *Badan Penyelenggara Jaminan Sosial (BPJS)* (P-Care), offer more complete data but are less accessible to MoH. Data fragmentation remains, with efforts underway by the Digital Transformation Office to unify systems and standardize national health information.

Progress of hypertension and diabetes coverage and control

The country has progressed in scaling up services for coverage and control of hypertension and diabetes management.

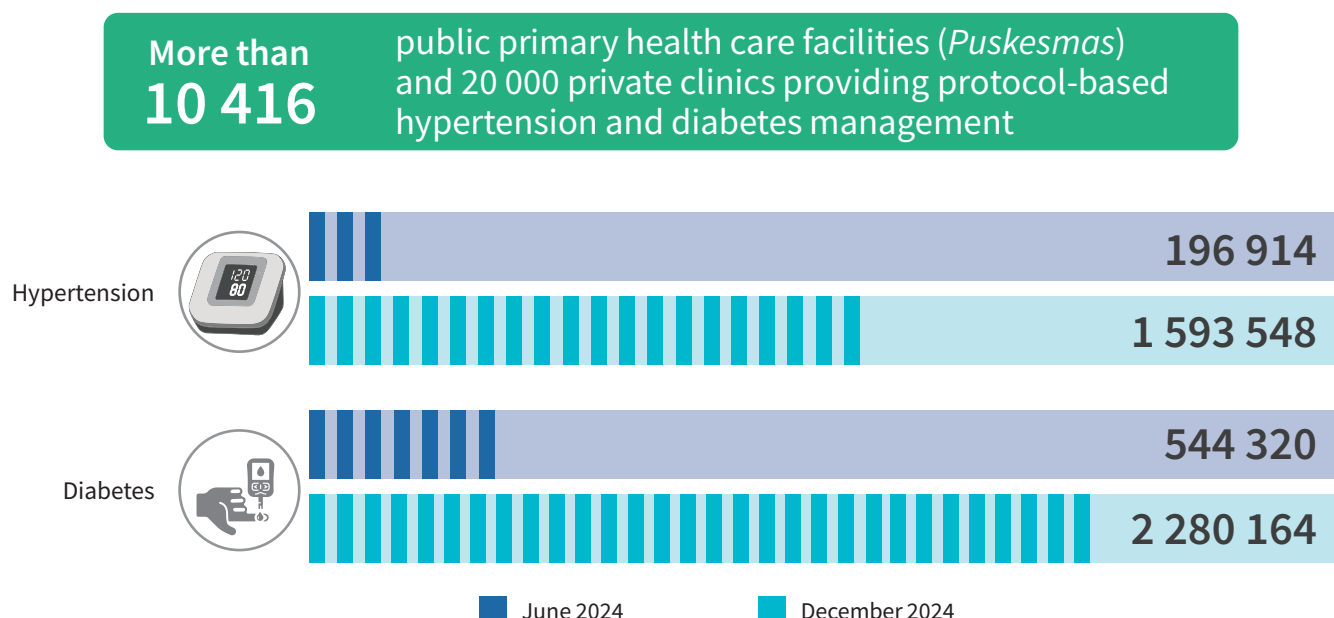


Figure 1: Number of persons with hypertension and/or diabetes placed on protocol-based management at health-care facilities/ clinics (2024)

MPOWER	Coverage	Achievements
M onitor tobacco use and prevention policies	 Complete measure achieved	- Regularly monitored the use of tobacco products among adults as well as youth. - Achieved the highest level of implementation in M in GTCR 2023 and 2025. - Tobacco Questions for Surveys (TQS) are integrated in national health surveys.
P rotect people from tobacco smoke	 Complete measure achieved	- All places completely smoke-free. - WHO Indonesia supported the MoH in the development of the smoke-free mobile application and dashboard (Dashboard E-Monev Kawasan Tanpa Rokok). This web-based platform monitors and visually represents the progress of subnational smoke-free policy implementation.
O ffer help to quit tobacco use	 Moderate measure achieved	- A “functional” national toll-free tobacco quitline is in place. - NRT is legally available.
W arning about dangers of tobacco	Health warnings on Cigarette Packages  Complete measure achieved	Pictorial health warnings covering 50% of the package area are mandated by law.
	Anti-tobacco Mass Media Campaigns  No measure or weak measure achieved	Achieved best-practice in mass media campaigns in GTCR 2017, 2019 and 2021 by conducting regular national-level anti-tobacco campaigns with at least 7 appropriate characteristics, including airing on television and/or radio.
E nforce bans on tobacco advertising, promotion and sponsorship	 No measure or weak measure achieved	Despite lacking national legislation on a comprehensive TAPS ban, 41 cities/districts in Indonesia have banned outdoor tobacco advertising and advertisements at the point of sale.
R aise taxes on tobacco	 Complete measure achieved	The share of total taxes in the retail price of the most widely sold brand of cigarettes in the country is 78.9%; taxes are also levied on HTPs, ENDS, and ENNDs; country dedicates a portion of tobacco tax revenues to specific health programmes.



REPLACE IMPLEMENTATION OF WHO REPLACE TECHNICAL PACKAGE⁷

 Best-practice TFA policy: Legislative or regulatory measures that limit industrially produced TFA in foods in all settings and are in line with the recommended approach	Not Achieved	 Less restrictive TFA limits: Legislative or regulatory measures that limit industrially produced TFA in foods in all settings, but are less restrictive than the recommended approach	Achieved
 Monitoring mechanism for mandatory TFA limits	Not Achieved	 Other complementary measures: Legislative or other measures that encourage consumers to make healthier choices about industrially produced TFA in foods in specific settings	Achieved

IMPLEMENTATION OF WHO SHAKE TECHNICAL PACKAGE FOR REDUCING SALT INTAKE

Technical package element		Implementation Status
Food reformulation policies	Policies that set scope and criteria for maximum limits or targets for the sodium content in prepackaged food	In process
Food and nutrition labelling policies	Policies that set scope and criteria to:	Achieved
	List and declare sodium on packaged food	Achieved
	Regulate the use of sodium-related claims on packaged food	Not achieved
	Use front-of-pack labelling (FOPL) to provide interpretive information about sodium content	In process
Food marketing restriction policies	Policies that set scope and criteria for restricting food marketing to protect children from its harmful impact on diet, including consumption of high-sodium food	In process
Behavior change communication and mass media campaigns for healthy diets	A set of communication activities designed to change behaviors to reduce consumption of salt and high sodium food	Achieved
Food procurement and service policies	Policies that set scope and criteria for sodium content in food served or sold in public settings and/or procured by government for food programmes	Not achieved
Taxation on unhealthy food	Policies that set scope and criteria for taxes on unhealthy food, including high-sodium food	Not achieved

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Indonesia's bold regulatory steps in addressing dietary risk factors for NCDs

Indonesia is facing a growing burden of noncommunicable diseases (NCDs). Dietary risks now rank as the third leading cause of death and disability, with more than 50% of ischemic heart disease cases, nearly one fifth of diabetes deaths (17.51%) and almost one third of strokes (29.41%) linked to poor diets. Obesity has become increasingly prevalent, with adult obesity climbing from 15.4% to 23.4% in just 10 years. Children have not been spared, with 4.2% of those under five now overweight.

Recognizing the urgent need for action, the Government of Indonesia – led by the Ministry of Health (MoH) and the Food and Drug Administration (BPOM) – spearheaded a national agenda focused on reducing dietary risk factors. Throughout 2023–2024, the Indonesian government, with support from WHO, convened a series of policy dialogues, workshops and technical discussions to strengthen national capacity on WHO recommendations and global best practices, and push for adaptation and adoption of proven interventions in Indonesia. These global proven interventions include sugar-sweetened beverage (SSB) taxation, food labelling, marketing restrictions, food reformulation to reduce salt, sugar and fat, and the elimination of industrially produced trans-fatty acids (TFA).



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These forums brought together various governmental bodies, academia, civil society organizations and international experts, facilitating knowledge sharing and providing meaningful insights into the adaptation of these interventions in the Indonesian context for implementation.

These collaborative efforts culminated in the enactment of government regulation No. 28 of 2024 – a major policy achievement led by the Indonesian government. The regulation mandates:

- Food reformulation to reduce sugar, salt and fat in packaged and ready-to-eat foods.
- Mandatory front-of-pack nutrition labelling to empower healthier consumer choices.
- There should be restrictions on the marketing of unhealthy foods.

This regulation represents a landmark moment in Indonesia's public health policy, reflecting a strong national commitment to tackling NCDs through upstream, population-wide interventions that address unhealthy diets. To operationalize the regulation, the government is in the process of developing technical implementing regulations for each of the measures.

MALDIVES

NCD prevention and management



Maldives recognizes the increasing prevalence of NCDs as a significant developmental issue and has launched strategic initiatives to address the growing burden. The National Multisectoral Action Plan for NCDs (2023–2031) outlines a comprehensive approach to achieve NCD reduction targets by 2025 and 2030. As part of these efforts, the 25–30 Campaign, launched in 2022, aims to reduce NCDs by 25% by 2030.¹

Since 2017, the country has implemented the WHO PEN interventions for primary health care, ensuring widespread access to curative services for chronic conditions such as hypertension and diabetes. These services, including medications, are fully covered under the national health insurance scheme, Aasandha.²

In 2022, the government introduced an integrated primary health care (PHC) model to enhance NCD prevention and control through better integration and expanded primary care capabilities. Piloted in Faafu Atoll, this initiative incorporates innovative approaches, including revised care and referral pathways, unified PHC teams, cardiovascular risk management tools, standardized treatment guidelines and a digital PHC portal for real-time data. Additionally, it emphasizes community-based NCD screening, early diagnosis, telemedicine consultations and robust systems for monitoring, evaluation and supportive supervision. In 2024, the government started scaling up the model to other atolls in a phased manner.³

National commitment: At least 25 000 people with hypertension and diabetes on protocol- based management by 2025.



Faafu atoll healthcare professionals trained to utilize the digital tool used to collect information for assessing baseline population screening for noncommunicable diseases

Healthy-lifestyle counselling

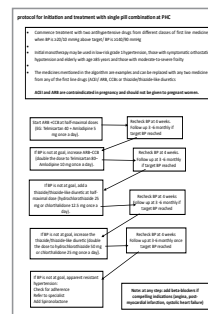


- ▶ National management protocols for hypertension and diabetes include counselling patients on tobacco use, diet, substance use and physical activity, using the 5As brief intervention approach.
- ▶ Country has dedicated personnel/health-care providers (such as counsellors) at primary care facilities to provide healthy lifestyle counselling.

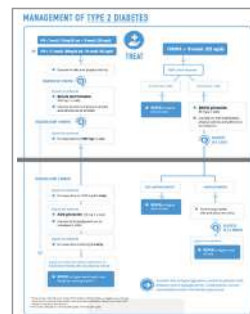
Evidence-based treatment protocols



- ▶ Evidence-based drug- and dose-specific national protocols for management of hypertension and diabetes in primary health care, aligned with WHO HEARTS and PEN technical packages, are being implemented since 2022.
- ▶ At least 50% of the primary health care facilities in the country are estimated to be implementing protocol-based treatment for hypertension and diabetes.⁴
- ▶ According to the protocol, there is a provision for medication refill for a duration of 90 days (90 days, if controlled, follow-up after 30 days, if not controlled).



Hypertension management protocol



Diabetes management protocol

Access to medicines and technology



Medicines[#]

- ▶ The National Essential Medicine List 2023 includes medicines (amlodipine, ACE inhibitors, thiazides, metformin, glimepiride) that are part of the hypertension and diabetes management protocol used at the primary health care level.⁵
- ▶ These medicines are estimated to be “generally available” (available at 50% or more pharmacies) at the primary care facilities of the public health sector.⁴
- ▶ There is an electronic logistics management information system (LMIS) for forecasting, procurement, distribution and monitoring of medicines at health facilities.⁶
- ▶ Medicines are procured biannually through a centralized mechanism by the State Trading Organization (for public sector pharmacies); and by the Ministry of Health (for government health facilities).

Technology^{##}

- ▶ Equipment for measuring blood glucose and blood pressure, urine strips for ketone and glucose, and urine strips for albumin assay are estimated to be “generally available” across public and private primary health care facilities.⁴
- ▶ HbA1c and total cholesterol testing are reported to be “generally available” at primary health care facilities of both public and private health sectors.

Risk-based CVD management



- ▶ Cardiovascular risk stratification is included in the nationally approved protocol for hypertension and diabetes management.
- ▶ More than 50% of primary care facilities provide laboratory-based risk stratification.⁴
- ▶ There is a protocol to provide statin to people having a 10-cardiovascular risk ≥ 20 .

[#] “Generally available” is defined as “at 50% or more pharmacies”.

^{##} “Generally available” is defined as “at 50% or more health-care facilities”.

Team-based care



- ▶ Community health workers (CHWs) are mainly based in the public health unit of the health centre or hospital and family health workers (FHWs) mostly conduct home visits. Urban-based CHWs focus mainly on NCDs and home visits for bedridden patients.
- ▶ Community health workers and health professionals function as a team to conduct screening, detection and management of hypertension and diabetes in the community and at health facilities.
- ▶ Treatment initiation is carried out by medical doctors, while drug refills are also facilitated by doctors.
- ▶ 132 health-care providers have been trained in protocol-based hypertension and diabetes care in 2024.
- ▶ Provision is available for health-care providers at primary health facilities to consult specialists regarding cases of hypertension and diabetes through telemedicine or through referrals. Routine teleconsultation services between community clinics and higher-level facilities are available.

Systems for monitoring



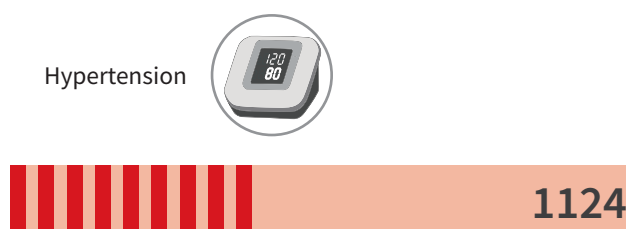
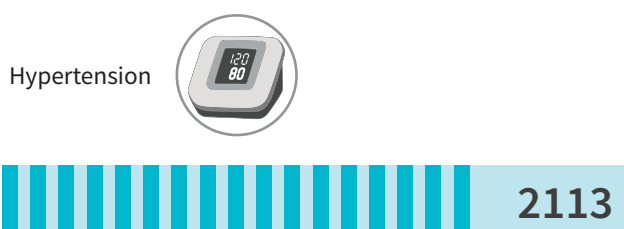
- ▶ A mixed system – paper-based and electronic (for now, as the electronic system is being rolled out) – is utilized to collect and report NCD programme indicators at primary health care facilities.
- ▶ The patient information system at primary health care facilities is capable of identifying and tracking hypertension and diabetes patients using a unique identifier during each visit.⁴
- ▶ Less than 25% of primary health care facilities use dashboards to visualize performance and trends of key hypertension and diabetes indicators.
- ▶ Country has an electronic system to facilitate reporting of aggregated patient data on NCDs from primary health facilities to district/subnational and national levels.⁴
- ▶ There is a mechanism/provision at the national level to publish the hypertension and diabetes treatment coverage data on a regular basis in the public domain.

Progress of hypertension and diabetes coverage and control

The country has progressed in scaling up services for coverage and control of hypertension and diabetes management.

More than
75%

health facilities providing
protocol-based
hypertension management



December 2024

December 2024

Figure 1: Number of persons with hypertension and/or diabetes placed on protocol-based management in health care facilities/ clinics (2024)

Figure 2: Hypertension and diabetes control status among people on treatment in health care facilities/ clinics (2024)

MPOWER	Coverage	Achievements
M onitor tobacco use and prevention policies	 Moderate measure achieved	- Regularly monitored the use of tobacco products among adults and youth. - Tobacco Questions for Surveys (TQS) are integrated in national health surveys.
P rotect people from tobacco smoke	 Minimal measure achieved	- Three to five public places completely smoke-free. - Maldives achieved the highest level of implementation in P in GTCR 2011.
O ffer help to quit tobacco use	 Moderate measure achieved	- NRT is legally available in the country. - Smoking cessation support in most health clinics and hospitals across the country.
W arning about dangers of tobacco	Health warnings on Cigarette Packages  Moderate measure achieved	Pictorial health warnings covering 90% of the principal display areas are mandated by law.
	Anti-tobacco Mass Media Campaigns  No measure or weak measure achieved	Maldives remains committed to implement mass media tobacco control campaigns in future.
E nforce bans on tobacco advertising, promotion and sponsorship	 Complete measure achieved	- Comprehensive ban on all forms of TAPS. - Achieved the highest level of achievement in bans on TAPS in GTCR 2023 and 2025.
R aise taxes on tobacco	 Moderate measure achieved	The share of total taxes in the retail price of the most widely sold brand of cigarettes in the country is 65.0%.



REPLACE IMPLEMENTATION OF WHO REPLACE TECHNICAL PACKAGE⁷

 National policy commitment to eliminate TFA: National policies, strategies or action plans that express a commitment to reduce industrially produced TFA in the food supply Achieved	 Less restrictive TFA limits: Legislative or regulatory measures that limit industrially produced TFA in foods in all settings, but are less restrictive than the recommended approach Not Achieved
 Best-practice TFA policy: Mandatory national limit of 2 g of industrially produced TFA per 100 g of total fat in all foods Not Achieved	 Other complementary measures: Legislative or other measures that encourage consumers to make healthier choices about industrially produced TFA in foods in specific settings Not Achieved

IMPLEMENTATION OF WHO SHAKE TECHNICAL PACKAGE FOR REDUCING SALT INTAKE

Technical package element		Implementation Status
Food reformulation policies	Policies that set scope and criteria for maximum limits or targets for the sodium content in prepackaged food	Not achieved
Food and nutrition labelling policies	Policies that set scope and criteria to:	
	List and declare sodium on packaged food	In process
	Regulate the use of sodium-related claims on packaged food	Not achieved
	Use front-of-pack labelling (FOPL) to provide interpretive information about sodium content	Not achieved
Food marketing restriction policies	Policies that set scope and criteria for restricting food marketing to protect children from its harmful impact on diet, including consumption of high-sodium food	Not achieved
Behavior change communication and mass media campaigns for healthy diets	A set of communication activities designed to change behaviors to reduce consumption of salt and high sodium food	Achieved
Food procurement and service policies	Policies that set scope and criteria for sodium content in food served or sold in public settings and/or procured by government for food programmes	Achieved
Taxation on unhealthy food	Policies that set scope and criteria for taxes on unhealthy food, including high-sodium food	Not achieved

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Integration of NCD services into primary health care: Faafu Atoll PHC demonstration site ¹⁻⁴

In response to the growing burden of NCDs in Maldives, the government, with support from WHO, launched the Faafu Atoll Primary Health Care Demonstration Site (FADS) in December 2022. The goal was to establish an efficient, integrated NCD care model at the primary care level by utilizing existing health infrastructure and human resources. The intervention focused on improving detection, management and follow-up care for individuals with NCDs, particularly with cardiovascular diseases, diabetes and mental health disorders.

The intervention introduced several reforms, including the implementation of the PHC Registry, the introduction of screening programmes for NCDs, the integration of mental health services and the creation of referral pathways for complex cases. Key components included regular health campaigns, clinic-based screenings, and the use of the PHC Registry for data collection and monitoring. Mental health services were integrated, with health-care workers trained in detection and referrals, using WHO's Mental Health Gap Action Programme (mhGAP) guidelines.

The intervention was implemented at Faafu Atoll Hospital in Nilandhoo and at island-level health facilities across the atoll. The target population included residents from the five islands of the atoll, with special attention paid to the elderly and individuals at high risk of NCDs such as hypertension, diabetes and mental health conditions.

A major strength of the initiative was robust community mobilization. House-to-house outreach was conducted, allowing health workers to directly engage with community members, address concerns and highlight the benefits of participation, and this approach led to a high level of community involvement. To ensure equitable access to services, mobile health units and telemedicine were used, reaching even the remotest part of the atoll. Regular follow-up visits were scheduled to maintain continuity of care, especially for individuals with chronic conditions.

The initiative achieved significant outputs: over 90% of the eligible population underwent screening for diabetes, hypertension, high cholesterol and NCD risk factors. Among those diagnosed, 88% of diabetics, 95% of hypertensive patients and 73% of individuals with high cholesterol started appropriate treatment. A Primary Health Care Register was established using the District Health Information System 2 (DHIS2), supporting early screening, continuous monitoring and development of an Island Health Profile Database. Capacity-building activities were carried out, training health-care workers on WHO's PEN Interventions and mhGAP guidelines. The initiative emphasized people-centred care, enhancing community participation and understanding of NCD risks. Key outcomes reflected notable progress in NCD management: hypertension treatment rates increased from 92% to 95.5%, diabetes treatment rates rose from 86% to 95.1% and treatment for high cholesterol from 57% to 71.6%.

The FADS project has set a strong precedent as a scalable and replicable model, leveraging the existing health-care workforce and integrating digital health technologies to enhance service delivery. It now stands as a promising template for other Small Island Developing States facing similar health challenges.



Faafu Atoll pilot project: a model for nationwide primary health care revitalization through integrated noncommunicable disease and mental health services.

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MYANMAR

NCD prevention and management



The Myanmar National Health Plan (2023–2027) identifies NCDs as a significant public health concern, requiring a multisectoral approach. Key strategies identified include advocacy and leadership for multisectoral action, health promotion, health system strengthening and evidence generation for decision-making.¹

To manage NCDs, Myanmar has implemented initiatives to introduce task-sharing among non-physician health workers. By 2020, essential service delivery using WHO PEN intervention protocols was established at the primary health care level, covering 232 out of 330 townships. Integrated guidelines for managing hypertension and diabetes, including specific medications, have been developed. In 2019, 1.67 million people were screened for NCD risk factors, with 205 495 individuals receiving diabetes treatment and 429 400 receiving hypertension treatment.²

WHO PEN training manuals, treatment guidelines and reporting formats have been created to strengthen service delivery. Diagnostic tools, such as glucometers and lipid analysers, have been introduced at PEN-implementing facilities. Myanmar has also established patient records and screening registries, with treatments and medications dispensed through primary health care centres, rural health centres (RHCs) and township hospitals. At subcentres, NCD screenings are available during Wednesday clinics, where health assistants (HAs) and basic health staff – trained to screen and prescribe medication – identify patients with hypertension. Initial treatments are provided at the PHC level, with referrals made to township medical officers (MOs), as needed.³ At rural health centres, treatment initiation is conducted by health assistants/lady health visitors (LHVs)/midwives (MWs).

Since early 2021, humanitarian response plans (HRPs) have been implemented to sustain NCD care services.



Volunteers refer patients to basic health staff for continuation of and adherence to NCD treatment

Healthy-lifestyle counselling

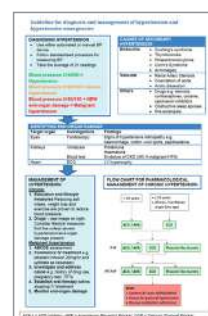


- ▶ National management protocols for hypertension and diabetes include counselling patients on tobacco use, diet, alcohol consumption and physical activity, using the 5As brief intervention approach.^{4,5}
- ▶ Country has dedicated personnel/health-care providers (such as Public Health Supervisor 2) at primary care facilities to provide healthy lifestyle counselling.

Evidence-based treatment protocols



- ▶ Evidence-based drug- and dose-specific Treatment Protocol for Medical Officers and Basic Health Staff had been used as the national treatment guideline by the Ministry of Health (MoH) since 2019; it was updated in September 2024.^{6,7}
- ▶ At least 50% of primary health care facilities in the country follow protocol-based treatment for hypertension and diabetes.⁸
- ▶ The protocol allows medication refill for a duration of 15–30 days, depending on medicine availability at the facility.



Hypertension management protocol



Diabetes management protocol

Access to medicines and technology



Medicines[#]

- ▶ The National List of Essential Medicines 2016 includes medicines (amlodipine, ARB, metformin, aspirin and rosuvastatin) that are part of the hypertension and diabetes management protocol used at the primary health care level.⁹
- ▶ These medicines are estimated to be “generally available” (available at 50% or more pharmacies) at the primary care facilities of the public health sector.⁸
- ▶ Hypertension and diabetes medicines are centrally procured by the Procurement and Distribution Division, Department of Public Health, Department of Medical Services, on an ad hoc and biannual basis.¹⁰
- ▶ Although there are temporary stockouts of medicines, a mechanism exists for local re-allocation among facilities.

Technology^{##}

- ▶ Equipment for blood pressure measurement are “generally available” at the primary care facilities of both public and private health sectors, whereas blood glucose, urine strips for ketone and glucose and albumin assay are “generally available” at primary care facilities.
- ▶ HbA1c, total cholesterol, foot vibration perception by tuning fork and dilated fundus examination testing are “generally not available” across public and private primary health care facilities.⁸

Risk-based CVD management



- ▶ Cardiovascular risk stratification is included in “Prevention of Cardiovascular Diseases: Evidence-based Treatment Protocol for Medical Officers and Basic Health Staff”.
- ▶ Over 50% of primary care facilities provide laboratory-based risk stratification.
- ▶ Statins are provided per national protocol to individuals having CVD risk score ≥ 10 .

[#] “Generally available” is defined as “at 50% or more pharmacies”.

^{##} “Generally available” is defined as “at 50% or more health-care facilities”.

Team-based care



- ▶ Treatment initiation is carried out by health assistants/lady health visitors/midwives at the rural health centre (RHC) level and medical doctors/nurse at hospitals; drug refills are facilitated by HAs/LHVs/MW at the RHC level.
- ▶ There were 70 NCD focal persons from the state/regional level, specialists (medicine) and physicians trained as trainers with updated protocol-based hypertension and diabetes care in 2024.
- ▶ Teleconsultation and social media messaging apps (Viber) are used to consult specialists regarding cases of hypertension and diabetes.

Systems for monitoring



- ▶ A mixed method – HMIS (electronic, DHIS2) and programme-specific (paper-based) at the community level – is utilized to collect and report NCD programme indicators at primary health care facilities.
- ▶ In Simple App-implementing townships, primary care facilities use unique identifiers to track patients with hypertension and diabetes.
- ▶ 25%–50% of primary health care facilities use dashboards to visualize performance and trends of key hypertension and diabetes indicators.⁸
- ▶ HMIS regularly publishes hypertension and diabetes treatment coverage data.
- ▶ The NCD Unit of the MoH collects data on patients with NCDs who attend PEN clinics operated by basic health staff at subcentres, rural health centres, station hospitals and township-level facilities. Data are compiled monthly at the township level and submitted as quarterly reports to the central level for reporting to the MoH.

Progress of hypertension and diabetes coverage and control

The country has progressed in scaling up services for coverage and control of hypertension and diabetes management.

219

health facilities are providing protocol-based management for hypertension and diabetes.

Hypertension



Diabetes



December 2024

Figure 1: Number of persons with hypertension and/or diabetes placed on protocol-based management in health care facilities/ clinics

MPOWER	Coverage	Achievements
M onitor tobacco use and prevention policies	 No measure or weak measure achieved	- Conducted DHS and GYTS in 2016. - Achieved the highest level of implementation for M in GTCR 2011, GTCR 2017, GTCR 2019 and GTCR 2021.
P rotect people from tobacco smoke	 Minimal measure achieved	Three to five public places completely smoke-free.
O ffer help to quit tobacco use	 Moderate measure achieved	Smoking cessation support available in some health clinics and hospitals across the country.
W arning about dangers of tobacco	Health warnings on Cigarette Packages  Complete measure achieved	Pictorial health warnings covering 90% of the principal display areas are mandated by law on all tobacco products, including smokeless tobacco products.
	Anti-tobacco Mass Media Campaigns  Minimal measure achieved	- Achieved best-practice in mass media campaigns in GTCR 2019 and 2021. - Conducted regular national anti-tobacco campaigns with at least 7 key features, including airing on television and/or radio.
E nforce bans on tobacco advertising, promotion and sponsorship	 Moderate measure achieved	Highest level of achievement (ban on all forms of direct and indirect advertising) in bans on TAPS in GTCR 2009 and 2011.
R aise taxes on tobacco	 Minimal measure achieved	The share of total taxes in the retail price of the most widely sold brand of cigarettes in the country is 34.1%



IMPLEMENTATION OF WHO REPLACE TECHNICAL PACKAGE¹¹

 A national policy commitment to reduce industrially produced TFA in the food supply Achieved	 Less restrictive TFA limits: Legislative or regulatory measures that limit industrially produced TFA in foods in all settings, but are less restrictive than the recommended approach Not Achieved
 Best-practice TFA policy: Legislative or regulatory measures that limit industrially produced TFA in foods in all settings and are in line with the recommended approach Not Achieved	 Other complementary measures: Legislative or other measures that encourage consumers to make healthier choices about industrially produced TFA or mandatory limits on industrially produced TFA in foods in specific settings Not Achieved

National Nutrition Centre, Ministry of Health, Myanmar, is planning to develop the National Strategic Plan for Nutrition (2025–2030). Unlike the previous Strategic Plan, the new approach aims to incorporate a comprehensive strategy for nutrition across the life-course, including food security (action towards reduction in industrially produced TFA, nutrition labelling, etc.).

IMPLEMENTATION OF WHO SHAKE TECHNICAL PACKAGE FOR REDUCING SALT INTAKE

Technical package element		Implementation Status
Food reformulation policies	Policies that set scope and criteria for maximum limits or targets for the sodium content in prepackaged food	Not achieved
Food and nutrition labelling policies	Policies that set scope and criteria to:	
	List and declare sodium on packaged food	In process
	Regulate the use of sodium-related claims on packaged food	In process
	Use front-of-pack labelling (FOPL) to provide interpretive information about sodium content	In process
Food marketing restriction policies	Policies that set scope and criteria for restricting food marketing to protect children from its harmful impact on diet, including consumption of high-sodium food	Not achieved
Behavior change communication and mass media campaigns for healthy diets	A set of communication activities designed to change behaviors to reduce consumption of salt and high sodium food	Achieved
Food procurement and service policies	Policies that set scope and criteria for sodium content in food served or sold in public settings and/or procured by government for food programmes	Not achieved
Taxation on unhealthy food	Policies that set scope and criteria for taxes on unhealthy food, including high-sodium food	Not achieved

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Breathing new life into Bagan: Transforming heritage sites into smoke-free sanctuaries for health^{1, 2}

Bagan, with its vast ancient temples and rich cultural heritage, was recognized as a UNESCO World Heritage Site in 2019. In response to rising concerns about the health risks associated with smoking, the city decided to implement a smoke-free programme with smoke-free zones within and around heritage monuments. The goal was to ensure clean air for both residents and visitors, making Bagan a healthier and more sustainable destination.

The initiative began in 2014, driven by the Policy Guidelines of Establishment of Smoke-Free Areas in the Bagan Heritage Zone. Under this policy, the district governors officially designated key areas within and around Bagan's heritage sites as 100% smoke-free. These designated zones included ancient monuments, government offices, hotels, the Nyaung U airport, pagodas, parks, restaurants, schools, temples and workplaces. A key feature of the policy was a strong stance against any engagement with the tobacco industry.

Enforcement was carried out by the Ministry of Health and regional government with the support of local law enforcement and community leaders.

By May 2023, Bagan had successfully established 102 smoke-free zones, covering the city's most important public and cultural spaces. These efforts were coordinated through the "Smoke-Free Bagan" initiative, delivered in collaboration with various stakeholders such as the People's Health Foundation, the Association of Hotels (Bagan Zone), pagoda and temple trustees, NGOs and the public.

Task forces were formed to enforce the Tobacco Products Control Law, with each group assigned specific roles. Regular inspections ensured compliance, while action plans and schedules were shared with the Supervision Committee to guide implementation. Public awareness was raised through mass media campaigns, supported by visible signage, such as "No Smoking" posters, graphic health warnings and billboards, including a prominent one at the city entrance, bearing the message, "SMOKE-FREE BAGAN – OUR HERITAGE OUR PRIDE." Efforts also extended beyond the heritage zone, with local authorities working to curb smoking in public areas and promote the use of designated smoking spaces.

The initiative was supported by strong coordination between the District Governor, local government departments and social organizations. A Supervisory Committee provided continuous oversight, ensuring accountability and sustained momentum.

In recognition of its pioneering efforts, Bagan was officially declared a smoke-free city in May 2023. This achievement was celebrated even further when Bagan received the ASEAN Smoke-Free Award in August 2024, honouring its exemplary policies and positioning it as a model for other cities. The success of this initiative reflects the power of collaboration – between the Ministry of Health, WHO, UNESCO, Bagan Heritage Conservation Committee and local businesses – in promoting public health while preserving cultural treasures.



Tobacco control health warning signage at heritage site in Bagan

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NEPAL

NCD prevention and management



Nepal has taken significant steps to prevent and manage NCDs through its primary health care system. The Multisectoral Action Plan for the Prevention and Control of NCDs (2021–2025) was recently launched with the aim of strengthening NCD prevention and control nationwide.¹ This comprehensive plan focuses on multisectoral collaboration to address NCDs through improved service delivery, early detection and management.

In 2017, Nepal became the first country in the Region to adopt the global HEARTS tool to enhance monitoring systems for WHO PEN services.² By 2021, the PEN programme had expanded to 51 districts and now covers all districts in Nepal. The country introduced task-sharing initiatives, empowering non-physician health workers to manage NCDs effectively. Medical officers at primary health care centres and health assistants at health posts have been designated as PEN focal points. Additionally, cascade training programmes were conducted to ensure that most primary health care workers are equipped with the WHO PEN approach. Nepal has also introduced diagnostic tools such as glucometers and lipid analysers at PEN-implementing facilities – this has improved the quality of NCD screening and strengthened referral systems.³

In 2023, Nepal launched the Hypertension Care Cascade Initiative (HCCI) on World Hypertension Day in Kavrepalanchowk District. This programme focuses on reducing the burden of hypertension by enhancing detection, treatment and follow-up care at the primary health care level. It is guided by evidence-based protocols and emphasizes people-centred care, with active commitment from all levels of government. The initiative has been scaled up to more than three districts.⁴

National commitment: The country announced its commitment in May 2023 that it will place 1.5 million people with hypertension or diabetes on protocol-based care by 2025.⁵



Awareness rally organised on World Hypertension Day at Dhulikhel, Nepal

Healthy-lifestyle counselling

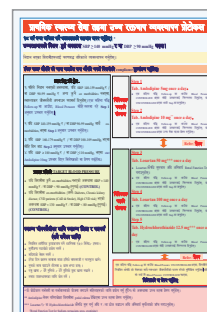


- National management protocols for hypertension and diabetes include counselling patients on tobacco use, diet, alcohol consumption and physical activity, using the 5As brief intervention approach.

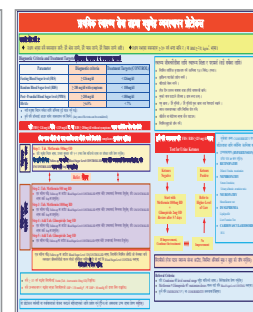
Evidence-based treatment protocols



- Evidence-based drug- and dose-specific national protocols for management of hypertension and diabetes in primary health care, aligned with WHO HEARTS and PEN technical packages, are being implemented since 2023.⁶
- According to the protocol, there is a provision for medication refill for a duration of 30 days.



Hypertension management protocol



Diabetes management protocol

Access to medicines and technology



Medicines[#]

- The National List of Essential Medicines Nepal, 2021, includes medicines (amlodipine, ARB, thiazides, metformin and glimepiride) that are part of the hypertension and diabetes management protocol used at the primary health care level.⁷
- Hypertension and diabetes management protocol medicines are estimated to be “generally available” (available at 50% or more pharmacies) at the primary care facilities of the public health sector.⁸
- An electronic logistics management information system is in place to support the forecasting, procurement, distribution and monitoring of medicines across health facilities.⁹
- Medicines are procured annually through a mix of centralized and decentralized mechanisms by the Logistic Management Section at the federal level, the Province Health Logistics Management Centre at the province level and the Basic Hospital at the local level.⁹
- More than 75% of primary health care facilities in the public sector reported stockout of hypertension and diabetes medicines in the past one year that are listed in the national management protocol.

Technology^{##}

- Equipment for measuring blood glucose, blood pressure, urine strips for ketone and glucose are estimated to be “generally available” across public and private primary health care facilities.⁸
- Total cholesterol, HDL and LDL, serum creatine and urea, and urine strips for albumin assay testing are reported to be “generally available” at the primary care facilities of the private health sector only. HbA1c is not available.

Risk-based CVD management



- Cardiovascular risk stratification is included in the nationally approved protocol of hypertension and diabetes management. CVD risk charts have been provided to many primary health care centres and health posts after PEN trainings.
- 25%–50% of primary care facilities provide non-laboratory-based risk stratification where there is no laboratory facility.⁸

[#] “Generally available” is defined as “at 50% or more pharmacies”.

^{##} “Generally available” is defined as “at 50% or more health-care facilities”.

Team-based care



- ▶ At the village level, services are delivered through health posts staffed by health assistants (HA), auxiliary health workers (AHWs) and auxiliary nurse midwives (ANMs). Doctors, staff nurses and laboratory staff are available at primary health centres. Team-based care has been much more advanced after the Hypertension Care Cascade Initiative in Nepal (HCCIN) was implemented in districts and municipalities.
- ▶ Health posts provide screening for hypertension and diabetes, CVD risk assessment, counselling, referral, follow-up of low-risk patients, drug refills and health promotion. Primary health centres confirm diagnosis, manage and follow up on high-risk patients, and refer complicated cases to district hospitals.
- ▶ Community health workers and professionals work together to screen, detect and manage hypertension and diabetes through health posts and in PHCs.
- ▶ Treatment initiation is carried out by health assistants, while drug refills are facilitated by health assistants and/or pharmacists/nurses.
- ▶ 917 health-care providers were trained in protocol-based hypertension and diabetes care in 2024.
- ▶ The provision for health-care providers at primary health facilities to consult specialists regarding cases of hypertension and diabetes via social media messaging platforms such as WhatsApp is not systematically appropriate. Routine teleconsultation services between community clinics and higher-level facilities are not available.

Systems for monitoring



- ▶ A mixed system of paper-based and electronic system is utilized to collect and report NCD programme indicators at primary health care facilities.
- ▶ Most primary health care facilities lack systems to track hypertension and diabetes patients using unique IDs, except in select districts such as Kavre, Parsa and Kailali, and some municipalities in Ilam, Sarlahi, Surkhet, Kaski and Tanahu, where HCCIN is implemented.⁸
- ▶ Kavre and Parsa districts' primary health care facilities use dashboards to visualize performance and trends of key hypertension and diabetes indicators.
- ▶ Country has an electronic system (DHIS2-based) to facilitate reporting of aggregated patient data on NCDs from primary health facilities to district/subnational and national levels.
- ▶ The NCD information system in primary health care integrates patient registration, diagnosis, treatment and follow-up using standardized tools such as PEN and mhGAP registers. Data are collected manually and digitally, aiding decision-making. Challenges include fragmented reporting and limited interoperability, but efforts such as dashboards and dose-specific treatment protocols enhance monitoring and care delivery.

Progress of hypertension and diabetes coverage and control

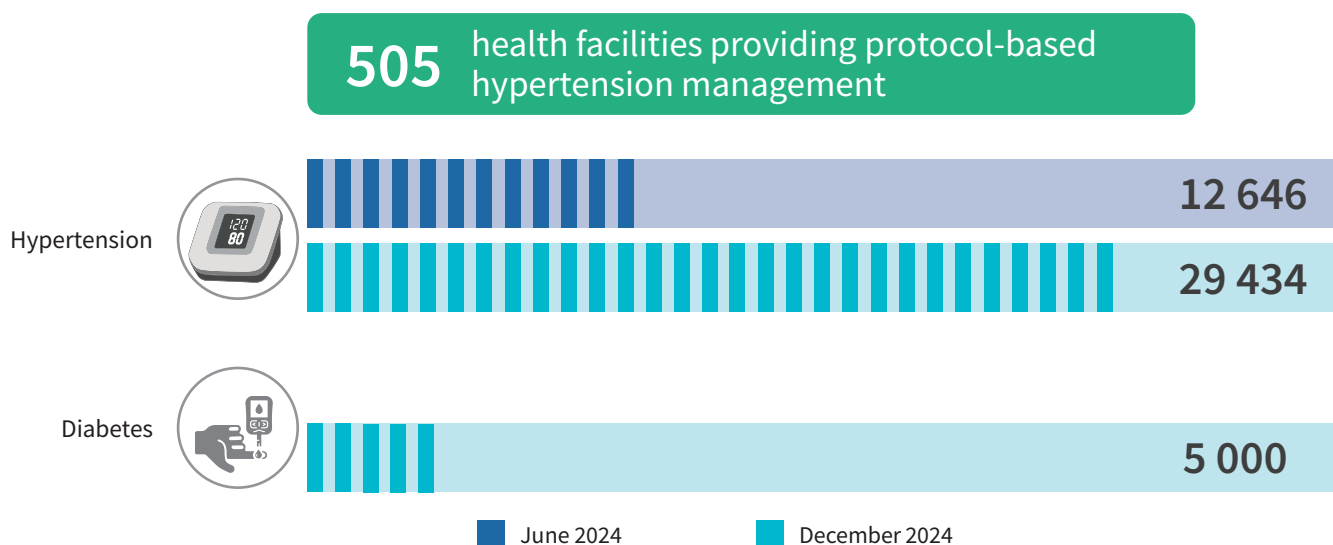


Figure 1: Number of persons with hypertension and/or diabetes placed on protocol-based management in health care facilities/ clinics (2024)

MPOWER		Coverage	Achievements
M	onitor tobacco use and prevention policies	 Minimal measure achieved	- Regularly monitoring the use of tobacco products among adults. - Achieved the highest level of implementation in M in GTCR 2015. - The Tobacco Questions for Surveys (TQS) are being integrated in national health surveys or multiple risk factor surveys.
	rotect people from tobacco smoke	 Complete measure achieved	All the public places are completely smoke-free.
O	ffer help to quit tobacco use	 Moderate measure achieved	- The Ministry of Health and Population has prioritized expansion of tobacco cessation services in the country. - The WHO Country Office provided technical support to develop the protocol and guidelines and recently conducted the training of trainers to build capacity for tobacco cessation services.
W	arning about dangers of tobacco	 Complete measure achieved	Pictorial health warnings covering 90% of the principal display areas are mandated by law.
	Anti-tobacco Mass Media Campaigns	 Moderate measure achieved	Achieved best-practice in mass media campaigns in GTCR 2015 and 2017 by conducting regular national-level anti-tobacco campaigns including airing on television and/or radio.
E	nforce bans on tobacco advertising, promotion and sponsorship	 Complete measure achieved	Comprehensive ban on all forms of TAPS.
R	aise taxes on tobacco	 Minimal measure achieved	The share of total taxes in the retail price of the most widely sold brand of cigarettes in the country is 31.9%; a health hazard tax is levied.





REPLACE
TRANS FAT

IMPLEMENTATION OF WHO REPLACE TECHNICAL PACKAGE¹⁰



Best-practice TFA policy: Legislative or regulatory measures that limit industrially produced TFA in foods in all settings and are in line with the recommended approach

Achieved



Monitoring mechanism for mandatory TFA limits

Not achieved



Other complementary measures: Legislative or other measures that encourage consumers to make healthier choices about industrially produced TFA or mandatory limits on industrially produced TFA in foods in specific settings

Not achieved



IMPLEMENTATION OF WHO SHAKE TECHNICAL PACKAGE FOR REDUCING SALT INTAKE

Technical package element		Implementation Status
Food reformulation policies	Policies that set scope and criteria for maximum limits or targets for the sodium content in prepackaged food	Not achieved
Food and nutrition labelling policies	Policies that set scope and criteria to:	List and declare sodium on packaged food
		Regulate the use of sodium-related claims on packaged food
		Use front-of-pack labelling (FOPL) to provide interpretive information about sodium content
Food marketing restriction policies	Policies that set scope and criteria for restricting food marketing to protect children from its harmful impact on diet, including consumption of high-sodium food	Not achieved
Behavior change communication and mass media campaigns for healthy diets	A set of communication activities designed to change behaviors to reduce consumption of salt and high sodium food	Achieved
Food procurement and service policies	Policies that set scope and criteria for sodium content in food served or sold in public settings and/or procured by government for food programmes	Not achieved
Taxation on unhealthy food	Policies that set scope and criteria for taxes on unhealthy food, including high-sodium food	Not achieved

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Municipal ownership for the advancement of NCD service through hypertension as an indicator in Kavre and Parsa districts of Nepal¹⁻³

Nepal has shown unwavering commitment to strengthening NCD prevention and control by focusing its efforts on people-centred primary health care and universal health coverage. With nearly one in four adults affected by hypertension – and less than 5% of those diagnosed have the condition under control – Nepal is taking bold steps to close the gap between detection and effective treatment. To support these efforts, Nepal adapted the WHO PEN intervention protocols to its local context in December 2015. These standardized tools have since improved hypertension and diabetes management by enabling health workers to provide protocol-based care at PHC facilities. Improved access to essential medicines and basic diagnostics at these centres has been pivotal in this transformation.

In May 2023, the Hypertension Care Cascade Initiative was launched to shift hypertension management away from overburdened tertiary hospitals towards more accessible primary health care settings. Led by the Ministry of Health and Population (MoHP) and supported by the Norwegian Agency for Development Cooperation (NORAD) and the WHO Country Office for Nepal, the project was piloted in Kavrepalanchowk District and is now expanding to Parsa.

The Kavre pilot was the result of a collaboration between the Kavre District Health Office, the Provincial Health Directorate (Hetauda), the Epidemiology and Disease Control Division (EDCD) of the Department of Health Services, Kathmandu University School of Medical Sciences and WHO. A key aspect was the intensive, hands-on training for over 200 PHC workers associated with 165 local health facilities. The training focused on hypertension care protocols, including accurate blood pressure measurement, patient monitoring, treatment initiation, medication management and regular follow-ups.

Notably, the programme emphasized the importance of proper documentation protocols, enabling accurate and complete recording of patient information and effective follow-up, starting from the point of diagnosis. WHO supported the effort by providing digital blood pressure monitors to each participating facility. Around 50 health workers were trained. Since the completion of the training, Roshi Municipality has conducted free health screenings in local communities, purchased essential medicines, and initiated the use of better reporting and patient tracking tools and treatments, as per updated protocols. Following the success of the training in Roshi Municipality, other municipalities in Kavrepalanchowk have also allocated budget for training their health-care workers.

The MoHP now plans to replicate the Hypertension Care Cascade Initiative in other districts, including Parsa, aiming to provide protocol-based hypertension and diabetes services to 1.5 million people by 2025. The initiative represents a model of integrated NCD service delivery driven by municipal leadership, community involvement and robust technical support.



Orientation session of health workers at Bethanchok Rural Municipality, Nepal

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SRI LANKA

NCD prevention and management



Sri Lanka has a well-established public health system for NCDs that prioritizes prevention; it is supported by healthy lifestyle centres (HLCs) and well women clinics. The National Multisectoral Action Plan for the Prevention and Control of NCDs (2023–2027) outlines a comprehensive approach to NCD prevention and management, emphasizing multisectoral collaboration and integration of NCD care into primary health care.¹ This aligns with the country’s Health Master Plan (2016–2025), which underscores the importance of promoting healthy lifestyles and improving primary care services.²

The HLCs, introduced in 2011, focus on reducing NCD risks for individuals aged over 35 years through early detection of risk factors and providing access to specialized care, particularly for those at higher cardiovascular disease risk. Using a total-risk approach and the WHO/International Society of Hypertension risk prediction chart, HLCs assess clients’ 10-year CVD risk and provide appropriate management. Early evaluations of the programme identified gaps in protocol adherence, inclusion of all NCDs and human resource capacity, which the subsequent action plan aimed to address.

Sri Lanka has introduced and updated several clinical guidelines for managing cancer, cardiovascular disease, hypertension, diabetes and cancer. These include “Guidelines for management of diabetes mellitus for primary health care providers 2018”, “National guidelines for management of hypertension for primary health-care providers 2021”, “Manual for screening of noncommunicable diseases 2021”, “Referral criteria from primary health care to secondary or tertiary health care for NCD patients” and “National guidelines on clinical management of diabetes, hypertension, dyslipidemia, cardiovascular diseases for secondary/tertiary health-care providers in 2021”.³ Furthermore, in 2024, the guidelines and protocols on the management of hypertension and diabetes were updated.

These guidelines enhanced patient-centred care by incorporating multidisciplinary teams and evidence-based, cost-effective treatment pathways.⁴ Diabetes guidelines emphasized the role of primary health care providers in delivering effective care. The guidelines also addressed the need for guidance on healthy lifestyles. The district-level NCD unit oversees the implementation and management of NCD programmes, ensuring alignment with national goals. Programme reviews are conducted regularly, with monthly evaluations led by district medical officers for NCDs and quarterly reviews involving hospital staff at regional and central levels. This systematic review process strengthens programme accountability and fosters continuous improvement in NCD prevention and management efforts.

National commitment: To put 1 000 000 people with hypertension and diabetes on protocol-based management by the end of 2025; the target is also to increase the effective control of hypertension from 30% in 2021 to 60% by 2027. Similarly, the effective control of diabetes will be increased from 25% in 2025 to 60% by 2030.



Community screening in Sri Lanka: Health workers measuring blood pressure for early detection and management of hypertension

Healthy-lifestyle counselling

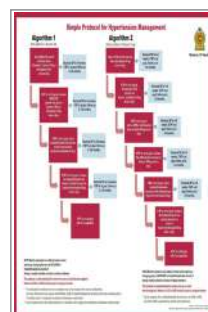


- ▶ National management protocols for hypertension and diabetes include counselling patients on tobacco use, diet, alcohol consumption, physical activity and healthy eating habits, using the 5As brief intervention approach.⁵
- ▶ There are plans to introduce a health promotion officer to facilitate lifestyle counselling at individual and community levels. At present, medical officers, nursing officers and public health nursing officers provide lifestyle guidance and counselling.

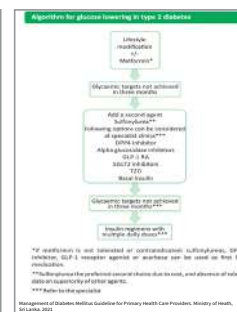
Evidence-based treatment protocols



- ▶ Evidence-based drug- and dose-specific national protocols for management of hypertension and diabetes in primary health care, aligned with WHO HEARTS and PEN technical packages, are being implemented since 2022.^{6, 7}
- ▶ Currently, six out of 25 districts are implementing protocol-based hypertension and diabetes management.
- ▶ According to the protocol, there is a provision for medication refill for a duration of 30 days or 60 days (based on drug availability), if the patient is under control.



Hypertension management protocol



Diabetes management protocol

Access to medicines and technology



Medicines[#]

- ▶ The Essential List 2022 includes medicines (amlodipine, losartan, hydrochlorothiazide, metformin, sulphonyl urea) that are part of the hypertension and diabetes management protocol used at the primary health care level.⁸
- ▶ These medicines are estimated to be “generally available” (available at 50% or more pharmacies) at the primary care facilities of the public health sector.⁹
- ▶ An electronic logistics management information system (LMIS) is in place to support procurement, distribution and monitoring of medicines across health facilities.¹⁰
- ▶ It is noteworthy that all the medicines indicated for protocol-based management are procured through buy-back guarantees from local manufacturers. When there is a stockout situation, the regions purchase drugs locally under the approval of the Medical Supplies Division.
- ▶ All primary health care facilities in the public sector reported stockout of hypertension and diabetes medicines in the past one year.

Technology^{##}

- ▶ Equipment for measuring blood glucose, blood pressure, urine strips for glucose are “generally available” at primary health facilities of both public and private health sectors.⁹
- ▶ HbA1c, urine strips for albumin assay testing are reported to be “generally not available” at primary health care facilities of the public and private health sectors.
- ▶ Testing for total cholesterol is reported to be “generally available” at primary care facilities of the public health sector only.

Risk-based CVD management



- ▶ Cardiovascular risk stratification is included in the nationally approved protocol for hypertension and diabetes management.¹¹
- ▶ More than 50% of primary care facilities provide laboratory-based risk stratification.⁹
- ▶ There is a diabetes protocol to provide statin to people having a 10-cardiovascular risk ≥ 20 .

[#] “Generally available” is defined as “at 50% or more pharmacies”.

^{##} “Generally available” is defined as “at 50% or more health-care facilities”.

Team-based care



- ▶ Country has established healthy lifestyle centres (HLCs) for NCD screening and health guidance at primary health care facilities and hospitals, which provide client-friendly services to serve hard-to-reach areas, according to priority to special groups.
- ▶ With the introduction of validated blood pressure monitoring at the primary care level, other staff members are involved in blood pressure measurement, which was exclusively carried out by medical officers earlier. Additionally, the Public Health Nursing Officer visits patients' houses, if they miss more than two appointments for clinic visit. In debilitated patients, medications are delivered to their homes by public health nursing officers. The nursing officers are trained on foot examination and wound care.
- ▶ Treatment initiation is carried out by medical doctors, while drug refills are facilitated by a pharmacist/dispenser following the approval of a medical doctor.
- ▶ 400 health-care providers have been trained in protocol-based hypertension and diabetes care in 2024.
- ▶ Provision is available at some primary care institutions for health-care providers at primary health facilities to consult specialists on cases of hypertension and diabetes via telemedicine. Routine teleconsultation services between community clinics and higher-level facilities are not available everywhere.

Systems for monitoring

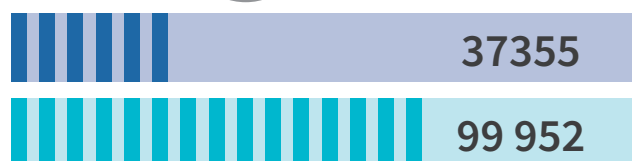


- ▶ A mixed system of paper-based (100%) and electronic systems (50%) is utilized to collect and report NCD programme indicators at primary health care facilities.⁹
- ▶ The patient information system at primary health care facilities with protocol-based management is capable of identifying and tracking hypertension and diabetes patients using a unique identifier during each visit at selected primary health care facilities.
- ▶ 25%–50% of primary health care facilities use dashboards to visualize performance and trends of key hypertension and diabetes indicators.
- ▶ There is still no mechanism/provision at the national level to publish the hypertension and diabetes treatment coverage data on a regular basis in the public domain.
- ▶ There are two information systems available at primary care institutions. The Simple App, which is a mobile app, is used to enter individual patient data at the clinic level. Another cloud-based application is available to enter data of patients attending healthy lifestyle centres. This app is not working well, as offline data entry is not available.

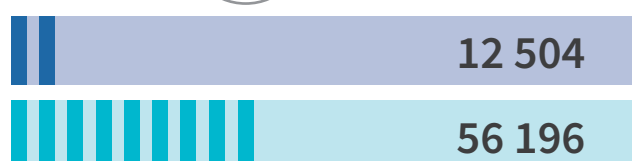
Progress of hypertension and diabetes coverage and control

The country has progressed in scaling up services for coverage and control of hypertension and diabetes management

Hypertension



Diabetes



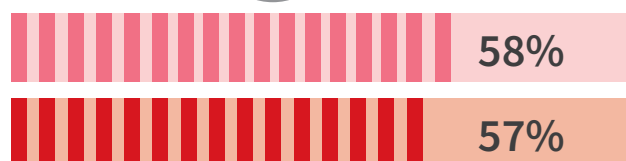
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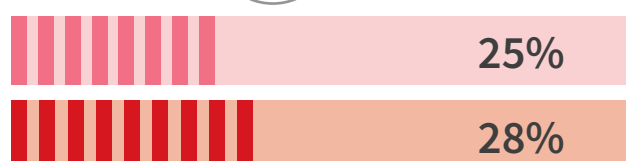
Figure 1: Number of persons with hypertension and/or diabetes placed on protocol-based management in health care facilities/clinics (2024)

200 PHCs providing protocol-based management for hypertension and diabetes

Hypertension



Diabetes



June 2024

December 2024

Figure 2: Hypertension and diabetes control status among people on treatment in health care facilities/clinics (2024)

MPOWER		Coverage	Achievements
M onitor tobacco use and prevention policies		 Minimal measure achieved	- Regularly monitored the use of tobacco products among adults. - Achieved the highest level of implementation in M in GTCR 2023. - Tobacco Questions for Surveys (TQS) are integrated in national health surveys or multiple risk factor surveys.
P rotect people from tobacco smoke		 Moderate measure achieved	- Six to seven public places completely smoke-free. - The tobacco-free zone reactivation programme, spearheaded by the National Authority on Tobacco and Alcohol (NATA) and funded by WHO, successfully revitalized efforts to establish tobacco-free zones across Sri Lanka.
O ffer help to quit tobacco use		 Moderate measure achieved	A “functional” national toll-free tobacco quitline is in place. Some smoking cessation support is available in the community.
W arning about dangers of tobacco	Health warnings on Cigarette Packages	 Complete measure achieved	Pictorial health warnings covering 80% of the principal display areas are mandated by law.
	Anti-tobacco Mass Media Campaigns	 Moderate measure achieved	Achieved best-practice in mass media campaigns in GTCR 2023 by conducting regular national-level anti-tobacco campaigns with at least 7 appropriate characteristics, including airing on television and/or radio.
E nforce bans on tobacco advertising, promotion and sponsorship		 Moderate measure achieved	Achieved the highest level of achievement (ban on all forms of direct and indirect advertising) in bans on TAPS in GTCR 2009.
R aise taxes on tobacco		 Moderate measure achieved	The share of total taxes in the retail price of the most widely sold brand of cigarettes in the country is 68.4%.





REPLACE
TRANS FAT

IMPLEMENTATION OF WHO REPLACE TECHNICAL PACKAGE¹²



Best-practice TFA policy: Legislative or regulatory measures that limit industrially produced TFA in foods in all settings and are in line with the recommended approach

Achieved



Monitoring mechanism for mandatory TFA limits

Not Achieved



Other complementary measures: Legislative or other measures that encourage consumers to make healthier choices about industrially produced TFA or mandatory limits on industrially produced TFA in foods in specific settings (mandatory declaration of TFA on nutrition labels)

Achieved



IMPLEMENTATION OF WHO SHAKE TECHNICAL PACKAGE FOR REDUCING SALT INTAKE

Technical package element	Implementation Status	
Food reformulation policies	Policies that set scope and criteria for maximum limits or targets for the sodium content in prepackaged food	Not achieved
Food and nutrition labelling policies	Policies that set scope and criteria to:	
	List and declare sodium on packaged food	Achieved
	Regulate the use of sodium-related claims on packaged food	Not achieved
Food marketing restriction policies	Use front-of-pack labelling (FOPL) to provide interpretive information about sodium content	Achieved
	Policies that set scope and criteria for restricting food marketing to protect children from its harmful impact on diet, including consumption of high-sodium food	Achieved
Behavior change communication and mass media campaigns for healthy diets	A set of communication activities designed to change behaviors to reduce consumption of salt and high sodium food	Achieved
Food procurement and service policies	Policies that set scope and criteria for sodium content in food served or sold in public settings and/or procured by government for food programmes	In progress
Taxation on unhealthy food	Policies that set scope and criteria for taxes on unhealthy food, including high-sodium food	In progress

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Turning the tide on hypertension control in Sri Lanka: A data-driven quality improvement approach leads to program success^{1,2}

Hypertension remains a pressing public health issue in Sri Lanka, where out of the country's 22 million people, 4.3 million have high blood pressure. Yet, only 16% have it under control, leaving the majority at risk of serious cardiovascular complications. In response, the Ministry of Health (MoH) launched a national initiative to improve hypertension management through the adoption of the WHO HEARTS technical package. At the heart of this initiative was a commitment to data-driven quality improvement. The MoH developed standardized protocols for managing hypertension and diabetes, tailored to the Sri Lankan primary health care system and aligned with WHO guidelines. These protocols were reviewed and endorsed by national academic institutions, and rolled out through comprehensive training programs for over 600 medical and nursing officers.

To strengthen program implementation, Sri Lanka's National Hypertension Programme managers participated in Resolve to Save Lives's Quality Improvement Academy. Over three months, 23 leaders from eight countries received virtual training in data analysis, identifying gaps in care, testing targeted interventions and using regular feedback to improve performance. Each country developed annual action plans targeting key drivers of blood pressure (BP) control: drug supply, missed visits, decentralizing care and medication titration.

Central to monitoring progress was the HEARTS360 dashboard, pre-configured with WHO HEARTS indicators. In Sri Lanka, data were collected using the customized Simple App, enabling real-time tracking of facility-level performance. Monthly meetings with national leaders and quarterly review meetings, involving the Ministry of Health, district officials and facility staff, created a strong accountability framework and a platform for peer learning.

The intervention was successfully implemented across several districts, including 34 primary care institutions in the Kalutara district, 43 in the Colombo district, 78 in the Kandy district, 34 in the Matale district, 47 in the Nuwara Eliya district and 44 in the Galle district. Each facility was equipped with validated digital blood pressure devices and smartphones for data entry. Training on accurate BP measurement and app usage ensured consistent implementation. Annual audits and patient experience surveys further supported continuous improvement. Effective BP control among enrolled patients rose from 30% to 60%, with some institutions achieving control rates nearing 80%. At hospitals that recently joined, the control rate remains around 30%. The diabetes protocol, introduced more recently, has shown modest improvements, with current control rates also estimated around 30%. The rate of medication titration increased from a mere 2% to approximately 10%.

There was a collaborative effort undertaken by the Deputy Director-General (NCD) and Director (NCD), College of Internal Medicine, College of Family Medicine, Provincial Health Services, Resolve to Save Lives and WHO.



Training programme at regional director of health services Galle and a medical officer consulting a patient and making entries in the software

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THAILAND

NCD prevention and management



Thailand is strongly committed to achieving universal health coverage, focusing on mitigating NCD risk factors and ensuring delivery of high-quality NCD care services. The National NCDs Prevention and Control Plan aims to reduce the impact of NCDs through collaborative efforts across all levels, contributing to improved public health outcomes and economic development.

Thailand has made significant strides in implementing WHO-recommended “best buys” interventions, outlined in the MPOWER, SHAKE and REPLACE packages. These efforts are supported by national strategies such as the National Plan for the Prevention and Control of Noncommunicable Diseases, the Multisectoral NCD Strategy, the Sodium Reduction Strategy, the Tobacco Control Plan, the Alcohol Control Plan, and the Physical Activity Strategy. Many of these achievements have been recognized globally as good practices, serving as models for other countries.¹

As part of primary prevention, Thailand emphasizes health promotion and disease prevention. Key initiatives include population-wide screening for diabetes and hypertension, behaviour modification for individuals with intermediate hyperglycaemia and at pre-hypertensive stages, cardiovascular disease risk assessments and home blood pressure monitoring. Opportunistic blood pressure screening is offered to all individuals visiting health-care facilities.² Primary health care infrastructure consists of a district hospital together with a number of health centres at the subdistrict level for the reporting purpose mentioned below.

Thailand has achieved high rates of diabetes and hypertension screening, with a significant proportion of individuals successfully managing their conditions.³ The use of a mobile application streamlines the screening process, while the availability of essential medications and technological innovations ensures broad coverage and accessibility of NCD care services across the population.

National commitment: The country announced its commitment in May 2023 to improve the hypertension cascade and achieve a target control rate of 60% through innovation.⁴



Health Station – a local healthcare facility in Thailand providing essential health services to the community.

Healthy-lifestyle counselling



- ▶ National management protocols for hypertension and diabetes include counselling patients on tobacco use, diet, alcohol consumption and physical activity, using the 5As brief intervention approach.
- ▶ Country has dedicated personnel/health-care providers (such as counsellors) at primary care facilities to provide healthy lifestyle counselling.

Evidence-based treatment protocols



- ▶ Evidence-based drug- and dose-specific national protocols for management of hypertension and diabetes in primary health care, aligned with WHO HEARTS and PEN technical packages, are being implemented. These include the 2024 Thai Guideline on the treatment of hypertension by the Thai Hypertension Society and 2023 Clinical Practice Guideline for Diabetes.^{5,6}
- ▶ More than 50% of primary health care facilities in the country follow protocol-based treatment for hypertension and diabetes.⁷
- ▶ The average follow-up time for refills for patients with good BP control varies from eight to 24 weeks for provincial hospitals and four to 12 weeks for district hospitals.



Hypertension management protocol



Diabetes management protocol

Access to medicines and technology



Medicines[#]

- ▶ The National List of Essential Medicines 2021 includes medicines (angiotensin converting enzyme inhibitors, angiotensin receptor blockers, beta-blockers, calcium-channel blockers, diuretics, metformin, sulphonyl ureas, insulin and glitazone) that are part of the hypertension and diabetes management protocol used at the primary health care level.⁸
- ▶ These medicines are estimated to be “generally available” (available at 50% or more pharmacies) at the primary care facilities of the public health sector.⁷
- ▶ There is an electronic logistics management information system (LMIS) for procurement, distribution and monitoring of medicines at health facilities.
- ▶ Medicines are procured on an ad hoc basis through a mix of centralized and decentralized mechanisms. Each hospital can procure through a decentralized mechanism and centrally through the National Health Security Office, Social Security Office, Department of Disease Control.⁹

Technology^{##}

- ▶ Equipment for measuring blood glucose, blood pressure, urine strips for ketone and glucose, foot vibration perception by tuning fork are estimated to be “generally available” across public and private primary health care facilities, whereas dilated fundus examination is “generally available” at primary care facilities of the private health sector only.⁷
- ▶ HbA1c, total cholesterol, urine strips for albumin assay testing are reported to be “generally available” at primary care facilities of the private health sector only.

Risk-based CVD management



- ▶ Cardiovascular risk stratification is included in the nationally approved protocol of hypertension and diabetes management. This led to early detection, treatment and timely prevention.
- ▶ Over 50% of primary care facilities provide laboratory-/non-laboratory-based risk stratification.⁷
- ▶ Clinical Practice Guideline on Management of Dyslipidemia for Atherosclerotic Cardiovascular Disease Prevention 2024 provides statin to people having a 10-cardiovascular risk ≥ 20 .

[#] “Generally available” is defined as “at 50% or more pharmacies”.

^{##} “Generally available” is defined as “at 50% or more health-care facilities”.

Team-based care



- ▶ Primary care is provided through networks of health centres – mostly at the subdistrict (tambon) level – termed “tambon health-promoting hospitals” and staffed by non-physician staff, such as nurses or public health officers. At the village level, there are about 10 village health volunteers (VHVs) for each village who provide health information/education to the community and support health centre staff in conducting community-based screening and other activities. VHVs also support patients and families in providing long-term care.
- ▶ Treatment initiation is carried out by medical doctors, while drugs refills are facilitated by pharmacists with doctor prescriptions.
- ▶ 100% of health-care providers have been trained in protocol-based hypertension and diabetes care in 2024.

Systems for monitoring



- ▶ An electronic system is utilized to collect and report NCD programme indicators at primary health care facilities.
- ▶ The patient information system at primary health care facilities is capable of identifying and tracking hypertension and diabetes patients using a unique identifier during each visit.
- ▶ More than 75% of primary health care facilities use dashboards to visualize performance and trends of key hypertension and diabetes indicators.
- ▶ There is a mechanism/provision at the national level to publish the hypertension and diabetes treatment coverage data on a regular basis in the public domain.¹⁰
- ▶ A standardized system for recording patient-level data that includes NCD status and risk factors is estimated to be existing in >75% of primary health care facilities.⁷
- ▶ Thailand has implemented a comprehensive digital health system, with the Health Data Centre (HDC) dashboard at its core. This web portal, developed by the Ministry of Public Health’s Health Data Centre, offers a wide range of health-related data and information for the country. The HDC dashboard provides access to reports on various aspects of public health, including population statistics, health personnel, health status, health-care service utilization, disease prevention and health promotion.¹¹

Progress of hypertension and diabetes coverage and control

100% health facilities are providing protocol-based management for hypertension and diabetes

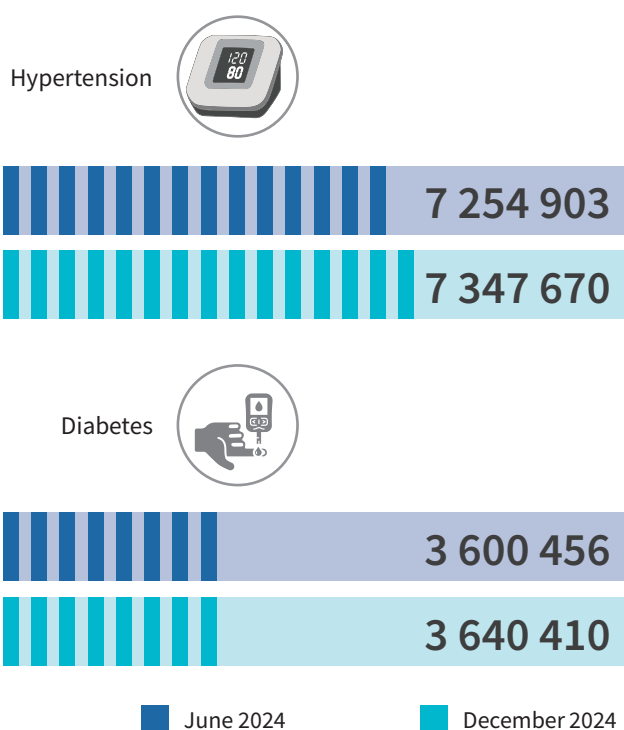


Figure 1: Number of persons with hypertension and/or diabetes placed on protocol-based management in health care facilities/clinics (2024)

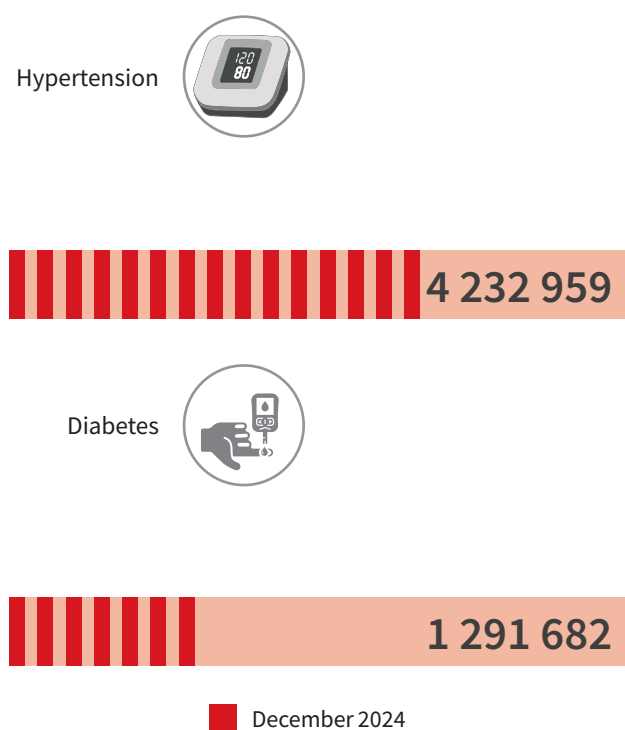
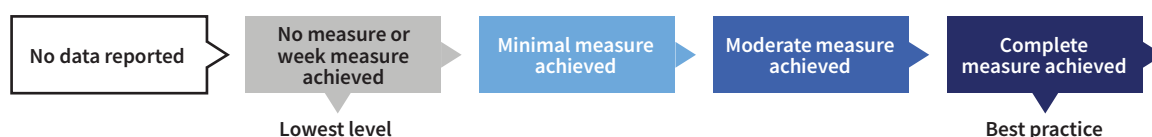


Figure 2: Hypertension and diabetes control status among people on treatment in health care facilities/clinics (2024)

MPOWER		Coverage	Achievements
M onitor tobacco use and prevention policies		 Complete measure achieved	- Thailand monitors the use of tobacco among adults and youth. - Achieved the highest level of implementation for M in GTCR 2023 and 2025. - The Tobacco Questions for Surveys (TQS) are integrated in national health surveys or multiple risk factor surveys.
P rotect people from tobacco smoke		 Complete measure achieved	- All public places completely smoke-free. - Continuously achieved the highest level of implementation in P.
O ffer help to quit tobacco use		 Moderate measure achieved	- Achieved the highest level of implementation in O in GTCR 2013. - It has a functional national toll-free quitline in place which provides quality cessation services at the population level. NRT is legally sold across the country.
W arning about dangers of tobacco	Health warnings on Cigarette Packages	 Complete measure achieved	Thailand tobacco control law mandates plain packaging of tobacco products.
	Anti-tobacco Mass Media Campaigns	 Complete measure achieved	- Achieved best-practice in mass media campaigns in GTCR 2025 (latest). - Conducted regular national-level anti-tobacco campaigns with at least 7 key features, including airing on television and/or radio.
E nforce bans on tobacco advertising, promotion and sponsorship		 Moderate measure achieved	Highest level of achievement (ban on all forms of direct and indirect advertising) in bans on TAPS in GTCR 2009 and 2011.
R aise taxes on tobacco		 Complete measure achieved	- The share of total taxes in the retail price of the most widely sold brand of cigarettes in the country is 78.6%. - The Health Promotion Foundation Act in 2001 empowers ThaiHealth to collect 2% levy on tobacco and alcohol excise tax.

MPOWER Key





REPLACE
TRANS FAT

IMPLEMENTATION OF WHO REPLACE TECHNICAL PACKAGE¹²

Best-practice TFA policy: Legislative or regulatory measures that limit industrially produced TFA in foods in all settings and are in line with the recommended approach

Achieved (imposed a ban on industrially produced partially hydrogenated oil in 2018)

Other complementary measures: Legislative or other measures that encourage consumers to make healthier choices about industrially produced TFA or mandatory limits on industrially produced TFA in foods in specific settings (mandatory declaration of TFA on nutrition labels)

Achieved

Monitoring mechanism for mandatory TFA limits

Achieved

Thailand received the WHO Validation Certificate of Trans-Fat Elimination.



IMPLEMENTATION OF WHO SHAKE TECHNICAL PACKAGE FOR REDUCING SALT INTAKE

Technical package element		Implementation Status
Food reformulation policies	Policies that set scope and criteria for maximum limits or targets for the sodium content in prepackaged food	In process
Food and nutrition labelling policies	Policies that set scope and criteria to: List and declare sodium on packaged food	Achieved
	Regulate the use of sodium-related claims on packaged food	Not achieved
	Use front-of-pack labelling (FOPL) to provide interpretive information about sodium content	Achieved
Food marketing restriction policies	Policies that set scope and criteria for restricting food marketing to protect children from its harmful impact on diet, including consumption of high-sodium food	In process
Behavior change communication and mass media campaigns for healthy diets	A set of communication activities designed to change behaviors to reduce consumption of salt and high sodium food	Achieved
Food procurement and service policies	Policies that set scope and criteria for sodium content in food served or sold in public settings and/or procured by government for food programmes	In process
Taxation on unhealthy food	Policies that set scope and criteria for taxes on unhealthy food, including high-sodium food	In process

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Community sodium reduction initiative in Thailand¹⁻⁵

Since 2016, Thailand has been implementing the National Strategic and Action Plan targeting a 30% reduction in sodium intake among its population by 2025. Despite several efforts, the average sodium intake among Thai adults remains high at 3636 mg/day, similar to global trends. To accelerate progress, in 2022, the Ministry of Public Health (MoPH), through the Department of Disease Control (DDC), collaborated with the stakeholders concerned to launch a nationwide community sodium reduction initiative across all 77 provinces.

The initiative was driven in collaboration with the provincial health offices (PHOs), the Bangkok Metropolitan Administration (BMA), local administrative offices, the Thai Low Salt Network and the Thai Dietitian Association. Key activities included the adoption of a Community Sodium Reduction Policy through the signing of memorandums of understanding (MoUs) and/or the establishment of sodium reduction steering committees across all 76 provinces and the BMA. A national surveillance system and database were also developed to monitor sodium content in non-packaged foods, utilizing a "Salt Meter", locally developed by Mahidol University to measure sodium content in liquid foods and supported by the "Thai Salt Survey" mobile application for data collection.

Additionally, provincial action and monitoring plans were formulated to guide implementation according to five strategic approaches: (1) raising awareness on sodium reduction by assessing eating behaviours and creating tailored educational messages; (2) reformulating local packaged foods by supporting food producers to develop low-sodium recipes and pursue certification of quality local products; (3) reformulating local out-of-home foods by training and establishing networks of provincial dietitians to assist food stalls and restaurants in offering low-sodium options; (4) creating enabling environments for sodium reduction in key settings such as schools, hospitals and workplaces by promoting healthy menus and low-sodium products; and (5) implementing low-sodium community initiatives through district health promotion committees, where tailored measures and target groups are defined and implemented jointly by multisectoral committees to reduce community sodium consumption.



Health workers promoting reduced salt use to prevent hypertension in Thailand

By 2023, all 77 provinces had launched the initiatives, guided by the operational guideline on sodium reduction at the provincial level that was developed to provide the information and methodology in planning, implementing and monitoring sodium reduction initiatives. As of 2024, 44 provinces had surveyed sodium content in both home-cooked and out-of-home foods using the "Salt Meter" and the "Thai Salt Survey" application. Results showed that many popular Thai dishes – such as noodles with soup, *tom yam* (hot and sour soup), *som tum* and spicy salads – contained high sodium levels (>0.9 g per 100 ml).

The data collected were used to raise awareness among local communities and food producers and to support targeted reformulation efforts. Networks of dietitians were trained in 77 provinces, helping local food producers develop low-sodium versions of renowned local dishes such as *Jaew Hon*, *Keang Hung Ley* and *Keang Tai Pla*. These efforts reflect a strong whole-of-society approach, engaging multiple sectors at both national and subnational levels to tackle the shared challenge of reducing NCD risk factors through sodium reduction.

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TIMOR-LESTE

NCD prevention and management



The Ministry of Health (MoH) in Timor-Leste is strongly committed to improving the health of its population. The Multisectoral NCD Action Plan (2018–2021) highlighted the importance of a coordinated multisectoral approach, health promotion, primary prevention, and early detection and management of NCDs.¹ In June 2016, the country enacted Decree Law Number 14/2016 – its primary tobacco control legislation – which includes measures such as smoke-free areas, restrictions on tobacco advertising, promotion and sponsorship as well as packaging and labelling requirements.

Since 2017, Timor-Leste has been implementing WHO PEN, designating a health professional, often a nurse, as responsible for NCD service delivery at health centres. Nurses and midwives play a crucial role in this effort as they are highly trusted by the community. The country has one national hospital, five regional hospitals, 333 health posts and 76 community health centres. Under the Multisectoral NCD Action Plan, community- and family-based health promotion, screening programmes and follow-up treatment for conditions such as hypertension and diabetes mellitus have been expanded.

Efforts are underway to standardize operational procedures for health-care workers to promote team-based care and ensure the availability of medicines and equipment at all levels of the health-care system.² Additionally, Timor-Leste utilizes DHIS2 to collect and track morbidity and mortality data, including key NCD indicators. This system plays a vital role in monitoring and managing health outcomes across the country.

National commitment: In May 2023, the VIII Government Constitutional announced its commitment to place 50 000 individuals with hypertension and diabetes on protocol-based care by 2025. This commitment was reaffirmed by the IX Government Constitutional during National Health Day in June 2024.³



Timor-Leste: 50 000 people with hypertension and diabetes on standard care by 2025.

Healthy-lifestyle counselling

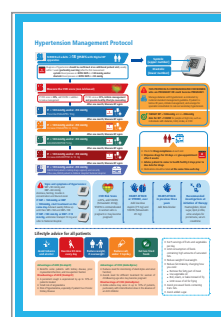


- ▶ National management protocols for hypertension and diabetes include counselling patients on tobacco use, diet, alcohol consumption and physical activity, using the 5As brief intervention approach.
- ▶ Counselling is provided by medical doctors, nurses and health-care providers.

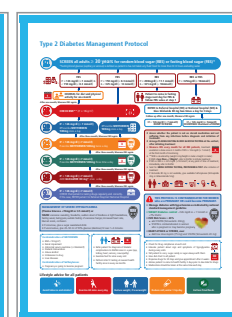
Evidence-based treatment protocols



- ▶ Evidence-based drug- and dose-specific national protocols for management of hypertension and diabetes in primary health care, aligned with WHO HEARTS and PEN technical packages, are being implemented since 2017. The protocols were again revised in 2024.
- ▶ At least 50% of the primary health care facilities in the country are estimated to be implementing protocol-based treatment for hypertension and diabetes.⁴
- ▶ According to the latest protocol, there is a provision for medication refill for a duration of 30 days.



Hypertension management protocol



Diabetes management protocol

Access to medicines and technology



Medicines[#]

- ▶ The Timor-Leste Essential Medicines List 2025 includes medicines (enalapril, amlodipine, metformin, glimepiride, gliclazide and insulin) that are part of the hypertension and diabetes management protocol used at the primary health care level.⁵
- ▶ These medicines are estimated to be “generally available” (available at 50% or more pharmacies) at primary care facilities of the public health sector.⁴
- ▶ m-Supply is an electronic logistics management information system (LMIS) for procurement, distribution and monitoring of medicines at health facilities.
- ▶ Medicines are procured annually, along with provisions for emergency tenders, through a centralized mechanism managed by the Central Medical Stores – SAMES I.P.⁶

Technology^{##}

- ▶ Equipment for measuring blood glucose, blood pressure, total cholesterol are estimated to be “generally available” at primary health care facilities of the public and private health sectors.⁴
- ▶ HbA1c is carried out at the National Hospital, while renal function test, liver function test, uric acid, cholesterol, plasma glucose, electrolytes, urine strips ketone and glucose, urine strips for albumin assay testing are reported to be “generally not available” at primary health care facilities of the public and private health sectors.

Risk-based CVD management



- ▶ Cardiovascular risk stratification is included in the nationally approved protocol for hypertension and diabetes management.
- ▶ 25%–50% of primary care facilities provide non-laboratory-based risk stratification.⁴
- ▶ There is a national guideline, as per WHO PEN protocol, to provide statin to people having a 10-cardiovascular risk ≥ 20 .

[#] “Generally available” is defined as “at 50% or more pharmacies”.

^{##} “Generally available” is defined as “at 50% or more health-care facilities”.

Team-based care



- ▶ Primary care is provided through a network of treatment posts, health posts and community health centres (CHCs). Under the Integrated Health Programme (PIS), there is a provision of specialist outreach clinic and mobile medical clinic. The staff of treatment posts, health posts, CHCs and the District Health Office function as a team to provide the screening and management of NCDs in the community.
- ▶ The Ministry of Health, Timor-Leste, is mandated to provide quality health services to the national population, covering the complete spectrum of health-care services. These health services will be delivered at different platforms – health facility visits, outreach sessions (SISCa and specialist outreach clinics, domiciliary/doorstep visits and school health checkups), through a diverse range of health-care workers, including not only the health workers at facilities, but also the mobile health teams (MHTs) and the community health-care volunteers (CHVs).
- ▶ Treatment initiation is ensured by a medical doctor, but drug refill can be conducted by a nurse, midwife or pharmacist.
- ▶ 255 health-care providers have been trained in protocol-based hypertension and diabetes care in 2024, of whom 54 were doctors, 133 nurses and 68 other health-care workers.
- ▶ There is a provision available for health-care providers at primary health facilities to consult specialists on issues related to specific individuals. Routine teleconsultation service between community clinics and higher facilities is currently not available.

Systems for monitoring



- ▶ Paper-based systems are utilized to collect and report NCD programme indicators at primary health care facilities.
- ▶ Initiated in two municipalities in 2024, the patient information system at primary health care facilities is capable of identifying and tracking hypertension and diabetes patients using a unique identifier during each visit.
- ▶ In the new NCD monthly reporting format, information will be generated for diabetes and hypertension control rates.⁷
- ▶ 25%–50% of primary health care facilities use dashboards to visualize performance and trends of key hypertension and diabetes indicators.
- ▶ Country has a DHIS2-based electronic system to facilitate reporting of aggregated patient data on NCDs from primary health facilities to district/subnational and national levels. The information is compiled at the municipality level and national level through data entry operators.
- ▶ The Timor-Leste Health Information System (TLHIS2) is a customized DHIS2 platform for Timor-Leste's primary health care. It registers NCD OPD data, covering hypertension, diabetes, cardiovascular diseases, COPD and cancers, thereby enabling health-care workers to analyse trends and inform policy decisions.

Progress of hypertension and diabetes coverage and control

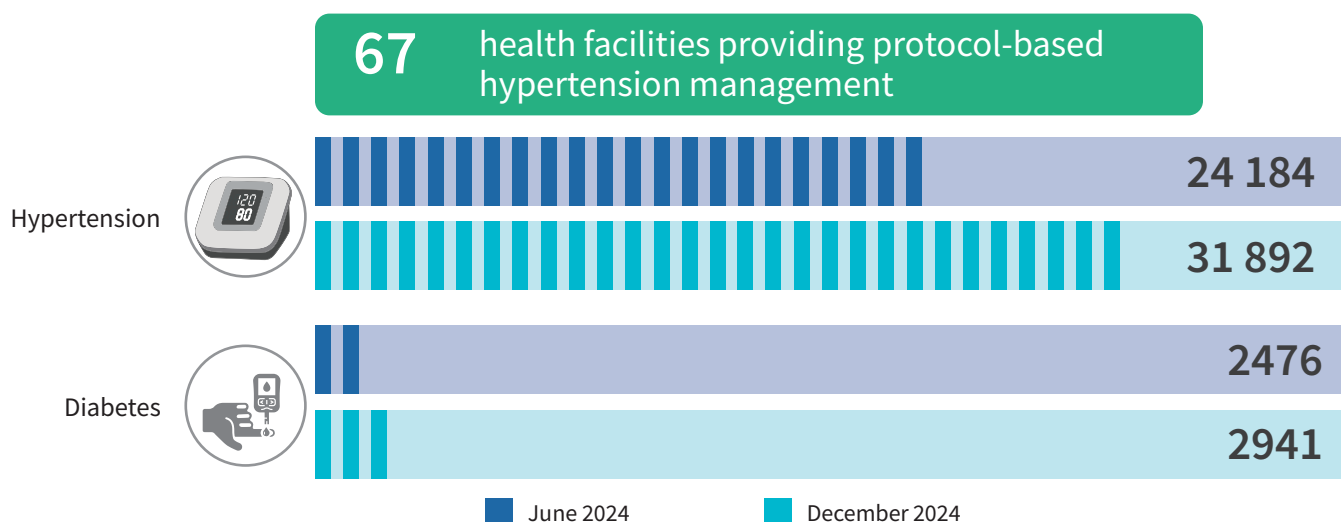


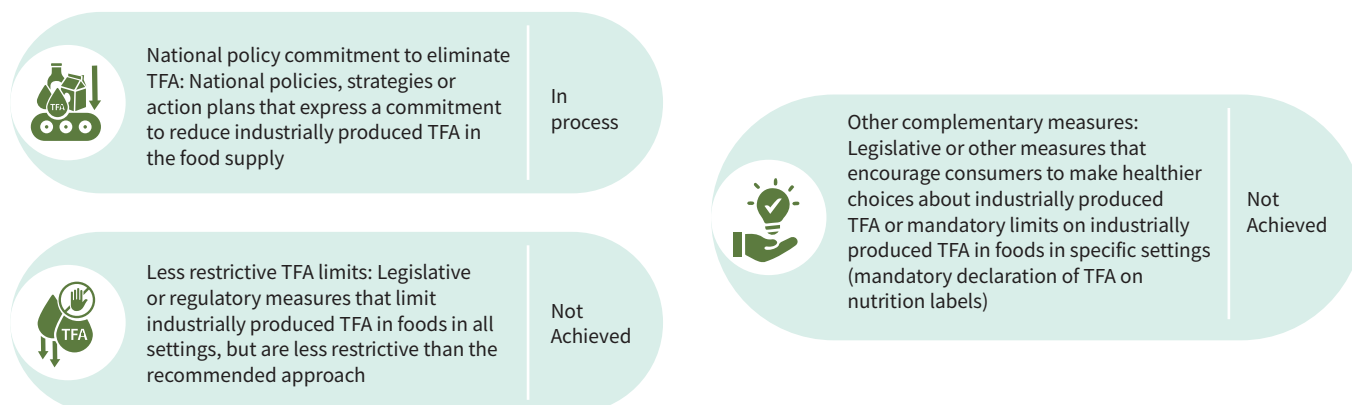
Figure 1: Number of persons with hypertension and/or diabetes placed on protocol-based management in health care facilities/clinics (2024)*

*Does not include multiple counts of single person

MPOWER		Coverage	Achievements
M onitor tobacco use and prevention policies		Moderate measure achieved	- Regularly monitoring the use of tobacco products among adults. - The Tobacco Questions for Surveys (TQS) are being integrated in national health surveys or multiple risk factor surveys.
P rotect people from tobacco smoke		Moderate measure achieved	Incremental progress in implementation of smoke-free laws with 6-7 public places completely smoke-free.
O ffer help to quit tobacco use		Moderate measure achieved	Smoking cessation support is available in some health clinics and hospitals across the country.
W arning about dangers of tobacco	Health warnings on Cigarette Packages	Complete measure achieved	Pictorial health warnings covering 93% of the principal display areas are mandated by law.
	Anti-tobacco Mass Media Campaigns	No measure or weak measure achieved	Achieved best-practice in mass media campaigns in GTCR 2019, 2021 and 2023 by conducting regular national-level anti-tobacco campaigns with at least seven appropriate characteristics including airing on television and/or radio.
E nforce bans on tobacco advertising, promotion and sponsorship		Moderate measure achieved	Comprehensive ban on some forms of TAPS.
R aise taxes on tobacco		Moderate measure achieved	The share of total taxes in the retail price of the widely sold brand of cigarettes in the country is 70.1%.



REPLACE TRANS FAT IMPLEMENTATION OF WHO REPLACE TECHNICAL PACKAGE⁸



IMPLEMENTATION OF WHO SHAKE TECHNICAL PACKAGE FOR REDUCING SALT INTAKE

Technical package element		Implementation Status
Food reformulation policies	Policies that set scope and criteria for maximum limits or targets for the sodium content in prepackaged food	Not achieved
Food and nutrition labelling policies	Policies that set scope and criteria to:	
	list and declare sodium on packaged food;	Not achieved
	regulate the use of sodium-related claims on packaged food; and	Not achieved
	use front-of-pack labelling (FoPL) to provide interpretive information about sodium content.	Not achieved
Food marketing restriction policies	Policies that set scope and criteria for restricting food marketing to protect children from its harmful impact on diet, including consumption of high-sodium food	Not achieved
Behaviour change communication and mass media campaigns for healthy diets	A set of communication activities designed to change behaviours to reduce consumption of salt and high-sodium food	Achieved
Food procurement and service policies	Policies that set scope and criteria for sodium content in food served or sold in public settings and/or procured by government for food programmes	Not achieved
Taxation on unhealthy food	Policies that set scope and criteria for taxes on unhealthy food, including high-sodium food	Not achieved

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Reducing cardiovascular risk in Timor-Leste: A case study on integrating NCD services into PHC^{1,2}

As part of the SEAHEARTS initiative, Timor-Leste has committed to placing 50 million people with hypertension and diabetes on protocol-based management by 2025, a target known as “50X25”. This initiative serves as a measure to bridge the gap between the undetected and the untreated, and to address the growing public health challenge of NCDs.



Community health workers providing screening and primary healthcare services at a local health facility.

The NCD Risk Factor Survey 2023 revealed that over 165 000 individuals live with hypertension and approximately 24 800 with diabetes in Timor-Leste. However, less than 15% of these individuals are currently receiving treatment, highlighting significant gaps in early detection, access and continuity of care.

Timor-Leste's flagship initiative, Integrated Health Programme (Programa Integradu ba Saúde - PIS), is at the forefront of bridging these gaps, linking community care with specialized services and streamlining referral pathways to address key health priorities. The Ministry of Health has adopted a comprehensive five-fold integration model under the PIS approach to strengthen the quality of diabetes and hypertension care at both facility and community levels.

Late diagnosis of diabetes remains a major hurdle, often driven by low community awareness. Critical gaps identified during health facility assessments included: missed opportunities to offer screening for diabetes and hypertension to eligible individuals visiting health facilities; the need for simplified management protocols that can be implemented at the health post level; inconsistent availability and accessibility of medicines and diagnostics; and inefficient referral and follow-up systems.

To address this multifaceted challenge, Timor-Leste, under the aegis of PIS, launched phased interventions in Aileu and Dili municipalities, with support of WHO and World Diabetes Foundation. These interventions aimed to enhance awareness, expand screening and treatment coverage, and improve the quality of care for diabetes and hypertension, covering a population of 380 000 (28% of the country's population).

Since January 2024, several key milestones have been achieved. A comprehensive NCD Service Delivery Model focused on hypertension and diabetes was developed with the involvement of health-care providers. Management protocols for hypertension and diabetes were revised with support from clinicians and experts. Protocol medicines were included in the National Essential Medicines List (2025). A drug forecasting tool for quantifying medicines and diagnostics was pilot-tested in municipalities, and is now incorporated into the National Procurement Plan to ensure continuous availability. This has resulted in a 32% average increase in the availability of metformin and an average 90% increase in the availability of amlodipine medicines from that in the preceding year.

Outreach camps under PIS provided opportunistic screening for attendees and helped follow up on patients who had missed previous visits. Capacity-building sessions were conducted for over 100 health-care providers on the new service delivery approach, supported by regular supervision to help integrate these interventions into routine practice. Recording and reporting formats were simplified and an NCD passport was introduced to empower patients and support tracking. Facility-based indicators were integrated into the Health Management Information System and digital tracking tools are being piloted to improve continuity of care. Bidirectional screening for diabetes in all TB patients has been initiated.

As of December 2024, 3422 people in Aileu and 26 802 in Dili Municipality were receiving treatment (an increase of 19.22% over 12 months) – a success attributed to the integrated interventions outlined above. Over 5700 individuals have been offered opportunistic screening at clinics.

References

1. Reported by the Ministry of Health Timor Leste, April 2025 (internal communication).
2. Policy brief: Reaching the un-reached through INTEGRATED HEALTH PROGRAM (IHP).

Annexure 1. Checklist to guide the national implementation of SEAHEARTS milestones

Country:

Total population (2024):

Baseline data	Age group (in years)	Prevalence (number and %)	Aware/ diagnosed among those with disease (number and %)	On treatment among those with disease (number and %)	Controlled status among those with disease (number and %)	Source with the year
Hypertension						
Diabetes						

Theme	Mapping of services to prepare roadmap/ implement priority action and its monitoring	Response		Source
		Hypertension	Diabetes	
Governance and leadership	National commitment with a time-bound target to place people with hypertension and/or diabetes on protocol-based management by 2025 (yes/no) If yes,			
	◉ What is the target?			
	◉ Has the target been disaggregated and assigned to subnational levels? (yes/no)			
	◉ Have plans been designed to achieve the targets at subnational levels? (yes/no)			
	◉ Are the subnational plans being implemented at present? (yes – fully/yes – partially/no)			
	◉ According to government-endorsed sources of information, how many people with hypertension and diabetes are on treatment in the country? (specify, if available, if not, indicate as NA – not available)			
	Are evidence-based national guidelines/ protocols/ standards available for management (diagnosis and treatment) of hypertension and diabetes? If yes,			
	◉ Is it also presented as a drug- and dose-specific protocol (as an algorithm)? (yes/no)			
	◉ Is it specifying the services to be delivered at the primary health care level? (yes/no)			

Theme	Mapping of services to prepare roadmap/ implement priority action and its monitoring	Response		Source
		Hypertension	Diabetes	
Access to essential medicines and basic technologies	Are the medicines specified in the hypertension and diabetes management protocol for primary health care included in the National Essential Medicines List or an equivalent listing? (yes /no)			
	Does the national/subnational medicine supply chain management system/inventory system have the provision for ensuring continuous supply of essential medicines specified to be used for hypertension and diabetes treatment in primary health care? (yes/no)			
	What is the approximate availability of medicines specified in the hypertension and diabetes management protocol of primary health care facilities in the public sector? (generally available at more than 75%/50%–75%/less than 50%)			
	What is the approximate availability of basic technologies specified in the hypertension and diabetes management protocol to be performed at primary health care facilities in the public sector? (generally available in more than 75%/50%–75%/ less than 50%)			
Health workforce	Are medical doctors or other health staff personnel, authorized to initiate hypertension and diabetes treatment, available in primary health care? If yes,			
	Will at least 50% of the primary health care facilities have the above-mentioned cadres in service? (yes/no)			
Health service delivery	Does the country have a national screening programme targeting the general population for hypertension and diabetes treatment, with linkages for diagnostic services for screened positives? (yes / no) If yes,			
	What is the approximate proportion of screened positives from the above screening who will be enrolled for diagnostic services? (75% or more/50%–75%/<50%)			
	Does the country have a policy to offer opportunistic screening of blood pressure and blood sugar for patients and visitors with a link to diagnostic services? (yes/no) If yes,			
	What is the approximate proportion of health facilities in the country that offer opportunistic screening of blood pressure and blood sugar for patients and visitors with a link to diagnostic services? (75% or more/50%–75%/ <50%)			

Theme	Mapping of services to prepare roadmap/ implement priority action and its monitoring	Response		Source
		Hypertension	Diabetes	
	What is the approximate proportion of primary health care facilities which will be implementing the national guidelines/protocols/standards for the management (diagnosis and treatment) of hypertension and diabetes? (75% or more/50%–75%/<50%)			
	Is there a mechanism in primary health care that allows longer periods of prescriptions/drug refills for uncomplicated patients (30 days or more)?			
	Is there a mechanism at the primary health care level to track patients who have defaulted on treatment (e.g. through calls, messages or other follow-up methods)?			
Health information system	Is there a patient information system at public health facilities in the country to identify/track hypertension and diabetes patients during each visit, using a unique identifier?			
	Which mechanism is used at public health facilities to facilitate reporting of facility-level or aggregated patient-level data on NCD indicators? (electronic/paper-based/mixed)			
	What is the approximate proportion of public health care facilities using the electronic system? (75% or more/50%–75%/<50%)			
	Does the system in operation at public health facilities have the provision to generate any of the following indicators? (yes/ no)			
	Number of new patients registered			
	Proportion of registered patients diagnosed with hypertension/diabetes			
	Proportion of registered patients on treatment for hypertension/diabetes			
	Proportion of registered patients defaulting/missing clinics			
	Proportion of patients with controlled hypertension/diabetes			
	Is there a mechanism/provision at the national/subnational level for the periodic publication of any of the above information? (yes/no)			
	If yes, which of the indicators?			
	Number of new patients registered (yes/no)			
	Proportion of registered patients diagnosed with hypertension/diabetes (yes/no)			
	Proportion of registered patients on treatment for hypertension/diabetes (yes/no)			

Theme	Mapping of services to prepare roadmap/ implement priority action and its monitoring	Response		Source
		Hypertension	Diabetes	
	Proportion of patients with controlled hypertension/diabetes?			
	Is there a mechanism wherein the data on hypertension and diabetes services are reviewed and used as a management tool to discuss reaching the targets at the following levels? (yes/no)			
	National level			
	Subnational level			
	Public health facility level			
	Is there a mechanism to obtain data on the number of persons treated for hypertension and/or diabetes in the private sector?			
Health financing	Are the hypertension and diabetes services provided free of cost/or as part of the benefit package in primary care?			

